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Roche

HY 2026 results

Basel, 23 July 2026



Group

Thomas Schinecker
Chief Executive Officer

Performance

Portfolio progress

Outlook

HY 2026: COP growth at +10% ahead of sales growth at +6%*

HY results all at CER

- Group sales growth +6%, Pharma +6% and Diagnostics +3% (still impacted by the healthcare pricing reforms in China)
- Strong bottom line performance with Core OP +10% and Core OP margin +1.7%p; Core EPS +9%
- LOE impact of roughly CHF 0.6bn expected for 2026

Key milestones achieved in Q2

- Pharma regulatory: US approval Tecentriq in ctDNA-pos high-risk MIBC, US & EU filing Tecentriq in stage III dMMR/MSI-H colon cancer, US filing Enspryng in TED, US & EU filing Gazyva in MN, EU filing Gazyva in INS
- Pharma readouts: Positive Ph III (KRASCENDO 1) divarasib in 2L+ KRAS G12C+ NSCLC, positive Ph III (POYANG) Vabysmo in mCNV
- Business development: PathAI** acquisition to transform AI-driven diagnostics, Nurix Therapeutics collaboration for bexobrutideg (BTK degrader)
- Diagnostics launches: AXELIOS 1 next generation sequencing platform, Elecsys pTau217 CE mark, Elecsys IGRA TB CE mark, cobas HDV CE mark

Significant newsflow in 2026 ahead

- Pivotal Ph III readouts: Itovebi in 1L *PIK3CA*-mut HER2+ mBC and post CDKi HR+/HER2- mBC, Lunsumio in 2L+ FL, sefaxersen in IgAN
- Ph II readouts: emugrobart in obesity, enicepatide (CT-388) in obesity with T2D (study 104), CT-996 in obesity
- Diagnostic launches: VENTANA BAP1, eplex gastrointestinal panel

*All at CER; **pending deal closure; BAP1: BRCA1 associated protein-1; BTK: Bruton tyrosine kinase; CER: Constant exchange rates (avg. full year 2025); CDK: Cyclin-dependent kinase; dMMR: Deficient DNA mismatch repair; FL: Follicular lymphoma; HDV: Hepatitis D virus; HER2: Human epidermal growth factor; HR: Hormone receptor; IgAN: IgA nephropathy; INS: Idiopathic nephrotic syndrome; LOE: Loss of exclusivity incl. global losses of Avastin, Herceptin, MabThera/Rituxan, Lucentis, Actemra; (m)BC: (Metastatic) breast cancer; mCNV: Myopic choroidal neovascularization; MIBC: Muscle-invasive bladder cancer; MN: Membranous nephropathy; MSI-H: Microsatellite instability-high; NSCLC: Non-small cell lung cancer; PIK3CA-mut: Phosphatidylinositol 3-kinase, catalytic, alpha polypeptide mutated; T2D: Type-2 diabetes; TB: Tuberculosis; TED: Thyroid eye disease

HY 2026: Strong Pharma sales driving Group growth

Sales impacted by low flu season and Diagnostics healthcare pricing reforms in China

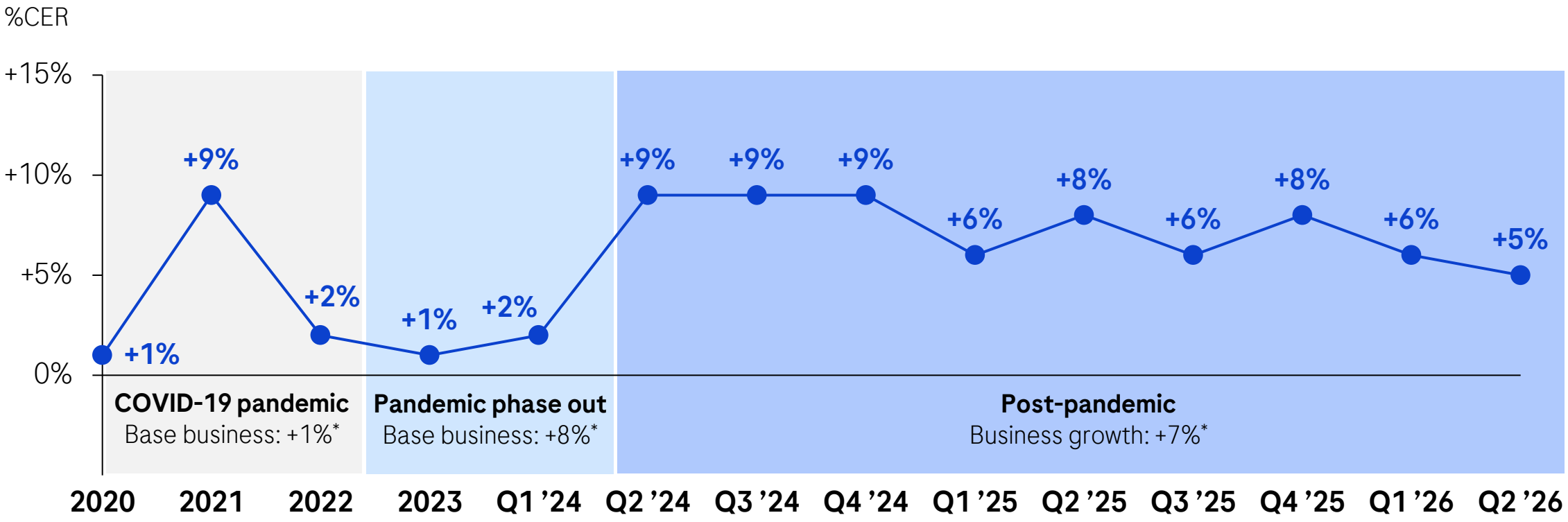
	HY 2026 CHFbn	HY 2025 CHFbn	Change in % CHF	Change in % CER
Pharmaceuticals Division	**23.6**	24.0	**-1**	**6**
Diagnostics Division	**6.7**	7.0	**-3**	**3**
Roche Group	**30.4**	30.9	**-2**	**6**

CER: Constant exchange rates (avg. full year 2025)

HY 2026: Consistent strong growth

Sales impacted by low flu season and Diagnostics healthcare pricing reforms in China

Group sales growth



*Average growth rate of quarterly CER growth rates for specified period; All growth rates at CER: Constant exchange rates (avg. full year of respective years); Base business: Pharma excluding Ronapreve and Diagnostics excluding COVID-19 diagnostic tests

Key growth drivers of the Roche portfolio

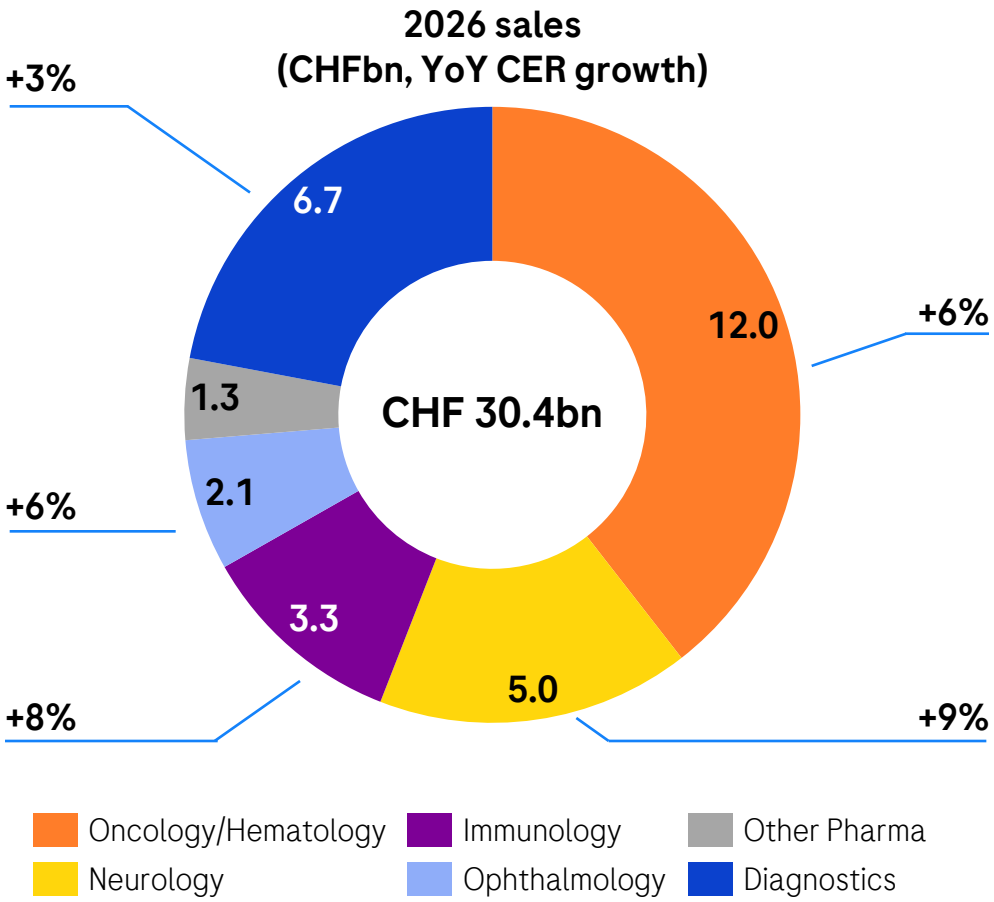
Growth driven by a broad and diversified portfolio

Diagnostics: Pathology Lab with strong growth; Core Lab growing despite China healthcare pricing reforms

Vabysmo: Continued global growth and market share gains

Xolair: Continued strong growth in food allergy, first biosimilar expected H2 '26

Gazyva: Strong initial uptake in LN



Phesgo: Conversion ongoing; HER2+ BC franchise to peak in 2026 with strong tail

Tecentriq: Accelerated growth driven by new indications

Alecensa: Flat growth expected for FY

Polivy: Continued strong 1L DLBCL uptake

Columvi: Driven by 2L+ DLBCL

Lunsumio: Strong 3L+ FL uptake

Hemlibra: Continued strong global growth

Ocrevus: Global growth driven by SC

Evrysdi: Expanding strong position in SMA

Totals and subtotals may include differences due to rounding; BC: Breast cancer; CER: Constant exchange rates (avg. full year 2025); DLBCL: Diffuse large B-Cell lymphoma; FL: Follicular lymphoma; HER2: Human epidermal growth factor; LN: Lupus nephritis; SC: Subcutaneous; SMA: Spinal muscular atrophy

Performance

Portfolio progress

Outlook

Pharma: Continuous pipeline progress through partnering and M&A

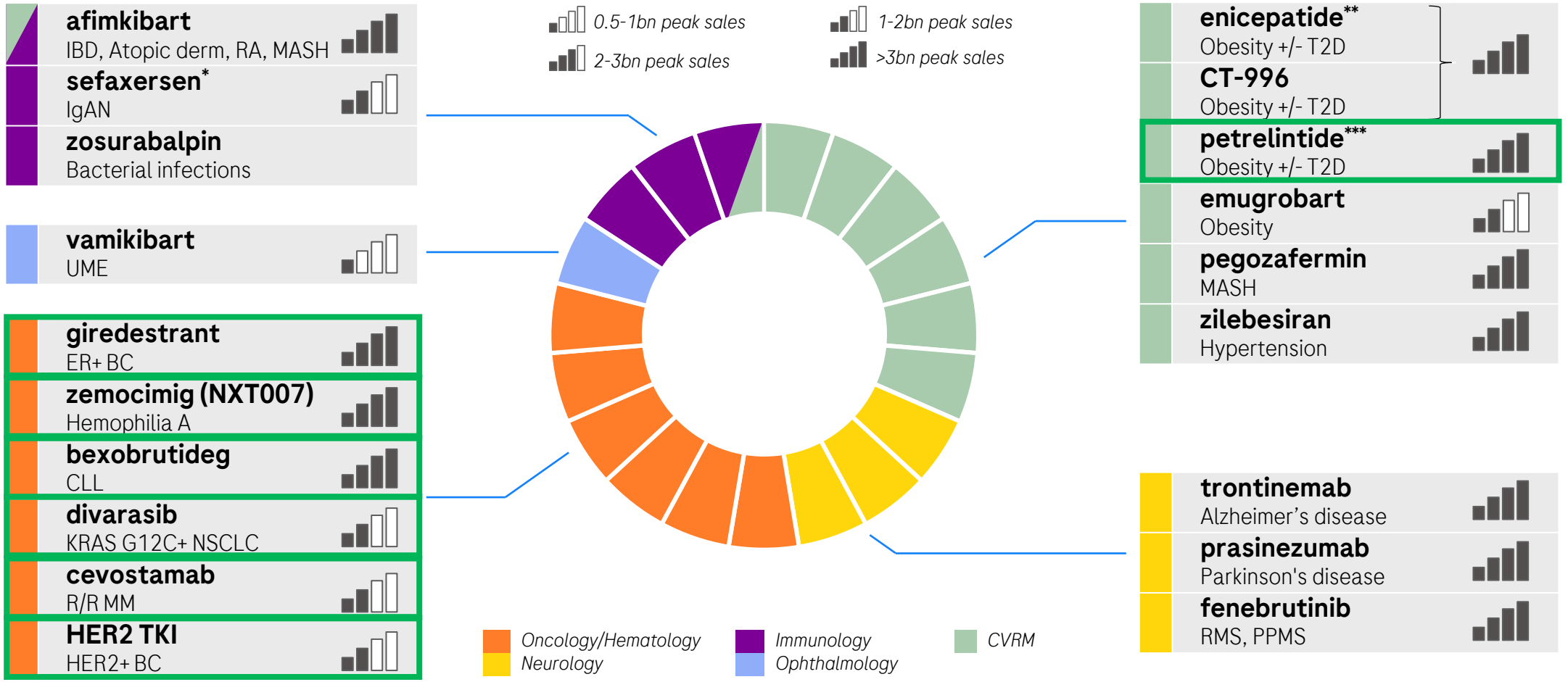
Addition of a BTK degrader with BIC potential through Nurix deal to strengthen oncology and beyond

ANTLERA	赞荣医药 Zion Pharma	Regor Therapeutics	NURIX	Telavant	Alnylam PHARMACEUTICALS	CARMOT THERAPEUTICS	ZEALAND PHARMA	89bio
Ophthalmology	**Onc/Heme**	**Onc/Heme**	**Onc/Heme, Immuno & Neuro**	**Immunology**	**CVRM**	**CVRM**	**CVRM**	**CVRM**
Wnt agonist in retinopathies	**HER2 TKI** in breast cancer	**CDKi portfolio** in breast cancer	**BTK degrader** in CLL, immunology & neurology	**Anti-TL1A** in inflammatory bowel disease	**siRNA targeting AGT** in hypertension	**Dual GLP-1/GIP RA** in obesity +/- diabetes	**Long-acting amylin analog** in obesity	**FGF21 analog** in MASH
Pre-clinical	**Ph II**	**Ph I**	**Ph III**	**Ph III**	**Ph III**	**Ph III**	**Ph II**	**Ph III**

ADC: Antibody-drug conjugate; AGT: Angiotensinogen; BIC: Best in class; BTK: Bruton tyrosine kinase; CDKi: Cyclin dependent kinase inhibitor; CLL: Chronic lymphocytic leukemia; CVRM: Cardiovascular, renal & metabolism; FGF21: Fibroblast growth factor 21; GLP-1/GIP: Glucagon-like peptide-1 / glucose insulinotropic polypeptide receptor agonist; MASH: Metabolic dysfunction-associated steatohepatitis; siRNA: Small interfering RNA; TKI: Tyrosine kinase inhibitor; TL1A: Tumor necrosis factor-like cytokine 1A; Wnt: Wingless-related integration site

Pharma: Up to 19 NMEs with launch potential by 2030

Positive Ph III (KRASCENDO 1) divarasib in 2L+ KRAS G12C+ NSCLC; upcoming sefaxersen readout in IgAN



*in collaboration with Ionis; **CT-388; ***in collaboration with Zealand Pharma; Peak sales shown unadjusted; CVRM: Cardiovascular, renal and metabolism; IgAN: IgA nephropathy; NME: New molecular entity; NSCLC: Non-small cell carcinoma

Diagnostics: Key Business Development across Customer Areas

PathAI acquisition* to expand pathology portfolio; recent deals to strengthen oncology ecosystem

STRATOS genomics inc.	TIB MOLBIOL	GenMark Dx	Freenome	SAGA DIAGNOSTICS	PathAI *	lumiraDx™
Molecular Lab	Molecular Lab	Molecular Lab	Molecular Lab	Molecular Lab	Pathology Lab	Near Patient Care
Sequencing-by-expansion technology	Ultra-rapid PCR assays development	Syndromic infectious disease testing	Early detection cancer screening	Ultra sensitive MRD technology	AI-powered pathology solutions	Portable PoC testing
2020	2021	2021	2021/2025	2026	2026	2024

Recent deals to expand oncology end-to-end offering

from early detection through diagnosis to disease monitoring

*pending deal closure; MRD: Minimal residual disease; PoC: Point of care

Diagnostics: Key technology launches driving future growth

AXELIOS 1 sequencing solution launched in June 2026

Key platform launches

Near Patient Care

Accu-Chek SmartGuide



Launched in 2024

Core Lab

cobas i 601 Mass Spec



Launched in CE markets in 2024, full US launch in 2026

Molecular Lab

AXELIOS 1 (SBX)



Launched in June 2026

Selected assays

- **cobas Lp(a) Gen 2** — Cardiovascular risk
- **Elecsys hs-TnT Gen 6** — Acute coronary syndrome
- **cobas HPV genotyping**[1] — Cervical cancer
- **FoundationOne Monitor**[2] — ctDNA monitoring
- **Elecsys pTau217** — Alzheimer's disease
- **Elecsys NfL** — Multiple sclerosis
- **cobas BV/CV** — BV/CV
- **Elecsys IGRA TB** — Tuberculosis

Customer Areas: Core Lab; Molecular Lab; Near Patient Care; Pathology Lab

Therapeutic Areas: *Cardiometabolic*; *Oncology*; *Neurology*; *Infectious Diseases*

1. In development; 2. Laboratory developed test which has not been reviewed or approved by the FDA; BV/CV: Bacterial vaginosis, candida vaginitis; HPV: Human papillomavirus; hs-TnT: high sensitivity Troponin T; IGRA: Interferon-gamma release assay; NfL: Neurofilament light chain; Lp(a): Lipoprotein(a); TB: Tuberculosis

Performance

Portfolio progress

Outlook

2026 guidance confirmed

LOE impact of roughly CHF 0.6bn expected for 2026

Group sales growth[1]	Mid single digit sales growth
Core EPS growth[1]	High single digit Core EPS growth
Dividend outlook	Further increase dividend in Swiss francs

1. At CER: Constant exchange rates (avg. full year 2025); LOE: Loss of exclusivity includes global losses of Avastin, Herceptin, MabThera/Rituxan, Lucentis and Actemra
Note: Guidance according to the official publication in the respective press release of Roche Holding Ltd



Finance

Alan Hippe
Chief Financial Officer

Results

Cash & balance sheet

Currency guidance & outlook

HY 2026: Group performance

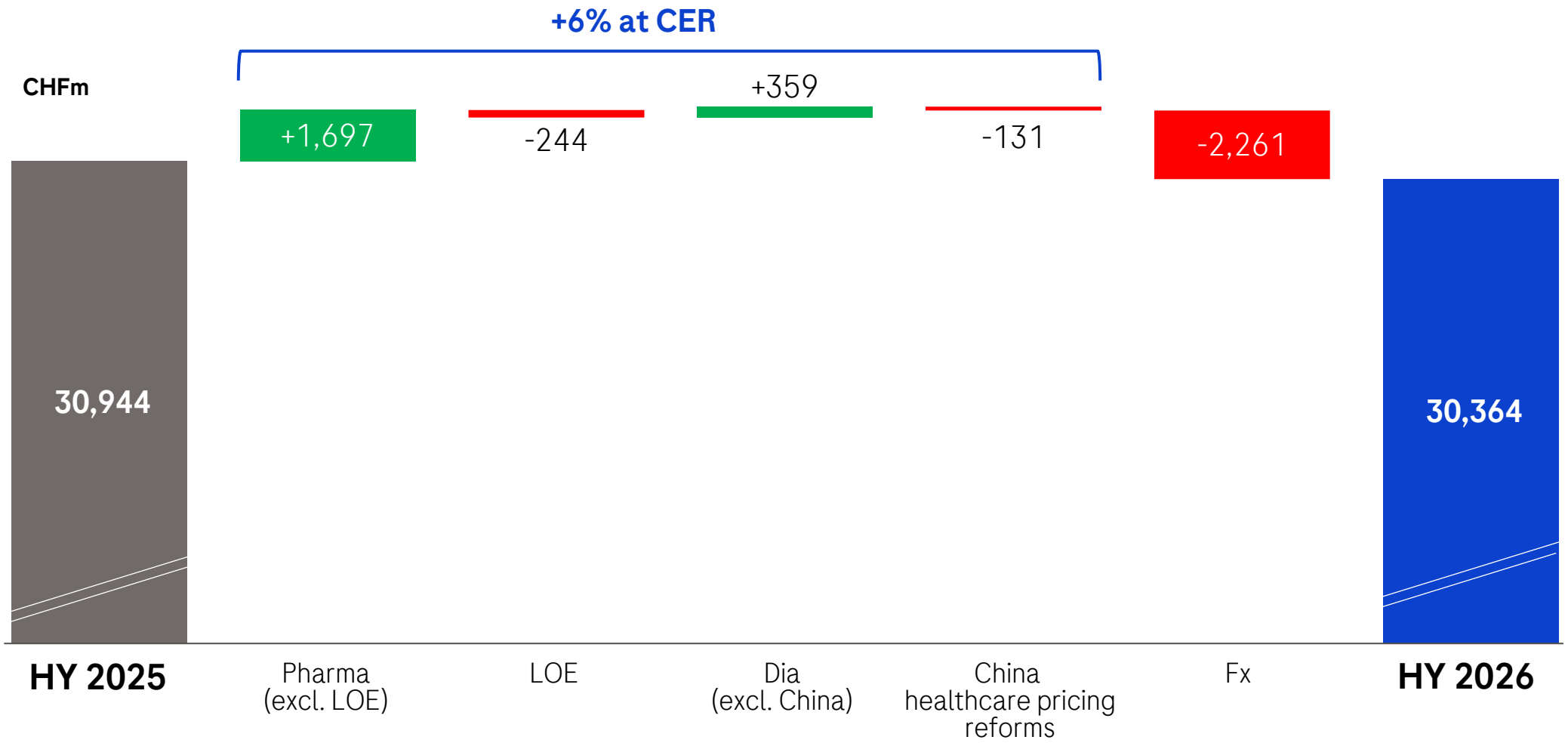
Sales increase of +6% and Core EPS increase of +9% at CER

	2026 CHFm	2025 CHFm	Change in % CHF	Change in % CER
Sales	**30,364**	**30,944**	**-2**	**+6**
Core operating profit	**11,856**	**12,010**	**-1**	**+10**
as % of sales	*39.0*	*38.8*		
Core net income	**9,216**	**9,319**	**-1**	**+11**
as % of sales	*30.4*	*30.1*		
Core EPS (in CHF)	**10.85**	**11.08**	**-2**	**+9**
IFRS net income	**7,325**	**7,832**	**-6**	**+6**
as % of sales	*24.1*	*25.3*		
Operating free cash flow	**6,264**	**6,114**	**+2**	**+21**
as % of sales	*20.6*	*19.8*		
Free cash flow	**4,197**	**3,319**	**+26**	**+58**
as % of sales	*13.8*	*10.7*		

All numbers in CHFm, except Core EPS in CHF; CER: Constant exchange rates (avg. full year 2025); EPS: Earnings per share

HY 2026: Group sales growth +6%

Business growth exceeding impact of healthcare pricing reforms in China and LOE



Totals may include differences due to rounding; CER: Constant exchange rates (avg. full year 2025); LOE: Loss of exclusivity includes global losses of Avastin, Herceptin, MabThera/Rituxan, Lucentis and Actemra

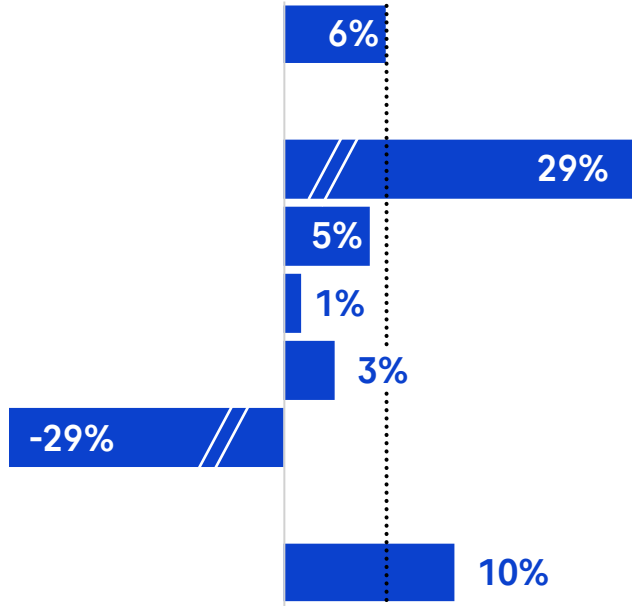
HY 2026: Group core operating profit

Core operating profit up +10%, driven by higher sales and effective cost management

	2026	
	CHFm	Abs. CERm
Sales	**30,364**	**+1,681**
Other revenue	1,088	+258
Cost of sales	-7,673	-391
R&D	-5,765	-66
SG&A	-6,370	-221
OOI&E	212	-87
Core operating profit	**11,856**	**+1,173**
Core OP as % of sales	*39.0%*	
At CER		*39.9%*
		(HY 2025: 38.2%)

2026 vs. 2025

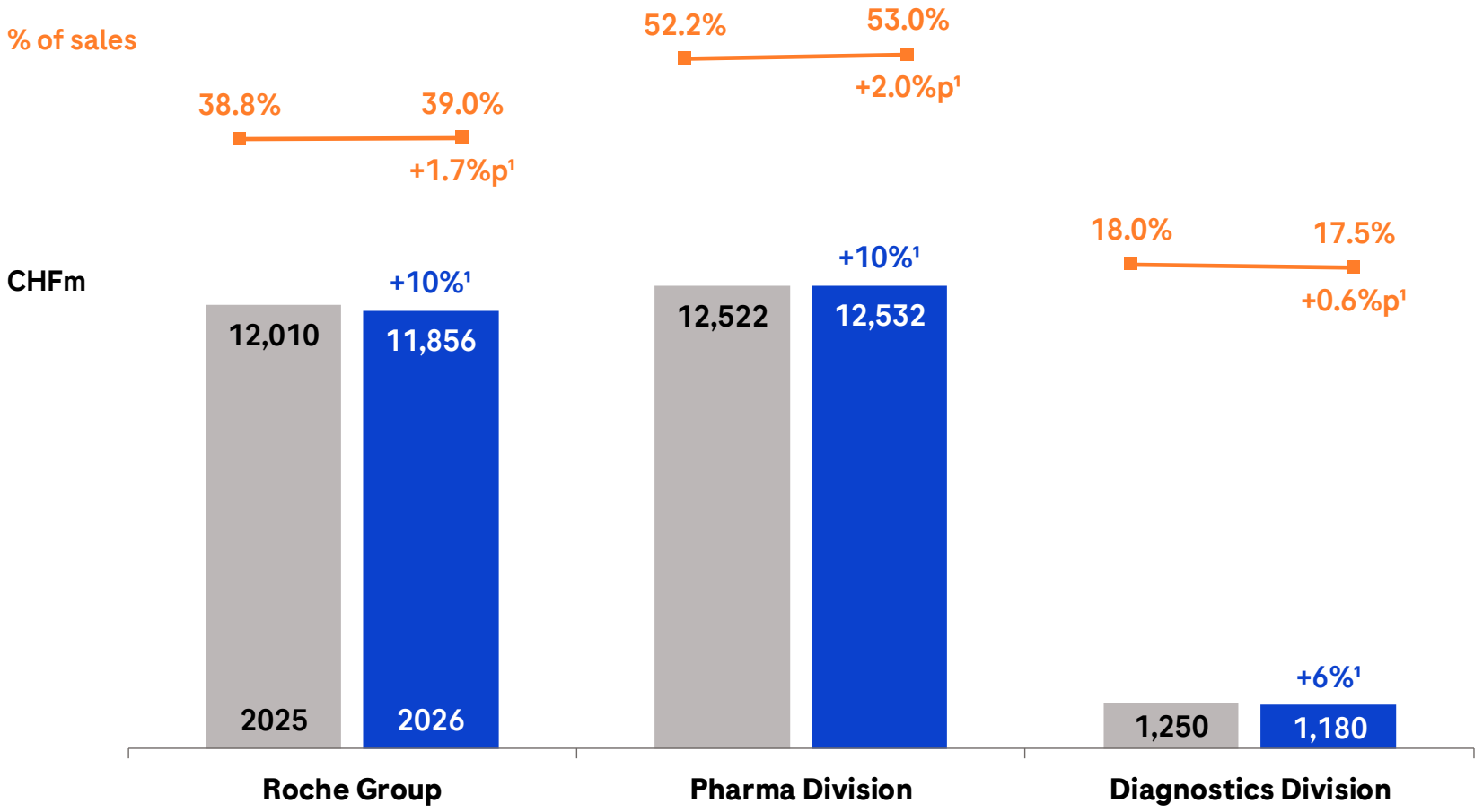
CER growth



-1% in CHF

CER: Constant exchange rates (avg. full year 2025); OOI&E: Other operating income & expense; OP: Operating profit; R&D: Research & development; SG&A: Selling, general & administration; Totals may include differences due to rounding

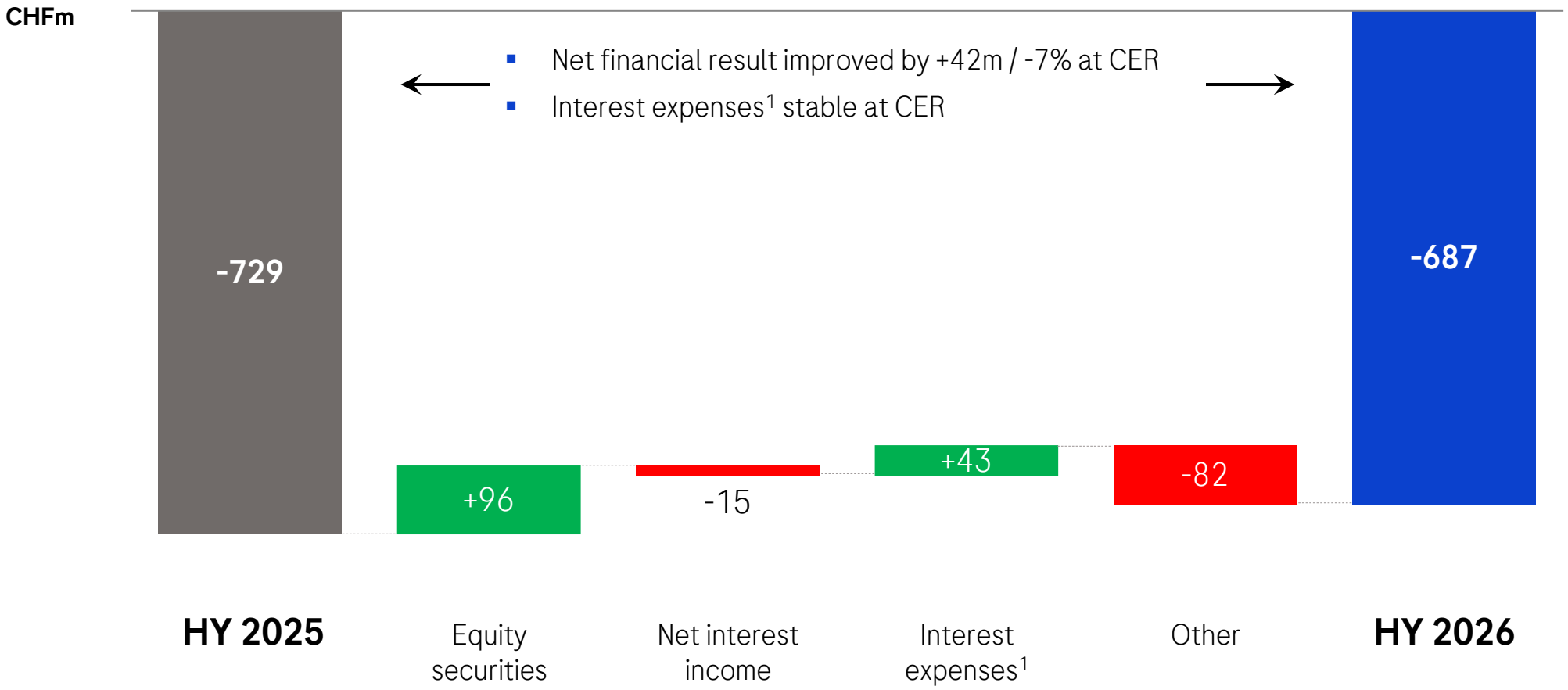
HY 2026: Core operating profit and margin



Note: Group core operating profit includes -1.9bn from Corporate (-1.8bn in 2025); 1. At CER: Constant exchange rates (avg. full year 2025)

HY 2026: Core net financial result

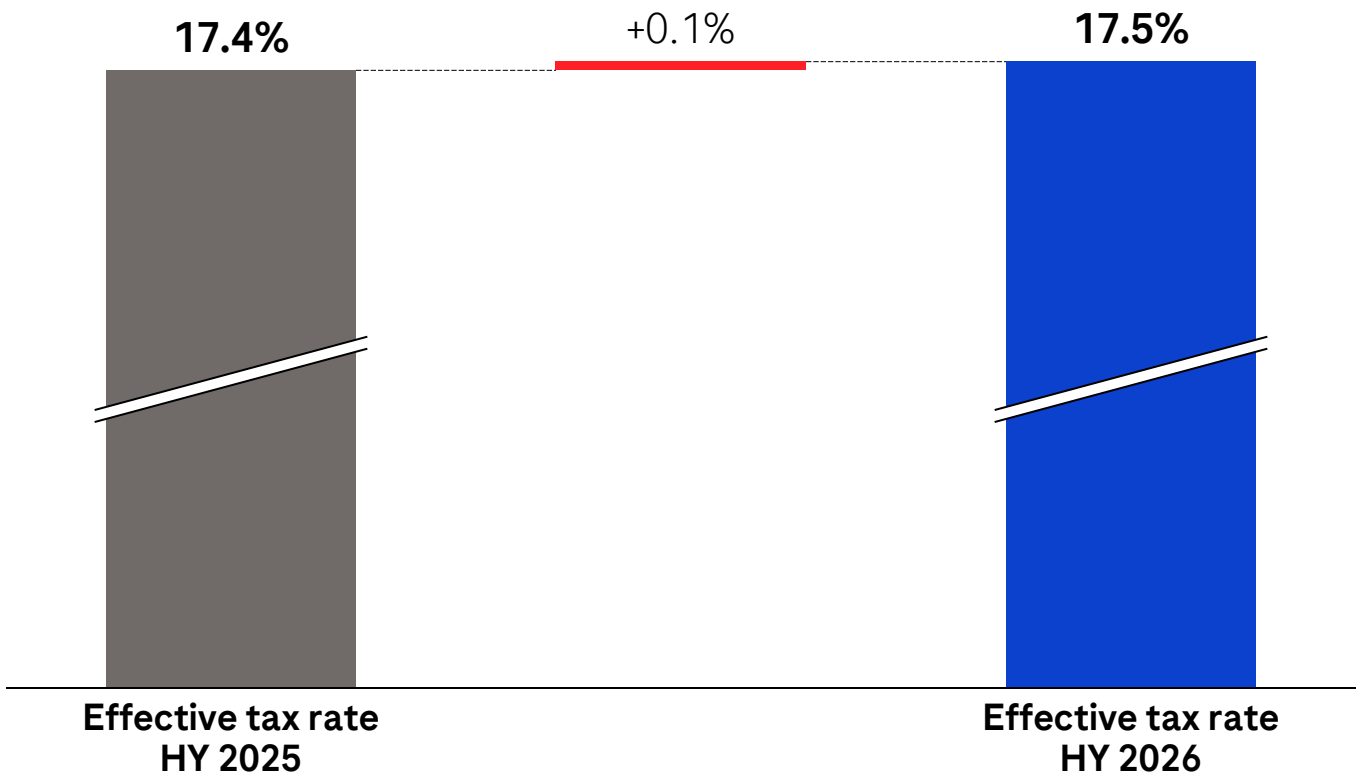
Improvement mainly driven by higher gains on equity securities



1. Incl amortization of debt discount and net gains on interest rate derivatives; CER: Constant exchange rates (avg. full year 2025)

HY 2026: Core tax rate

Tax rate stable compared to HY 2025



All at reported exchange rates

HY 2026: Core EPS

Increase in operations partially offset by lower gains on product disposals



All at CER: Constant exchange rates (avg. full year 2025); 1. Core operating profit excl. gains/losses on product disposals and gains from disposal of subsidiaries (InterMune sale)

HY 2026: Non-core and IFRS income

Non-core items up vs. PY mainly due to higher impairment of intangible assets

	2026 CHFm	2025 CHFm	Var. at CERm	Change in % CHF	Change in % CER
Core operating profit	**11,856**	**12,010**	**+1,173**	**-1**	**+10**
Global restructuring plans	-765	-1,023	+222		
Amortisation of intangible assets	-391	-348	-67		
Impairment of intangible assets[1]	-1,045	-235	-855		
M&A and alliance transactions	18	10	+8		
Legal & environmental[2]	-2	-84	+82		
Total non-core operating items	-2,185	-1,680	-609		
IFRS operating profit	**9,671**	**10,330**	**+564**	**-6**	**+6**
Total financial result & taxes	-2,346	-2,498	-77		
IFRS net income	**7,325**	**7,832**	**+488**	**-6**	**+6**

Totals & sub-totals may include differences due to rounding; 1. Incl. goodwill; 2. Incl. pension plan settlements; CER: Constant exchange rates (avg. full year 2025)

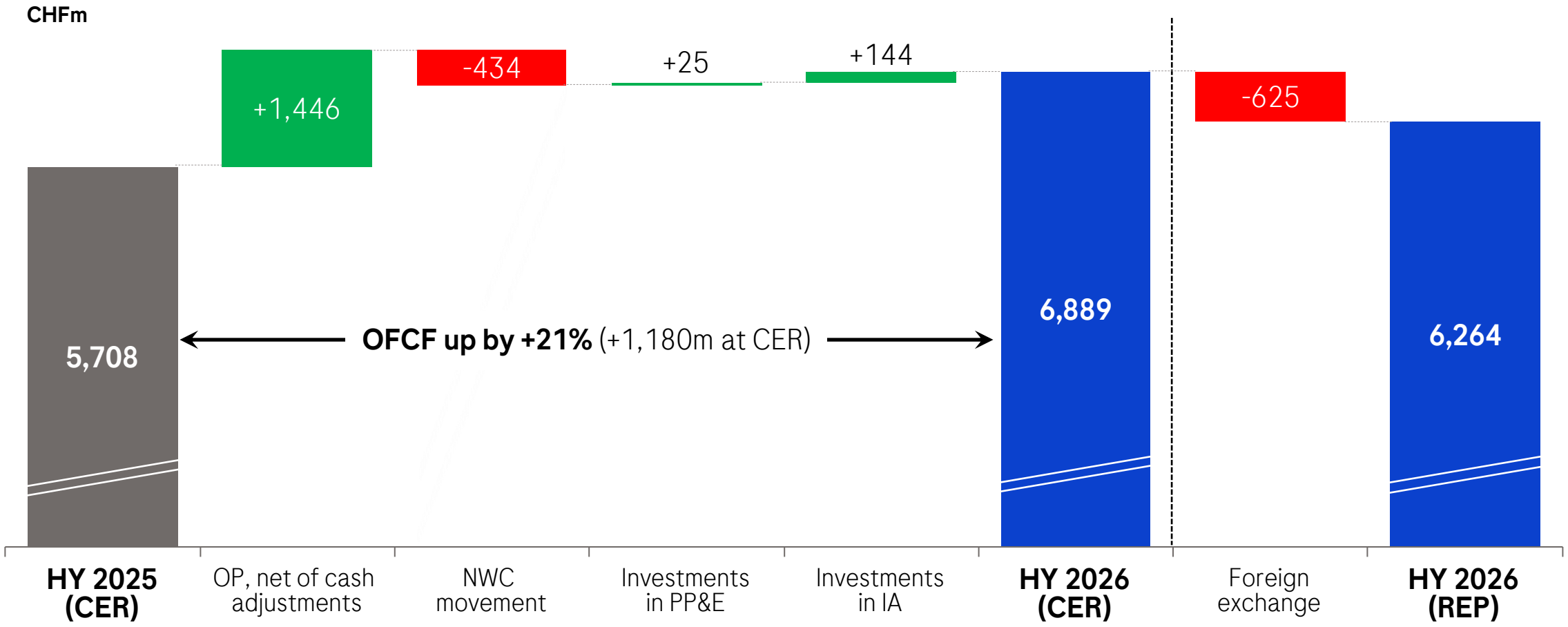
Results

Cash & balance sheet

Currency guidance & outlook

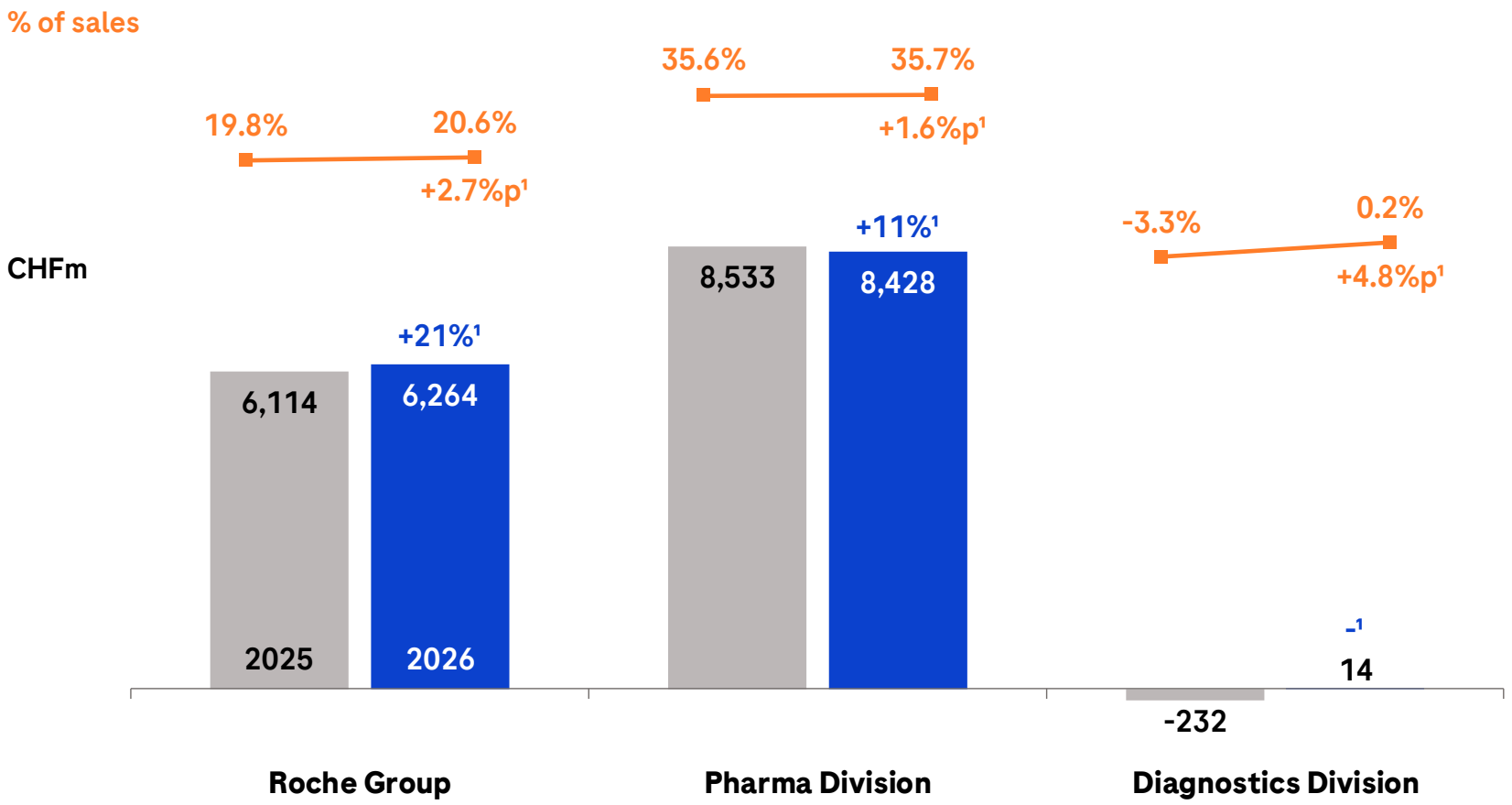
HY 2026: Group operating free cash flow

OFCF up by +21% mainly driven by higher cash operating profit, partly offset by NWC movement



CER: Constant exchange rates (avg. full year 2025); IA: Intangible assets; NWC: Net working capital; OP: Operating profit; PP&E: Property, plant & equipment incl. lease liability paid; Totals may include differences due to rounding

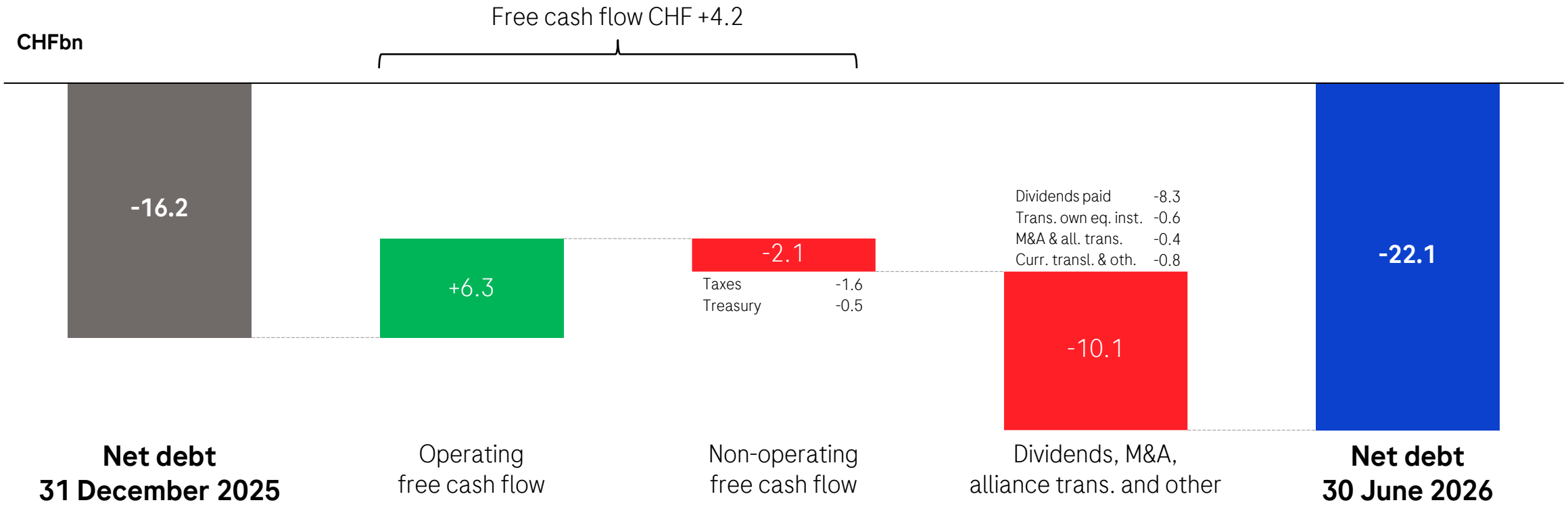
HY 2026: Operating free cash flow and margin



Note: Group operating free cash flow includes -2.2bn from Corporate (-2.2bn in 2025); 1. At CER: Constant exchange rates

HY 2026: Group net debt development

Net debt higher by CHF -5.9bn vs. YE 2025

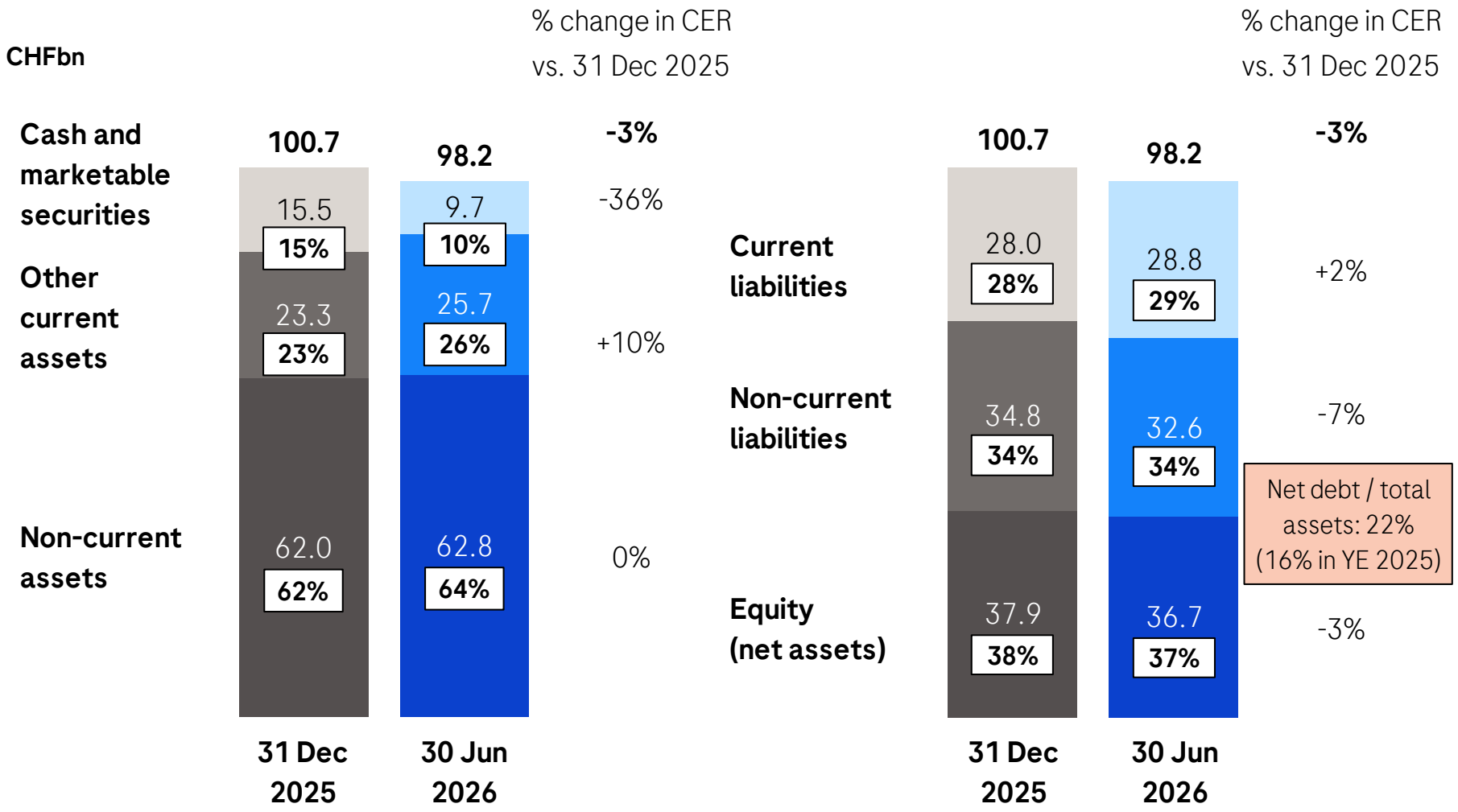


		Intangible assets	Equity	M&A	Total
Thereof investments in innovation:	**2026**	-1.3	-0.1	-0.4	**-1.8**
	2025	-1.5	0.0	-0.9	**-2.4**

M&A: Mergers & acquisitions

Balance sheet 30 June 2026

Equity ratio at 37% (31 Dec 2025: 38%; 30 Jun 2025: 35%)



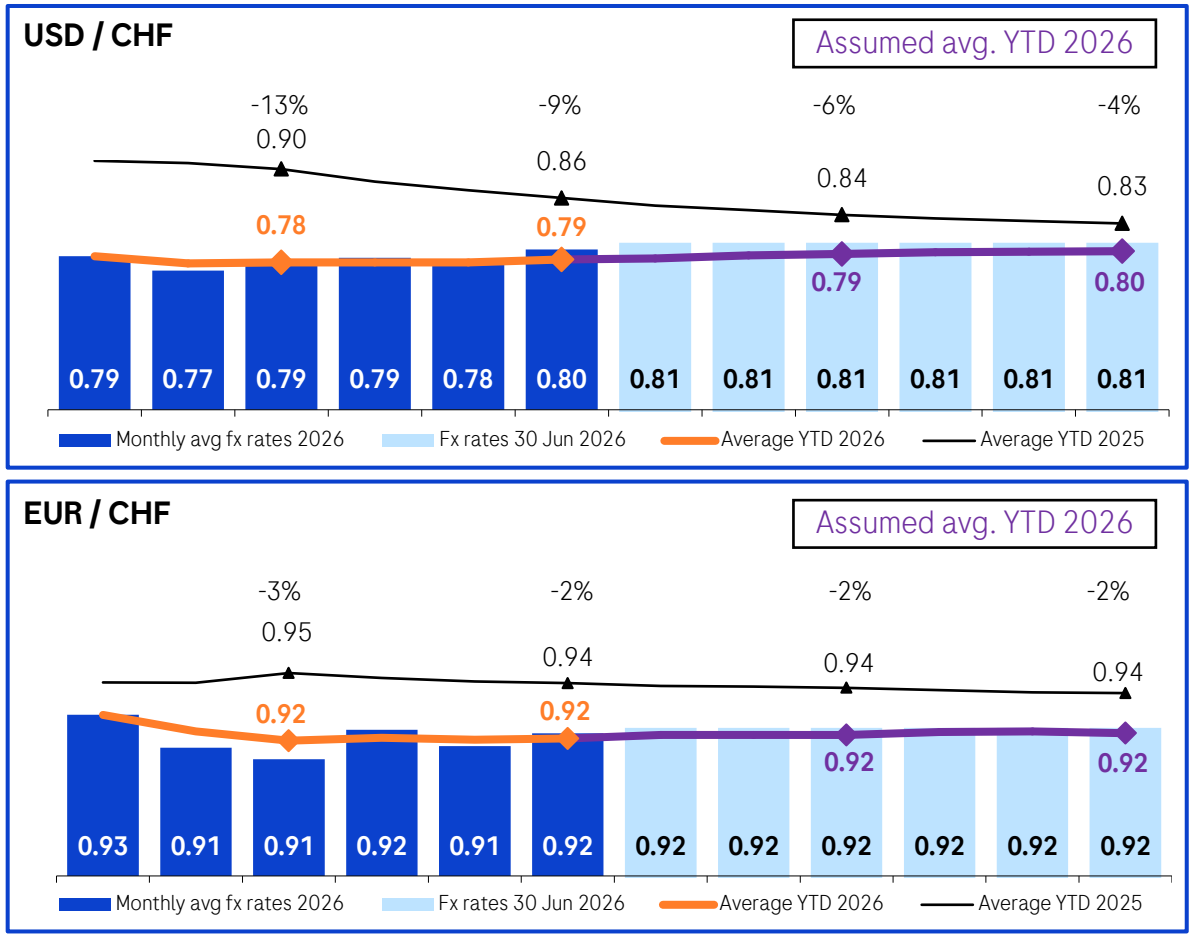
CER: Constant exchange rates (avg. full year 2025)

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Expected 2026 currency impact



Assuming the 30 June 2026 exchange rates remain stable until the end of 2026.

2026 impact[1] is expected to be (%p):

	Q1	Q2	Q3	Q4
Sales	**-11**	**-4**	0	0

	Q1	HY	YTD Sep	FY
Sales	**-11**	**-8**	-5	-4
Core operating profit		**-11**		-6
Core EPS		**-11**		-6

1. On Group growth rates

2026 guidance confirmed

LOE impact of roughly CHF 0.6bn expected for 2026

Group sales growth[1]	Mid single digit sales growth
Core EPS growth[1]	High single digit Core EPS growth
Dividend outlook	Further increase dividend in Swiss francs

1. At CER: Constant exchange rates (avg. full year 2025); LOE: Loss of exclusivity includes global losses of Avastin, Herceptin, MabThera/Rituxan, Lucentis and Actemra
Note: Guidance according to the official publication in the respective press release of Roche Holding Ltd



Pharmaceuticals Division

Teresa Graham
CEO Roche Pharmaceuticals

HY 2026: Pharmaceuticals sales

All regions delivering growth

	HY 2026 CHFm	HY 2025 CHFm	Change in % CHF	Change in % CER
Pharmaceuticals Division	**23,629**	**23,985**	**-1**	**6**
United States	12,229	12,670	-3	6
Europe	4,520	4,566	-1	1
Japan	1,360	1,425	-5	11
International	5,520	5,324	4	10

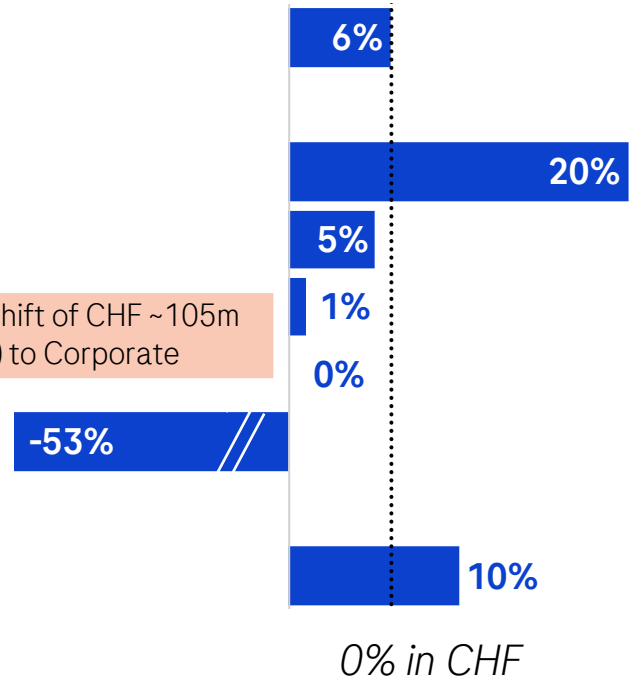
CER: Constant exchange rates (avg. full year 2025)

HY 2026: Pharmaceuticals core operating profit

Core operating profit growing ahead of sales with +10%, driven by effective cost management

	2026	
	CHFm	Abs. CERm
Sales	**23,629**	**+1,453**
Other revenue	965	+168
Cost of sales	-4,231	-217
R&D	-4,895	-40
SG&A	-3,031	-10
OOI&E	95	-109
Core operating profit	**12,532**	**+1,244**
Core OP as % of sales	*53.0%*	
At CER		*53.8%*
		(HY 2025: 51.8%)

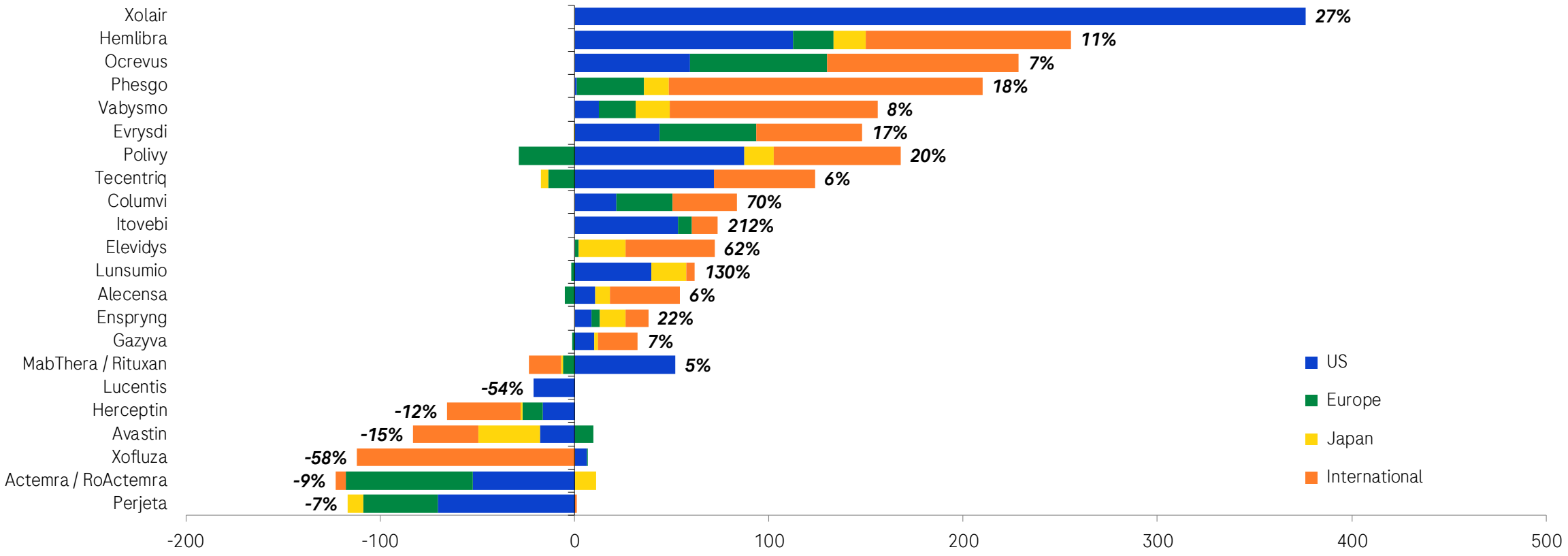
Includes shift of CHF ~105m (CER) to Corporate



CER: Constant exchange rates (avg. full year 2025); OOI&E: Other operating income & expense; OP: Operating profit; R&D: Research & development; SG&A: Selling, general & administration

HY 2026: Diversified set of key growth drivers

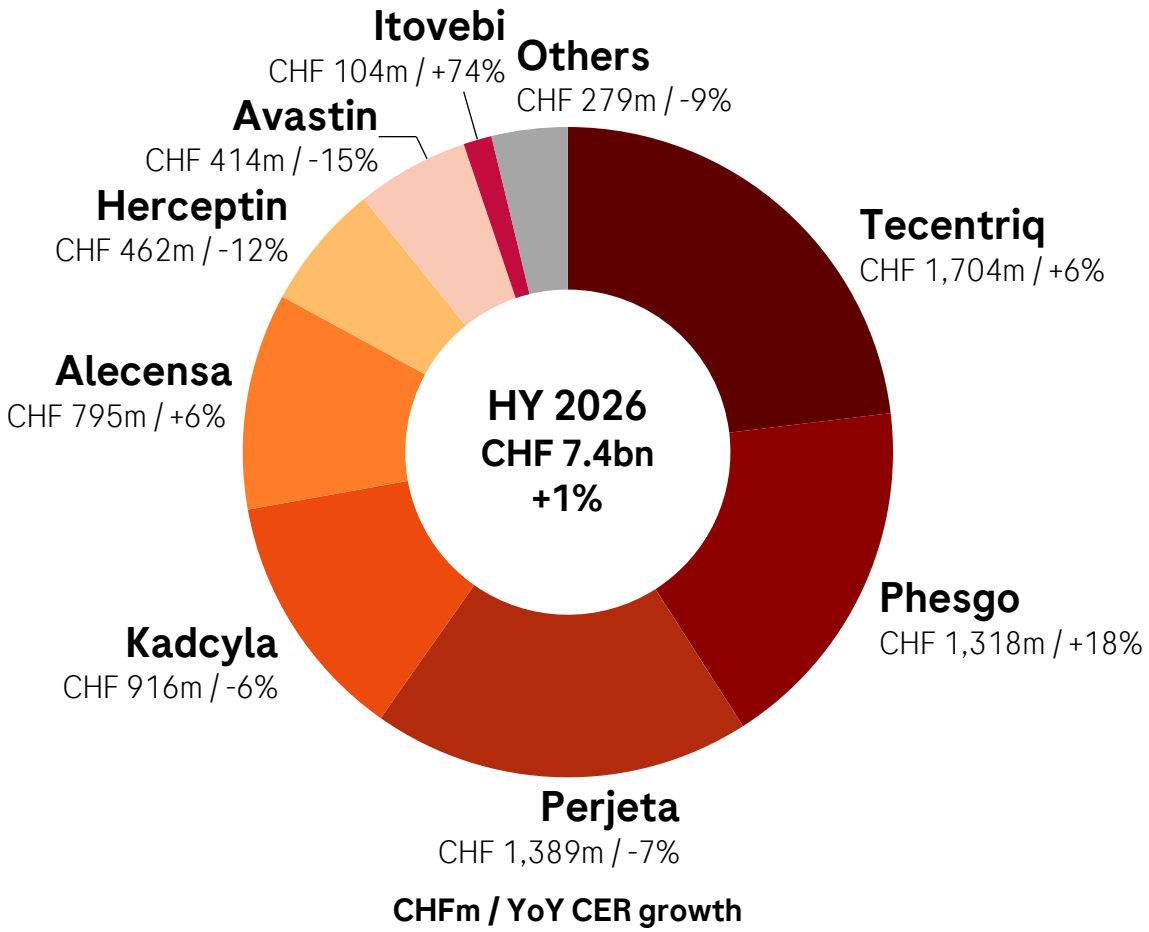
Xolair remaining top growth driver, closely followed by Ocrevus and Hematology franchise



Absolute values and growth rates at CER: Constant exchange rates (avg. full year 2025)

Oncology growth driven by Phesgo and Tecentriq

Giredestrant: lidERA PDUFA set for 30th Nov; positive Ph III for divarasib in 2L+ KRAS G12C+ NSCLC



Q2 update

- Phesgo: Global conversion rate at 54%*
- Perjeta: Conversion to Phesgo ongoing
- Kadcyla: Competitive pressure in US & EU as expected
- Itovebi: Launch in 1L *PIK3CA*-mut HR+ BC ongoing
- Tecentriq: Growth driven by new indications; US approval in MIBC (IMvigor011) achieved; US & EU filing in dMMR CC (ATOMIC) achieved, US PDUFA set for 9th Oct
- Alecensa: Flat growth expected for FY
- Giredestrant in BC: PDUFA in adj. ER+/HER2- (lidERA) set for 30th Nov
- Divarasib: Positive Ph III (KRASCENDO 1) in 2L+ KRAS G12C+ NSCLC

Outlook 2026

- Giredestrant in adj. ER+/HER2- BC: US approval (PDUFA 30th Nov)
- Giredestrant in post CDKi ER+/HER2- mBC: US approval (PDUFA 18th Dec)
- Ph III (INAVO121) Itovebi in post CDKi *PIK3CA*-mut HR+/HER2- mBC
- Ph III (INAVO122) Itovebi + Phesgo in 1L *PIK3CA*-mut HER2+ mBC

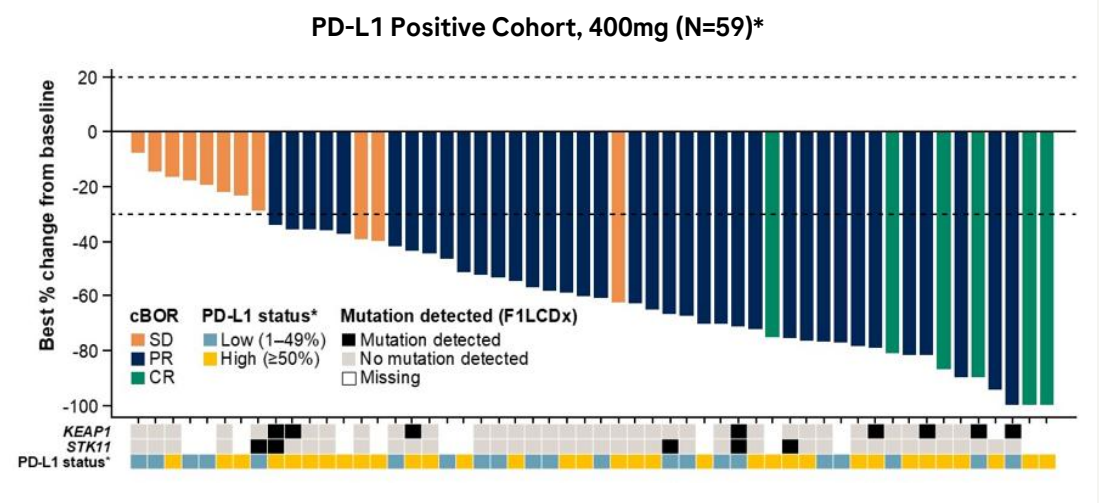
*Perjeta/Phesgo conversion rate calculated using volumes, currently taking 85 launch countries into account (83 countries at Q1 2026); CC: Colon cancer; CDKi: Cyclin dependent kinase inhibitor; CER: Constant exchange rates (avg. full year 2025); dMMR: Deficient mismatch repair; ER: Estrogen receptor; HER2: Human epidermal growth factor 2; HR: Hormone receptor; (m)BC: (Metastatic) breast cancer; NSCLC: Non-small cell lung cancer; MIBC: Muscle-invasive and metastatic bladder cancer; PDUFA: Prescription drug user fee act; PIK3CA-mut: Phosphatidylinositol 3-kinase, catalytic, alpha polypeptide mutated

Divarasib in 2L+ NSCLC: Superiority in PFS and OS vs. SoC

Potentially best-in-class KRAS G12C inhibitor; strong divarasib activity in 1L NSCLC in combination with pembro

Ph II KRASCENDO 170 efficacy[1]

2026 ASCO ANNUAL MEETING



- PD-L1 positive cohort[2], cORR of 73% (CR: 10%), and mPFS of 19.3m
- PD-L1 negative cohort, unconfirmed ORR of 70% (PFS immature with median follow-up of 3.4m)
- Manageable safety profile; most common TRAEs were low grade, manageable and reversible
- Ph III (KRASCENDO 2) in 1L NSCLC: divarasib + pembro vs chemo + pembro ongoing

Roche RAS-targeted pipeline

Molecule	Indication	Ph I	Ph II	Ph III	Status
divarasib	2L+ mNSCLC H2H vs sotorasib/adagrasib	KRASCENDO 1			✓
divarasib + pembro	1L mNSCLC		KRASCENDO 2		
divarasib	eNSCLC (adjuvant)			KRASCENDO 3	FPI exp. H2 2026
KRAS G12Di	Solid tumors				
panRASi	Solid tumors				
pan-KRASi	Solid tumors				

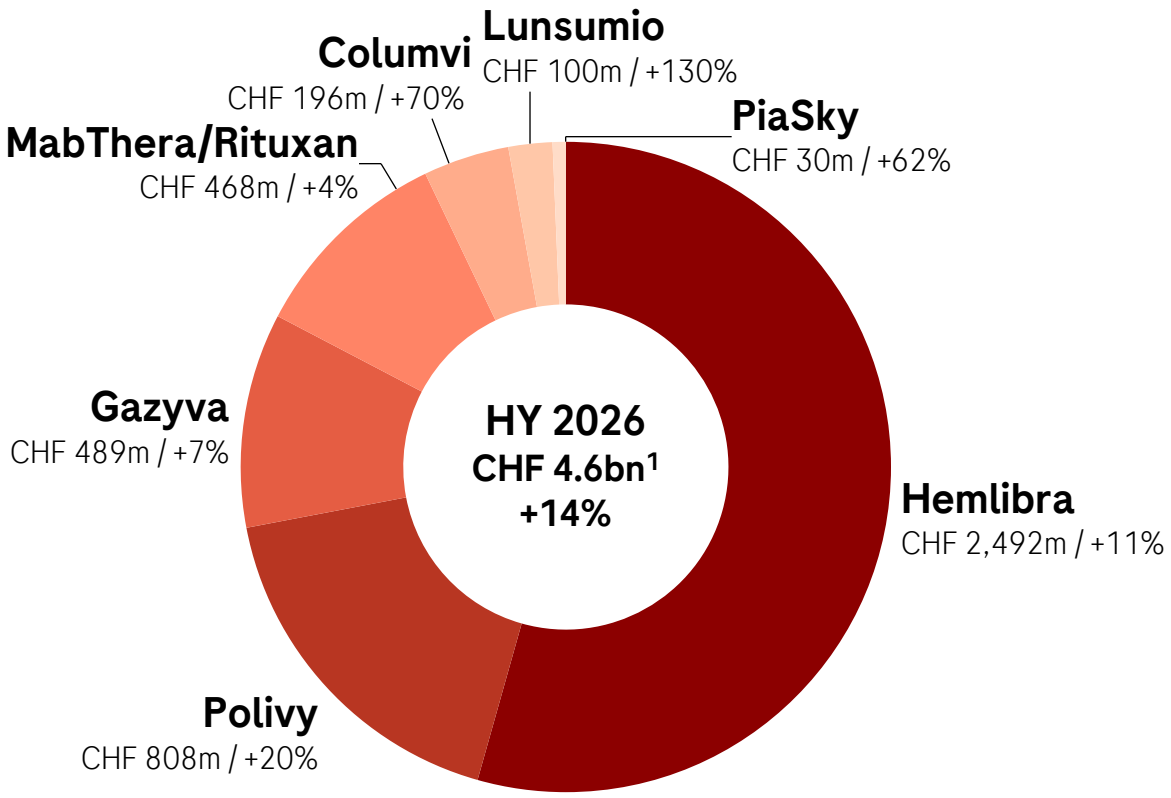
✓ Primary endpoint met

- Divarasib in 2L+ NSCLC: Superiority in PFS and OS vs. approved KRAS G12Ci demonstrated in Ph III H2H; OS significance reached at interim
- Broad Ph III program for divarasib in NSCLC covering adjuvant, 1L (chemo-free pembro combo), and 2L+ (H2H vs sotorasib/adagrasib)
- RG6620 (KRAS G12Di) potential for differentiation based on potency and selectivity vs other G12Di

1. Skoulidis et al., ASCO 2026; 2. All pts with a post-baseline assessment with tumor size reduction; *3 patients did not receive treatment, 1 patient received divarasib 200 mg; (c)ORR: (Confirmed) objective response rate; CR: Complete response; H2H: Head-to-head; (e)NSCLC: (Early) non-small cell lung cancer; (m)PFS: (Median) progression free survival; (TR)AE: (Treatment-related) adverse event

Polivy expanding strong global position in 1L DLBCL (IPI 0-5)

Lunsumio in 2L+ DLBCL (SUNMO) with US PDUFA set for 9th Feb



CHFm / YoY CER growth

Q2 update

- Hemlibra: Continued global growth; increasing adoption in non-inhibitor patients
- Polivy: Strong 1L DLBCL uptake with >120k pts treated globally
- Gazyva: Growth despite increasing competitive pressure in CLL and FL
- Columvi: Driven by 2L+ DLBCL
- Lunsumio: Strong 3L+ FL uptake; US filing in 2L+ DLBCL (SUNMO) achieved with PDUFA set for 9th Feb
- PiaSky in aHUS: Positive Ph III (COMMUTE) presented at ERA

Outlook 2026

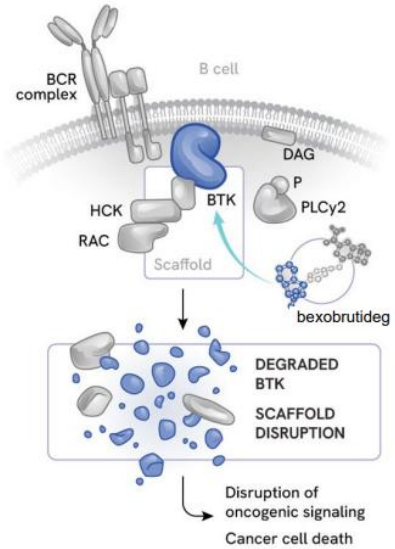
- PiaSky in aHUS: US/EU filing
- Ph III (CELESTIMO) Lunsumio + lenalidomide in 2L+ FL

1. Venclexta sales booked by AbbVie and therefore not included; aHUS: Atypical hemolytic uremic syndrome; CER: Constant exchange rates (avg. full year 2025); CLL: Chronic lymphocytic leukemia; DLBCL: Diffuse large B cell lymphoma; FL: Follicular lymphoma; PDUFA: Prescription drug user fee act

Bexobrutideg: Potential BIC BTK-targeted therapy

Broad development program in Hematology; ongoing expansion to Neurology and Immunology indications

Bexobrutideg (BTK degrader)



- Validated target for hematologic malignancies, as well as in Neurology and Immunology
- Oral, CNS-penetrant, highly potent and selective BTK degrader[1]
- Degradation removes all functions of BTK, unlike BTK inhibition, and overcomes BTKi resistance mutations[2-4]

Development program

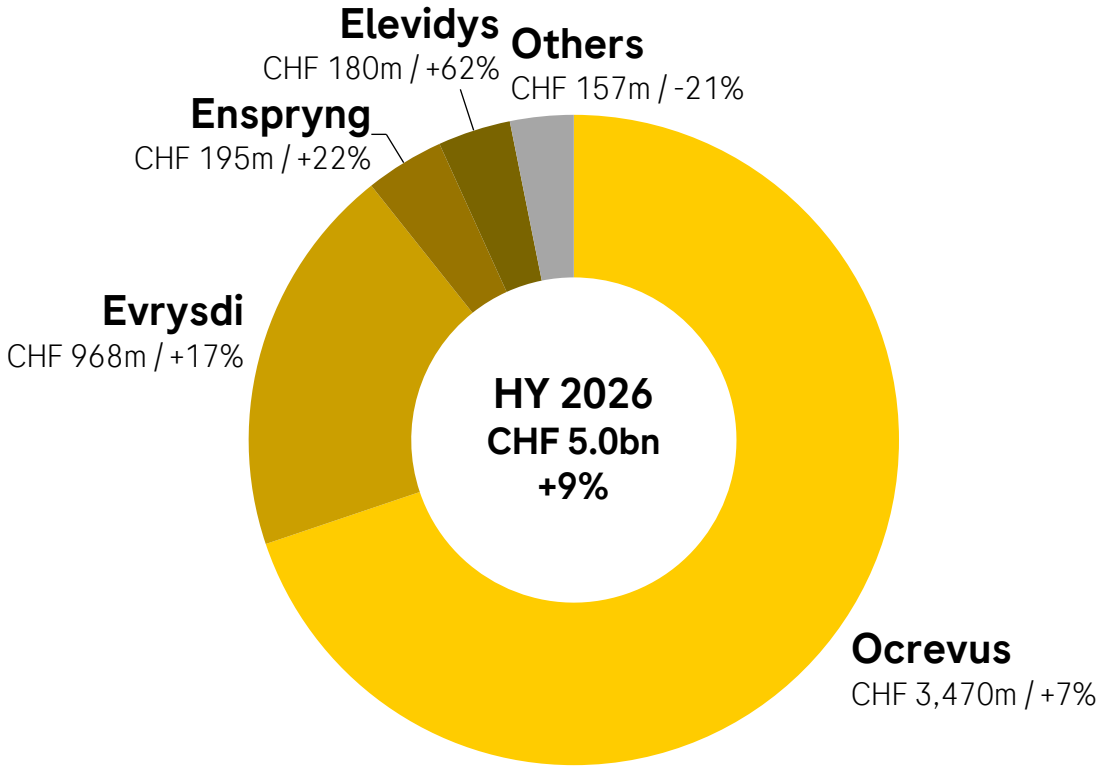
TA	Indication	Ph I	Ph II	Ph III	Status
Hematology	3L+ CLL	DAYBreak CLL-201			Ongoing
	2L+ CLL	DAYBreak CLL-306			Ongoing
	1L+ CLL / NHL	Basket combination trial*			Ongoing
	MCL				Ph III planned
	WM				Ph III planned
Neurology	MS				Ph II planned
Immunology	CSU				Ph II planned

- Broad development program in malignant hematology, incl. multiple lines of therapy and combinations in B-Cell malignancies; Ph III in 2L+ CLL and Ph I/II in 1L+ CLL have been initiated
- Planning to initiate Ph II trial in MS and CSU based on strong scientific rationale and preclinical findings for bexobrutideg[5,6]
- Evaluating initiation of additional Ph III trials, including combinations in CLL

1. Noviski. 8th TPD & Induced Proximity Summit, 2025; 2. Noviski et al. 20th Biennial International Workshop on CLL, 2023; 3. Hansen. TPD Basel, 2023; 4. Montoya et al. Science 2024; 383; eadi5798; 5. Noviski et al. ACR Convergence, 2024; 6. Noviski et al. SID Annual Meeting, 2026; *Ph Ib/II trial; BTK: Bruton's tyrosine kinase; CLL: Chronic lymphocytic leukemia; CNS: Central nervous system; CSU: Chronic spontaneous urticaria; NHL: Non-Hodgkin's Lymphoma; MCL: Mantle cell lymphoma; MS: Multiple sclerosis; WM: Waldenström macroglobulinemia; In partnership with Nurix Therapeutics

Ocrevus: Global growth driven by SC

Ocrevus Zunovo is the fastest growing aCD20 MS treatment in the US



CHFm / YoY CER growth

Q2 update

- Ocrevus: ~44,000 patients on Ocrevus SC globally with significant momentum vs. Q1
  - US: ~60% of SC volume coming from community practices
- Evrysdi: Expanding strong position in SMA
- Trontinemab in AD: Updates shared at AAIC
  - New long-term safety, amyloid removal and biomarker data from Ph Ib/IIa (Brainshuttle™ AD) OLE
  - Ph III (PrevenTRON) study design in preclinical AD

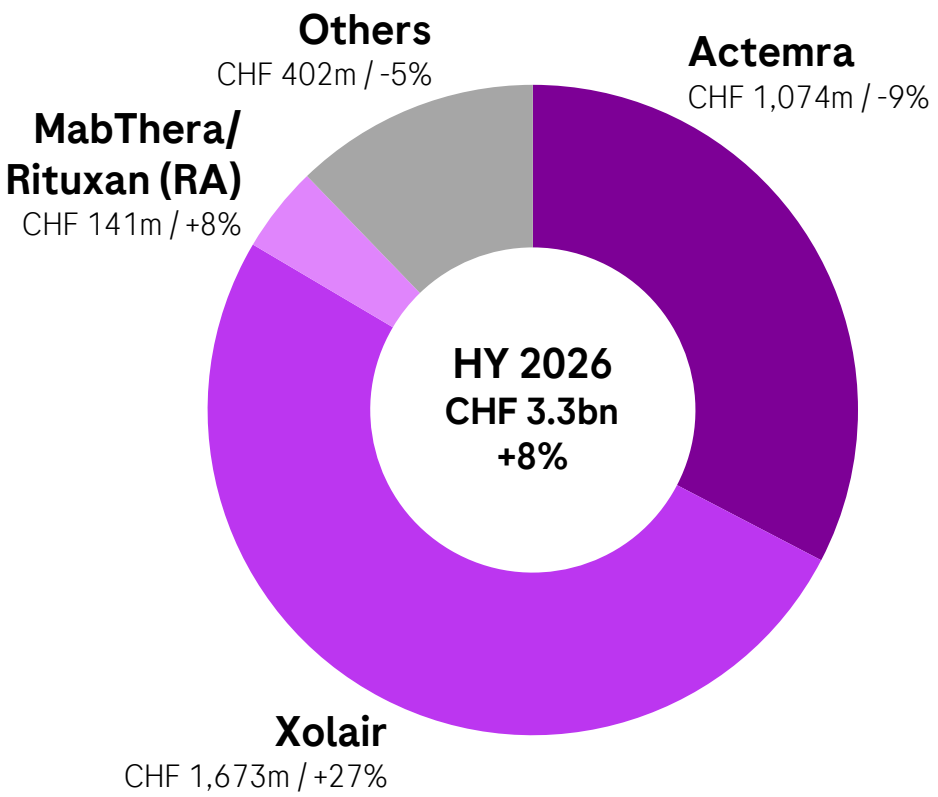
Outlook 2026

- Fenebrutinib in RMS/PPMS: US/EU filing
- Enspryng in MOGAD: US/EU filing

CER: Constant exchange rates (avg. full year 2025); AD: Alzheimer's disease; MOGAD: Myelin oligodendrocyte glycoprotein antibody-associated disease; (R/PP)MS: (Relapsing/Primary progressive) multiple sclerosis; SC: Subcutaneous; OLE: Open-label extension; SMA: Spinal muscular atrophy

Xolair driving overall Immunology growth

Gazyva: US & EU filing for MN achieved with PDUFA set for 15th Nov; US PDUFA in INS set for 27th Sep



CHFm / YoY CER growth

Q2 update

- Xolair: Growth driven by food allergy
  - Food allergy: >130,000 patients treated since launch
  - Biosimilar launch expected H2 2026
- Actemra: Biosimilar impact in US, EU and International
- Gazyva:
  - INS: US PDUFA set for 27th Sep & EU filing achieved
  - MN: US filing achieved/PDUFA set for 15th Nov & EU filing achieved; FDA BTD granted; Ph III (MAJESTY) data presented at ERA

Outlook 2026

- Gazyva in SLE: US approval (PDUFA 4th Dec)
- Gazyva in INS: US approval (PDUFA 27th Sep)
- Gazyva in MN: US approval (PDUFA 15th Nov)
- Ph III (IMAgINATION) sefaxersen in IgAN interim analysis

CER: Constant exchange rates (avg. full year 2025); BTD: Breakthrough therapy designation; IgAN: IgA nephropathy; INS: Childhood onset idiopathic nephrotic syndrome (also known as pediatric nephrotic syndrome (PNS); MN: Membranous nephropathy; PDUFA: Prescription drug user fee act; SLE: Systemic lupus erythematosus

Gazyva: Potential to become first approved treatment for MN

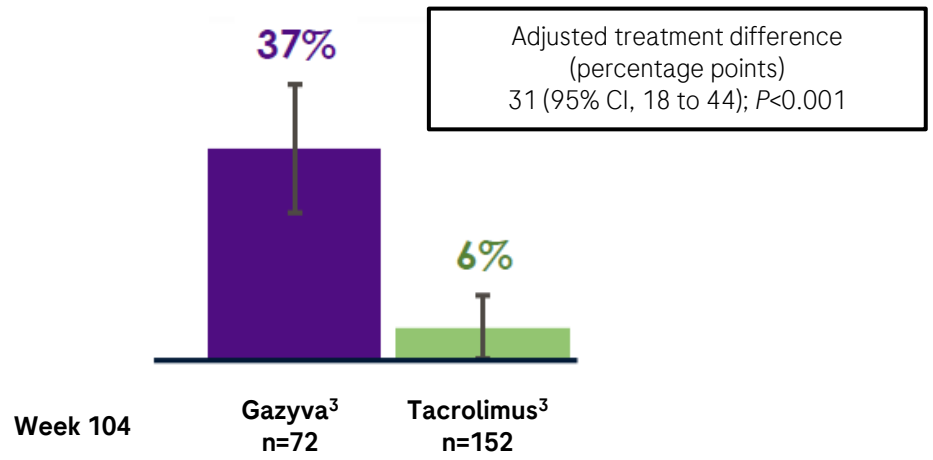
Gazyva demonstrated superiority over tacrolimus in CR at week 104; all Gazyva indications filed in US & EU

Ph III (MAJESTY) in MN results[1,2]

63rd ERA CONGRESS

The NEW ENGLAND JOURNAL of MEDICINE

Primary endpoint: CR[3] (UPCR ≤0.3 g/g and stable eGFR (≥85% of baseline))



- Primary endpoint met: 37% patients on Gazyva achieved complete remission[3] vs. 6% on tacrolimus at week 104
- Superior to tacrolimus on key secondary endpoints, incl. overall remission[4] (51% vs. 13%) at week 104
- Safety consistent with well-characterized Gazyva profile

Development program

Indication	Ph I	Ph II	Ph III	Status
LN			REGENCY	✓ Approved in US/EU
SLE			ALLEGORY	✓ Filed in US/EU
MN			MAJESTY	✓ Filed in US/EU
INS			INShore	✓ Filed in US/EU

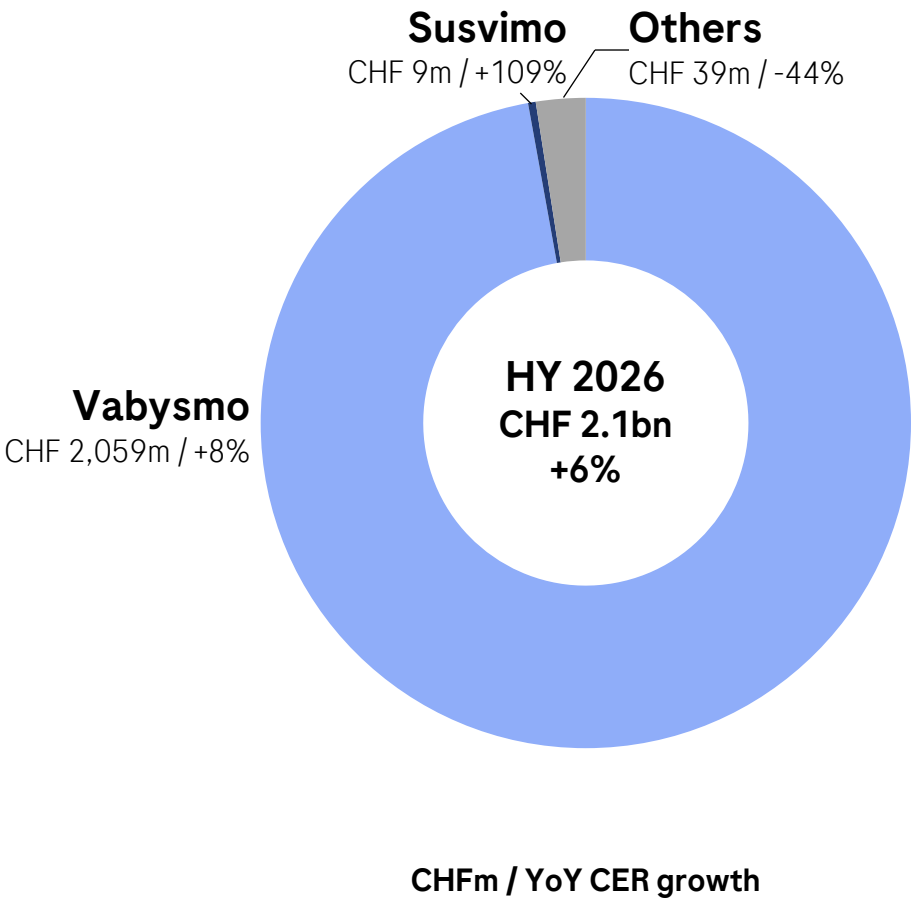
✓ Primary endpoint met

- LN launch in line with expectations
- US & EU filing in SLE achieved; PDUFA set for 4th Dec
- Positive MN results filed in US & EU; PDUFA set for 15th Nov; presented at ERA
- Positive INS results filed in US & EU; PDUFA set for 27th Sep
- FDA BTD and priority review granted for MN and INS

1. Fervenza FC et al., ERA Congress 2026; 2. Fervenza FC, et al. N Engl J Med. 2019; 381:36-46; 3. CR is defined as achievement of UPCR ≤0.3 g/g and eGFR ≤15% below baseline. eGFR is calculated using the Chronic Kidney Disease Epidemiology Collaboration equation. Patients experiencing intercurrent events of escape therapy, treatment failure or early study discontinuation were considered to be non-responders under the composite strategy; 4. Overall remission was defined as CR or PR at Week 104, wherein CR was defined as a UPCR of ≤0.3 g/g from a 24-hour urine collection and a stable eGFR defined as being no worse than 15% below the baseline value, calculated using the Chronic Kidney Disease Epidemiology Collaboration equation, and PR was defined as a ≥50% reduction in UPCR from baseline and ≤3.5 g/g but >0.3 g/g, with stable eGFR at Week 104. Data (UPCR or eGFR) were missing for 1 patient in the obinutuzumab group at Week 104 and were imputed using multiple imputation; BTD: Breakthrough therapy designation; CI: Confidence interval; CR: Complete remission; eGFR: Estimated glomerular filtration rate; INS: Childhood onset idiopathic nephrotic syndrome (also known as pediatric nephrotic syndrome (PNS); LN: Lupus nephritis; MN: Membranous nephropathy; PDUFA: Prescription drug user fee act; SLE: Systemic lupus erythematosus; UPCR: Urine protein-to-creatinine ratio

Vabysmo with continued global market share expansion

Enspryng in TED US filing and priority review achieved with PDUFA set for 15th Oct



Q2 update

- Vabysmo: Continued global growth and market share gains
  - US: >60% of patient starts are naïve
  - Positive Ph III (POYANG) in mCNV
- Enspryng in TED: US filing achieved; priority review granted

Outlook 2026

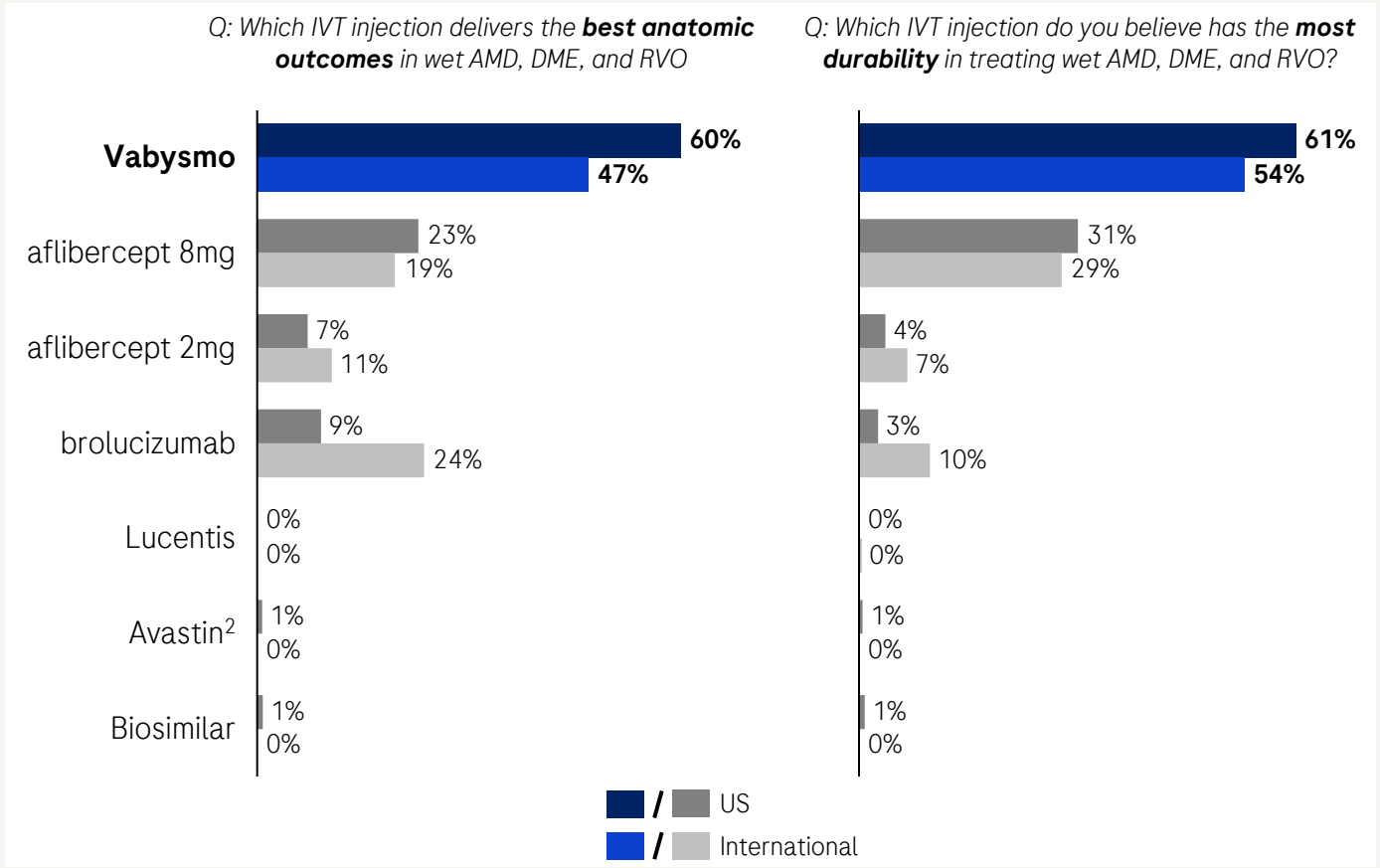
- Vamikibart in UME: US/EU filing
- Enspryng in TED: US approval (PDUFA 15th Oct)
- Susvimo in nAMD: EU approval

CER: Constant exchange rates (avg. full year 2025); mCNV: Myopic choroidal neovascularization; nAMD: Neovascular age-related macular degeneration; PDUFA: Prescription drug user fee act; RVO: Retinal vein occlusion; TED: Thyroid eye disease; UME: Uveitic macular edema

Vabysmo: Expecting continued global market share gains

Strong, consistent clinical and real-world data on anatomic outcomes and durability across indications

ASRS PAT survey: Anatomic outcomes and durability in AMD, DME & RVO[1]



Global growth outlook

US

- Branded market has stabilized
- Continued branded market share gains expected

EU

- Further share expansion across EU5 markets expected
- Strong volume growth to overcompensate pricing effects

ROW

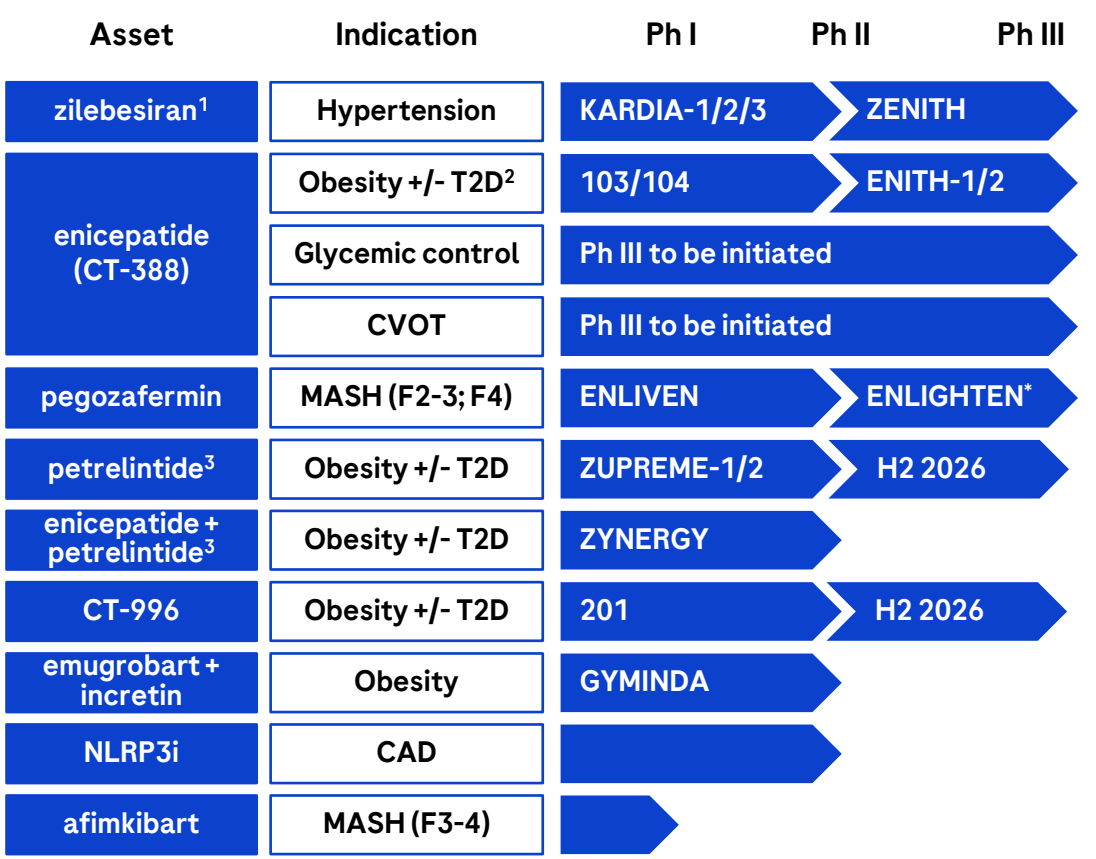
- Strong growth across key markets driven by market share expansion
- Significant growth potential remaining

1. Hahn P, ASRS PAT (Preferences and Trends) Membership Survey 2026, n=978 (anatomic outcomes), n=980 (durability), adapted visualization without changes to underlying data; 2. Avastin is not approved for use in the eye; ASRS: American society of retinal specialists; DME: Diabetic macular edema; nAMD: Neovascular age-related macular degeneration; Totals and subtotals may include differences due to rounding; Note: Survey data represents self-reported, subjective physician feedback and practice preferences. These observational findings do not constitute controlled clinical evidence and cannot be used to establish comparative clinical safety or efficacy.

CVRM pipeline progressing

Ph II data for enicepatide and petrelintide shared at ADA; pipeline progressing towards late-stage

Clinical development



Q2 update

- Enicepatide: Ph II (103) data in obesity presented at ADA
- Petrelintide: Ph II (ZUPREME-1) data in obesity presented at ADA
- Acmopatide (CT-868): Development will be discontinued as other T1D development program get prioritized

Outlook 2026

Results:

- Ph II (104) enicepatide in obesity with T2D
- Ph II CT-996 in obesity
- Ph II (ZUPREME-2) petrelintide in obesity with T2D
- Ph II (GYMINDA) emugrobart + tirzepatide in obesity

Pipeline progress:

- Enicepatide + petrelintide: Ph II (ZYNERGY) in obesity to be initiated
- Petrelintide: Ph III in obesity to be initiated
- CT-996: Ph III in obesity initiation planned

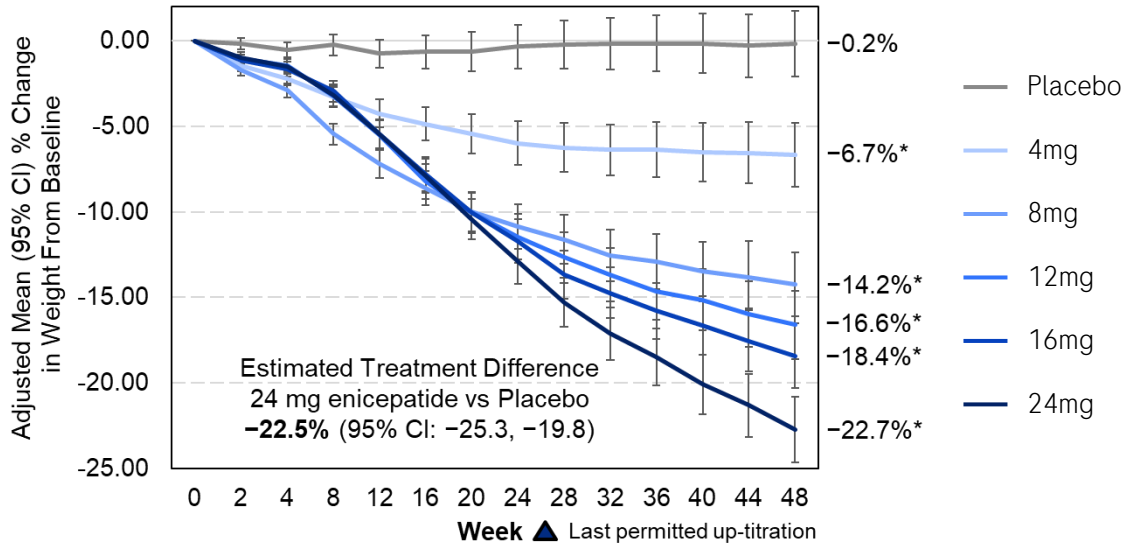
1. Zilebesiran in partnership with Alnylam; 2. Patients with obesity or overweight with at least one weight-related comorbidity including type 2 diabetes; 3. Petrelintide in partnership with Zealand Pharma; *Ph III program consisting of ENLIGHTEN fibrosis in MASH (F2-F3) and ENLIGHTEN cirrhosis in MASH (F4); CAD: Coronary arterial disease; CVOT: Cardiovascular outcomes trial; MASH: Metabolic dysfunction-associated steatohepatitis; T2D: Type-2 diabetes

Enicepatide: Potential for BIC weight loss

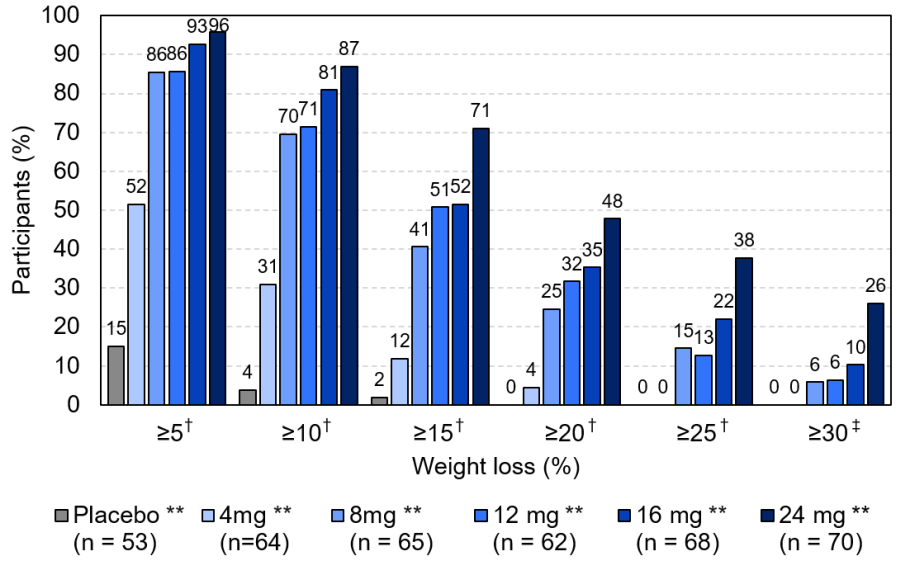
Broad Ph III development program in obesity, T2D and CVOT with additional trials under evaluation

Ph II (103) results in overweight/obesity[1]

Weight loss (efficacy estimand)



Weight reduction thresholds (at 48 weeks)



- No weight loss plateau reached at 48 weeks for 24mg dose, indicating potential to achieve additional weight loss with longer treatment; at 24mg 48% of patients achieved >20% weight loss, 38% > 25% weight loss and 26% > 30% weight loss
- Tolerability profile generally consistent with other incretin-based therapies at similar development stage
- Ph III (ENITH-1/2) in obesity ongoing; Ph III trials in glycemic control and CVOT to be initiated; Ph II (ZYNERGY) for enicepatide + petrelintide in obesity to be initiated

1. Lingvay I et al., ADA 2026, Poster-2813-LBA; *p ≤ 0.001 vs placebo; **The denominator to calculate the proportion is based on the number of participants who have reached week 48.
†Prespecified endpoint; BIC: Best in class; CI: Confidence interval; CVOT: Cardiovascular outcomes trial; T2D: Type 2 diabetes

2026: Key newsflow outlook*

Giredestrant + everolimus (post CDKi ER+/HER2- mBC) filed in US; 5 FDA priority reviews achieved

	Compound	Indication	Milestone		
Regulatory	**giredestrant**	adjuvant ER+/HER2- early BC	US/EU filing	✓	*(US)*
	giredestrant + everolimus	post CDKi ER+/HER2- mBC	EU filing/US approval		
	Gazyva	SLE	US/EU filing	✓	*(US&EU)*
	Gazyva	INS	US/EU filing	✓	*(US&EU)*
	Enspryng	MOGAD	US/EU filing		
	Enspryng	TED	US filing	✓	
	vamikibart	UME	US/EU filing		
	Susvimo	nAMD	EU approval		
	Susvimo	DME	EU filing		*2027*
	PiaSky	aHUS	US/EU filing		
	Lunsumio + Polivy	2L+ DLBCL	US approval		*2027*
Clinical results	**divarasib**	2L+ KRAS G12C+ NSCLC	Ph III KRASCENDO 1	✓	
	giredestrant + palbociclib	1L ER+/HER2- mBC	Ph III persevERA	✗	
	Itovebi + fulvestrant	post CDKi *PIK3CA*-mut HR+/HER2- mBC	Ph III INAVO121		
	Itovebi + Phesgo	1L *PIK3CA*-mut HER2+ mBC	Ph III INAVO122		
	Lunsumio + lenalidomide	2L+ FL	Ph III CELESTIMO		
	Enspryng	MOGAD	Ph III METEOROID	✓	
	fenebrutinib	RMS	Ph III FENhance 1/2	✓	
	Gazyva	MN	Ph III MAJESTY	✓	
	sefaxersen	IgAN	Ph III IMAgINATION		
	Vabysmo	mCNV	Ph III POYANG	✓	
	Evrysdi + emugrobart	SMA	Ph II MANATEE	✗	
	emugrobart	FSHD	Ph II MANOEUVRE	✗	
	emugrobart + tirzepatide	Obesity	Ph II GYMINDA		
	enicepatide (CT-388) (QW SC)	Obesity	Ph II	✓	
	acmopatide (CT-868) (QD SC)	T1D with Obesity	Ph II	✓	
	CT-996 (QD oral)	Obesity	Ph II		
	petrelintide	Obesity	Ph II	✓	

Additional 2026 newsflow: ✓ **Tecentriq** US approval in MIBC

*Outcome studies are event-driven: timelines may change



Diagnostics Division

Matt Sause

CEO Roche Diagnostics

HY 2026: Diagnostics sales

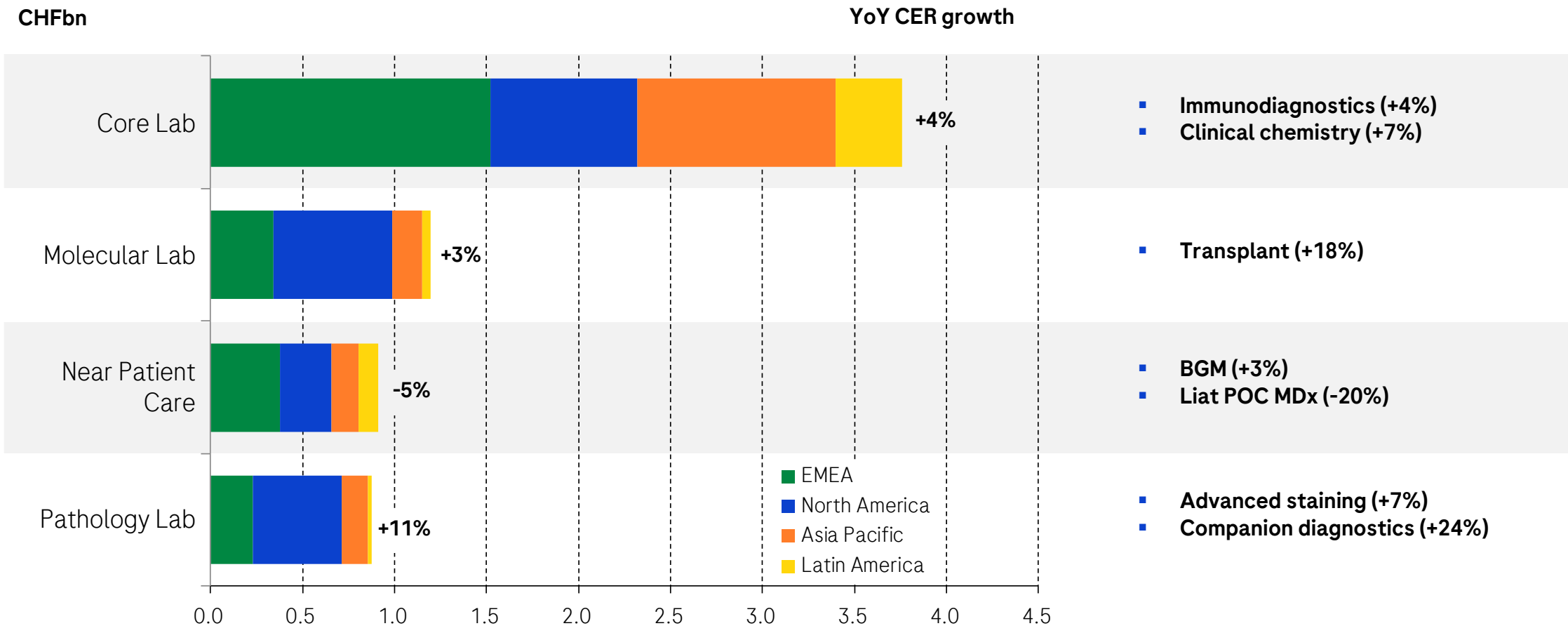
Diagnostics growing at +3%, impacted by China healthcare pricing reforms

	HY 2026 CHFm	HY 2025 CHFm	Change in % CHF	Change in % CER
Diagnostics Division	**6,735**	**6,959**	**-3**	**3**
Core Lab	3,756	3,839	-2	4
Molecular Lab	1,196	1,250	-4	3
Near Patient Care	909	1,018	-11	-5
Pathology Lab	874	852	3	11

CER: Constant exchange rates (avg. full year 2025)

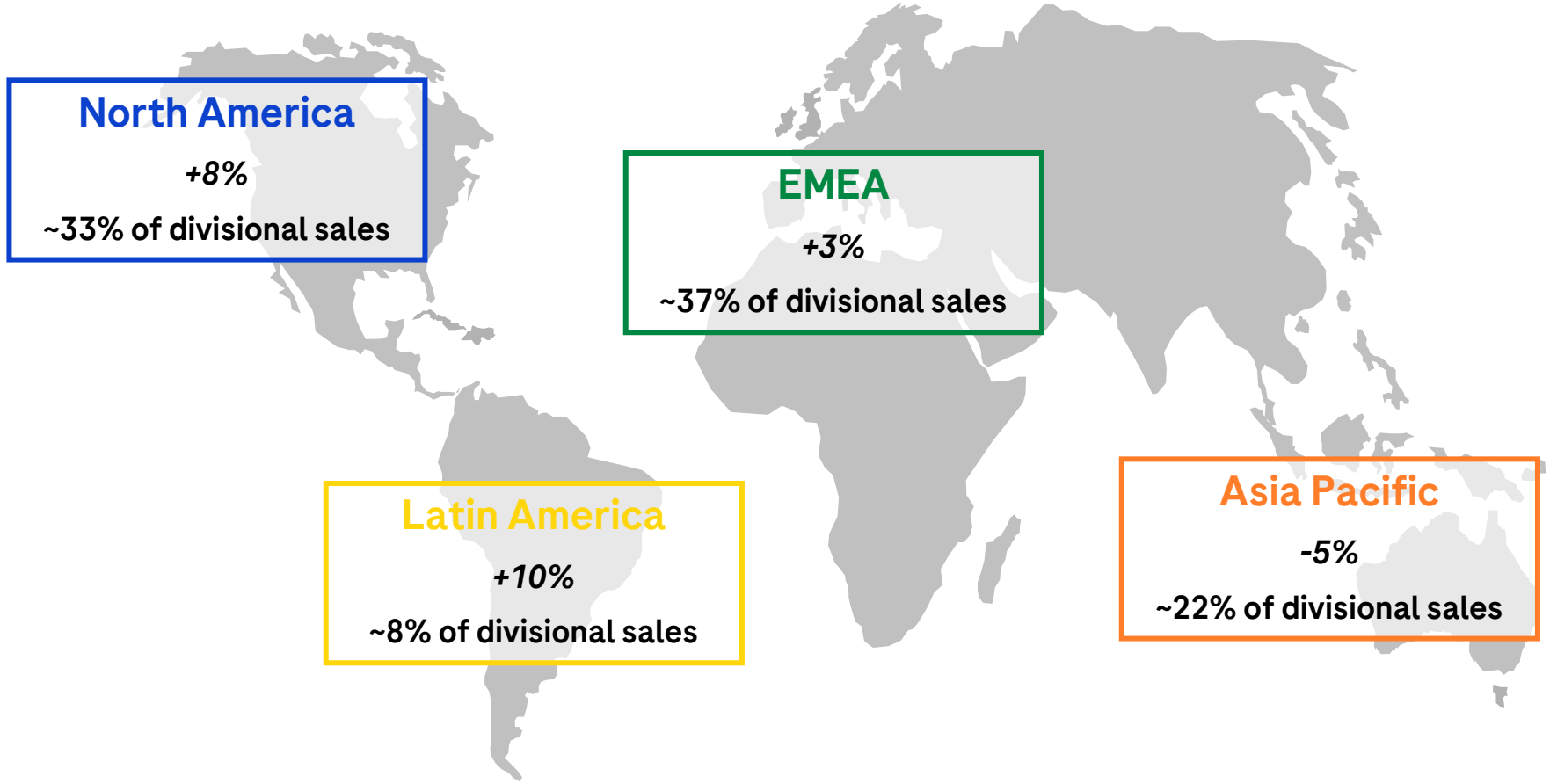
HY 2026: Diagnostics highlights

Diagnostics growing at +3%, impacted by China healthcare pricing reforms



BGM: Blood glucose monitoring; CER: Constant exchange rates (avg. full year 2025); EMEA: Europe, Middle East and Africa; POC MDx: Point of care molecular diagnostics

HY 2026: Diagnostics regional sales



Growth rates at constant exchange rates (avg. full year 2025); EMEA: Europe, Middle East and Africa

HY 2026: Diagnostics core operating profit

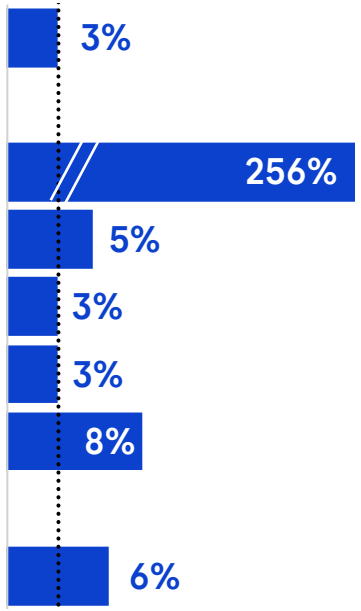
Core operating profit higher by +6%, driven by higher sales and other revenue

	2026	
	CHFm	Abs. CERm
Sales	**6,735**	**+228**
Other revenue	123	+90
Cost of sales	-3,442	-173
R&D	-870	-27
SG&A	-1,388	-43
OOI&E	22	+2
Core operating profit	**1,180**	**+77**
Core OP as % of sales	*17.5%*	
At CER		*18.0%*
		(HY 2025: 17.4%)

Includes shift of CHF ~24m (CER) to Corporate

2026 vs. 2025

CER growth



-6% in CHF

CER: Constant exchange rates (avg. full year 2025); OOI&E: Other operating income & expense; OP: Operating profit; R&D: Research & development; SG&A: Selling, general & administration

AXELIOS 1 Sequencing platform launch

AXELIOS 1 delivers world-record speed, high accuracy, and multi-modal flexibility

Performance[1]

SBX-D	SBX-duplex (SBX-D) output (concordant duplex bases per 4 hr run)	**≥ 1.8 Tb**
	Mean insert length[2]	**~ 230 to 260 bp**
	Accuracy[3] (concordant duplex bases)	**~ Q38**
SBX-S	SBX-simplex output	**~40bn total reads (~175 bp insert length)**
	Insert length	**Up to 1500 bp[4]**
Maximum run time[5]	AXELIOS 1 Sequencing Instrument	**1 run: ~5.5 hr 4 runs (queued): ~22 hr**

Key partners

Hartwig Medical Foundation

wellcome sanger institute

Olink

BROAD INSTITUTE

10x GENOMICS

Market opportunity and differentiation

- The sequencing market was valued at USD 7.3bn in 2025[6]
- Demonstrated high performance in ultra-sensitive applications such as MRD detection, tumor/normal analysis, and WGS using SBX-duplex mode
- Efficient high-throughput single-cell and spatial omics for large-scale, read intensive research with SBX-simplex mode

AXELIOS 1 is for research use only; 1. Performance metrics, output, mean read length, quality score, etc. can vary based on sample type and quality, library prep conditions and loading concentration, and other experimental conditions 2. Includes post-consensus adapter-trimmed full length and partial duplex reads 3. Based on empirical accuracy calculated using BWA aligned bam files for high confidence regions of Genome in a Bottle (GIAB) samples; 4. SBX technology is capable of generating longer reads of up to ~1500bp (with a distribution from ~200bp to ~1500 bp) under appropriate sample and library prep conditions 5. Max run time for sequencing instrument includes pre-sequencing setup, sequencing time and post-run cleanup; 6. DeciBio. (2025). 2025 NGS Manufacturer Market Size, Growth and Trends (2022–2028); bp: Base pairs; MRD: Minimal residual disease; SBX-D: SBX-duplex; SBX-S; SBX-simplex; Tb: Terabases; WGS: Whole genome sequencing

PathAI acquistion[1]

Expanding our pathology portfolio with the best-in-class digital pathology solution

Roche Diagnostics oncology ecosystem[2]



PathAI overview

- PathAI is an AI-powered precision pathology company
- Its core offering includes a best-in-class Image Management System alongside a broad suite of diagnostic algorithms

Value to Roche Diagnostics

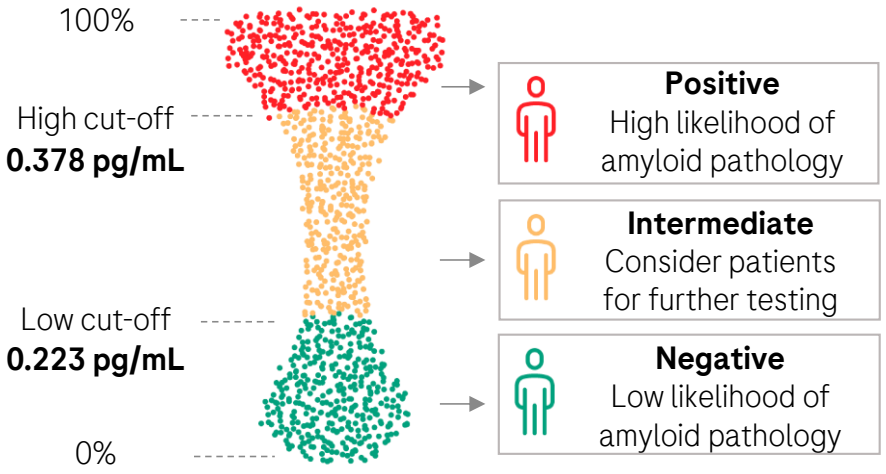
- PathAI Image Management Solution is complementary to Roche IHC and H&E, creating a highly differentiated solution
- AI-powered insights, including FDA approved algorithms, offer additional value to pathologists
- Roche's CDx leadership and PathAI's AI capabilities improve leading position with biopharma partners
- AI solutions enhance our Roche Pharma business by supporting clinical trials and introducing new biomarkers

Note: 1. Pending deal closure; 2. Roche ecosystem refers to internal assets, acquisitions, and partnerships; 3. AXELIOS is for research use only; CDx: Companion diagnostics; H&E: Hematoxylin and eosin; IHC: Immunochemistry; MRD: Minimal residual disease

Elecsys® pTau217

First blood-based biomarker to rule in and rule out Alzheimer's pathology in primary care

Elecsys® pTau217



PPV: 78.3% and NPV: 98.6% in primary care
PPV and NPV>90% in secondary care

Market needs and opportunity

- Alzheimer's is the leading cause of dementia, a condition projected to affect 80m people by 2030[1]
- Diagnosis takes three years after symptom onset and standard testing methods are invasive or not widely accessible[1]

Differentiation

- Enables rapid, minimally invasive and timely diagnosis via blood-based testing in primary care settings
- Validated in a real world cohort across settings and clinical stages demonstrating high diagnostics accuracy
- Fulfills CEOi consensus criteria for blood based biomarkers for rapid and safe triage and confirmatory diagnosis of patients

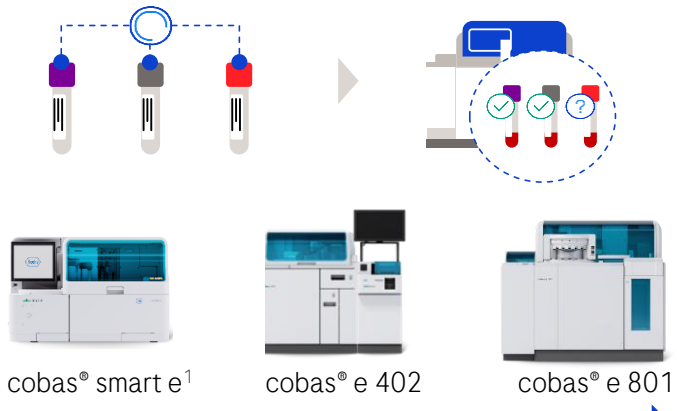
1. ADI's annual World Alzheimer's Report (2022); CEOi: Global CEO Initiative on Alzheimer's Disease; PPV: Positive predictive value; pTau: Phosphorylated tau; NPV: Negative predictive value

Elecsys® IGRA TB

Entering the attractive latent TB segment and strengthening Roche's Infectious Disease offering

46TH ANNUAL CONGRESS

Elecsys® IGRA TB



Performance		
Mixed risk cohort[3]	**Low risk cohort**[4]	**Active TB cohort**[5]
PPA: 91.12% NPA: 94.57%	Specificity: 95.32%	Relative sensitivity:[6] 100%

Market needs and opportunity

- ~25% of the global population may have been infected with the bacteria that cause TB[7]
- Expanding access to diagnostics for latent TB infection is essential to breaking the cycle of progression and transmission

Differentiation

- On par performance with standard of care and high intermediate precision and repeatability (CV < 2%)
- Improved workflow vs. standard of care, with faster turnaround and scalable throughput on the large Elecsys installed base
- Enables future automation of the pre-analytical steps with cobas ultra solutions[8]

1. In development; 2. Scalable to up to 4 e801 analytical units (400 IGRA samples/hour) 3. Intended use population: individuals seeking TB infection testing in low- and high-TB burden settings in the European and Western Pacific regions (n=1852); 4. Apparently healthy individuals without known risk factors for TB infection, living in a low-burden setting in the European region (n=449); 5. Not intended use population: confirmed active TB patients in low- and high-TB burden settings in European, Americas, and African region (n=78); WHO guidelines recommend against testing in this population, cohort was necessary to estimate sensitivity due to lack of a gold standard; 6. Relative to standard-of-care/established IGRA methods 7. WHO tuberculosis Fact Sheet November 2025; 8. Technical feasibility, availability beyond 2028; CV: Coefficient of variation; NPA: Positive predictive agreement; PPA: Positive predictive agreement; TB: Tuberculosis

Diagnostics key launches 2026

	Area	Product	Description	Market	Status
Systems	**Molecular Lab**	AXELIOS 1 Sequencing Solution	Novel high-throughput solution for ultra-rapid, scalable, and flexible sequencing powered by Sequencing by Expansion (SBX) technology	Global	✓
Tests	**Core Lab**	Elecsys® pTau 217	Non-invasive blood-based biomarker to enable earlier detection, rule in and rule out Alzheimer's amyloid pathology	CE	✓
		Elecsys® NfL	Non-invasive blood-based biomarker to enable monitoring of disease activity in multiple sclerosis	CE	✓
		Elecsys® ApoE4p	Non-invasive blood-based biomarker to identify ApoE4 carrier, the strongest genetic risk factor for the development of Alzheimer's disease	CE	✓
		Elecsys® IGRA TB	Interferon-Gamma Release Assay (IGRA) to detect latent tuberculosis (TB) infection	CE	✓
	Molecular Lab	cobas® MPX-E	Molecular PCR test for the detection of HIV, HCV, HBV, and HEV in one assay for Donor Screening	CE	✓
		cobas® HDV	Molecular PCR test for aid in diagnosis of initial Hepatitis D virus infection and therapy monitoring	CE	✓
		eplex gastrointestinal panel	Multiplex Molecular PCR assay for the comprehensive detection of 19 potential pathogens associated with gastrointestinal infections	CE	
	Near Patient Care	cobas® liat lesion panel EUA	Rapid test to enable accurate detection and differential diagnosis of patients presenting with cutaneous and mucocutaneous lesions/ulcers to enable timely treatment and management. Supporting mpox health emergency	US EUA	
	Pathology Lab	VENTANA® BAP1	Highly specific marker for differentiating mesothelioma from benign mesothelial proliferation	CE	
Digital solutions	**Healthcare Insights**	LungFlag algorithm	Machine learning algorithm that identifies patients at high risk for lung cancer	CE	

AXELIOS 1 is for research use only; ApoE4p: Apolipoprotein E4 protein; BAP1: BRCA1 Associated Protein-1; HCV: Hepatitis C Virus; HBV: Hepatitis B Virus; HDV: Hepatitis D Virus; HEV: Hepatitis E Virus; MPX-E: Multiplex HEV; NfL: Neurofilament light chain; PCR: Polymerase Chain Reaction; SBX: Sequencing by Expansion

IR events in 2026

Additional events driven by readouts

Neurology Update 22 Apr

- MS franchise update at AAN
- Focus on fenebrutinib (FENhance 1&2) data in RMS

Diagnostics Day 12 May

- Group and Diagnostics division strategy updates
- Launch updates for mass spectrometry, SBX sequencing, and CGM
- Foundation Medicine strategy and highlights
- Portfolio updates on Core Lab, Molecular Lab, Near Patient Care, and Pathology Lab

Oncology Update 2 Jun

- Oncology franchise update at ASCO
- Focus on giredestrant key data (lidERA, persevERA, evERA) and clinical development program update
- Additional details on the oncology pipeline

CVRM Update 8 Jun

- Obesity franchise update at ADA
- Focus on obesity key data (enicepatide Ph II, petrelintide Ph II) and clinical development program update

Pharma Day 28 Sep

- Update on Pharma strategy and business performance
- Update on R&D excellence
- Update on AI in R&D
- Deep-dive into the current product portfolio
- Building blocks for future growth: Late-stage portfolio update

Neurology Update ✓ Virtual Mon, 9 Feb 16:30-17:30 CET	**Immunology Update** ✓ Virtual Mon, 9 Mar 17:00-18:00 CET	**Neurology Update** ✓ Virtual Wed, 22 Apr 17:00-18:00 CEST	**Diagnostics Day** ✓ London & virtual Tue, 12 May 13:00-16:20 BST	**Oncology Update** ✓ Chicago & virtual Tue, 2 Jun 14:00-15:30 CDT	**CVRM Update** ✓ Virtual Mon, 8 Jun 17:00-18:30 CEST	**Pharma Day** London & virtual Mon, 28 Sep 09:00-15:00 BST

AD: Alzheimer's disease; CGM: Continuous glucose monitor; CVRM: Cardiovascular, renal and metabolism; RMS: Relapsing multiple sclerosis; SBX: Sequencing by expansion

Doing now what patients need next

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

Pharma sales appendix

Diagnostics sales appendix

Foreign exchange rates information

Changes to the development pipeline

Q2 2026 update

New to phase I

4 NMEs:

RG6697 NME - DME
RG6852 NME - mCRPC
RG6733 NME - MM
RG6441 NME - DR

New to phase II

3 NMEs:

RG6596 HER2 TKI (ZN-1041) – 2L+ HER2+ mBC
RG6418 selnoflast (NLRP3i) – coronary artery disease
CHU anti-HLA-DQ2.5 x gluten peptides antibody - celiac disease

2 AIs:

RG7828 Lunsumio - SLE
RG6114 Itovebi + ribociclib + fulvestrant – 1L HR+ Chr8P-loss PIK3CA-wt mBC

New to phase III

3 NMEs:

RG6512 zemocimig (NXT007) – hemophilia A
RG6160 cevostamab – 2-4L r/r MM
RGxxxx[+] bexobrutideg – r/r CLL

New to registration

2 AIs:

RG7159 Gazyva - membranous nephropathy
RG7446 Tecentriq – stage III dMMR/MSI-H colon cancer

1 AI (US):

RG6168 Enspryng - TED

Removed from phase I

4 NMEs:

RG6344 mosperafenib (BRAFi) - solid tumors
RG6382 CD19 x CD3 - SLE
RG6496 HTT SNP ASO - Huntington's disease
RG6468 NME – solid tumors

1 AI:

RG6418 selnoflast (NLRP3i) - inflammation

Removed from phase II

3 NMEs:

RG6351 anti-Tie2 agonist - DME
RG6641 acmopatide – T1D
RG6042 tominersen – Huntington's disease

Removed from phase III

Approvals

1 AI (US):

RG7446 Tecentriq - ctDNA-pos high-risk MIBC

Status as of July 23, 2026

[+]RGxxxx number to be confirmed

Roche Group development pipeline

Phase I (37 NMEs + 6 AIs)

RG6026	Columvi monotherapy + combos	heme tumors
RG6171	giredestrant monotherapy + combos	solid tumors
RG6330	divarasib monotherapy + combos	solid tumors
RG6411	enzelkitug (anti-CCR8)	solid tumors
RG6505	PanRAS inhibitor	solid tumors
RG6537	AR degrader	mCRPC
RG6538	P-BCMA-ALLO1	r/r multiple myeloma
RG6540	P-CD19 x CD20 - ALLO1	heme tumors
RG6620	KRAS G12D inhibitor	solid tumors
RG6648[1]	cMET ADC	solid tumors
RG6733	-	MM
RG6735	-	heme tumors
RG6794	CDK4/2i	HR+ HER2- BC
RG6795	CDK4i	HR+ HER2- BC
RG6810[2]	DLL3 ADC	SCLC
RG6852	-	mCRPC
RG6919[1]	B7H3 ADC	solid tumors
RG6975[3]	CDH17 ADC	CRC
RG7828	Lunsumio monotherapy + combos	heme tumors
CHU	clesitamig (DLL3 trispecific)	solid tumors
CHU	codrituzumab	HCC
CHU	MINT91	solid tumors
CHU	anti-CTLA-4 switch antibody	solid tumors
CHU	pan-KRAS inhibitor (AUBE00)	solid tumors
RG6421	TMEM16A potentiator	Muco-obstructive respiratory disease
RG6498	-	IBD
RG6981[4]	Il33:TSLP	COPD/Asthma
RG7828	Lunsumio	SLE
CHU	anti-C1s recycling antibody	immunology
RG6622	-	coronary artery disease
RG6631	afimkibart (anti-TL1A)	MASH
RG6035	Brainshuttle™ CD20	multiple sclerosis
RG6434	NLRP3i	neurodegenerative disorders
RG6627[5]	APOE ASO	Alzheimer's disease
RG6662	HTT miRNA GT	Huntington's Disease
RG6209	VEGF x IL6 DutaFab	DME
RG6327	-	geographic atrophy
RG6441	-	DR
RG6697	-	DME
RG6858	-	retinal diseases
RG6913	-	DME
RG6006	zosurabalpin	bacterial infections
CHU	REVN24	acute diseases

Phase II (12 NMEs + 6 AIs)

RG6114	Itovebi + enzalutamide	CRPC
RG6114	Itovebi + ribociclib + fulvestrant 1L HR+ Chr8P-loss PIK3CA-wt mBC	
RG6180	autogene cevumeran	solid tumors
RG6596	HER2 TKI (ZN-1041)	HER2+ mBC
RG6797	dirloctocogene samoparvovec (SPK-8011QQ)	hemophilia A
RG6631	afimkibart (anti-TL1A)	atopic dermatitis
RG6631	afimkibart (anti-TL1A)	rheumatoid arthritis
RG6730[6]	p40xTL1A bispecific antibody	UC
RG7828	Lunsumio	SLE
CHU	anti-HLA-DQ2.5 x gluten peptides antibody	celiac disease
RG6237	emugrobart (GYM 329)	obesity
RG6418	selnoflast (NLRP3i)	coronary artery disease
RG6652	GLP-1 RA (CT-996)	obesity/ T2D glycemic control
RG6849[7]	petrelintide	obesity +/- T2D
RG6182	MAGL inhibitor	multiple sclerosis
RG6289	nivegacetor (γ-secretase modulator)	Alzheimer's
RG6356[8]	Elevidys	DMD, <4 years
RG6501	OpRegen	geographic atrophy

RG-No - Roche/Genentech; CHU - Chugai managed; [1]MediLink managed; [2]Innovent managed; [3]In collaboration with Hansoh Pharma, RG Nr TBD; [4]Qyuns Therapeutics managed; [5]In collaboration with IONIS; [6]In collaboration with Pfizer; [7]Zealand Pharma managed; [8]US rights with Sarepta; *also developed in neurology

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology

Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

Status as of July 23, 2026

Roche Group development pipeline

Phase III (14 NMEs + 21 AIs)

RG3502	Kadcyla + T	HER-2+ eBC high-risk
RG6013	Hemlibra	Type 3 VWD
RG6026	Columvi + Polivy + R-CHP	1L DLBCL
	Columvi	r/r MCL
RG6107	PiaSky	aHUS
RG6114	Itovebi + fulvestrant	post CDKi HR+ PIK3CA-mut. BC
	Itovebi + Phesgo	1L HER2+ PIK3CA-mut. mBC
	Itovebi + CDK4/6i + letrozole	1L ES PIK3CA-mut. HR+ HER2- advanced BC
RG6160	cevostamab	r/r MM
RG6171	giredestrant + Phesgo	1L ER+/HER2+ BC
	giredestrant + CDK4/6i	1L ET resistant ER+/HER2- BC
RG6330	divarasib	2L NSCLC
RG6330	divarasib + pembrolizumab	1L NSCLC
RG6512	zemocimig (NXT007)	hemophilia A
RGxxxx+	bexobrutideg	r/r CLL
RG7828	Lunsumio + lenalidomide	2L+ FL
RG7853	Alecensa	LA stage III NSCLC
RG6149	astegolimab	COPD
RG6299[2]	sefaxersen (ASO factor B)	IgA nephropathy
RG6631	afimkibart (anti-TL1A)	ulcerative colitis
	afimkibart (anti-TL1A)	Crohn's disease
RG6615[1]	zilebesiran	hypertension
RG6640	enicepatide (CT-388)	obesity +/- T2D
RG6881	pegozafermin	MASH with fibrosis stage F2 or F3
	pegozafermin	MASH with fibrosis stage F4
RG6102	trontinemab	early Alzheimer's
RG6168	Enspryng	MOGAD
	Enspryng	autoimmune encephalitis
RG6356[3]	Elevidys	DMD, 8 years and older
RG7845	fenebrutinib	RMS
	fenebrutinib	PPMS
RG7935	prasinezumab	Parkinson´s
RG6179	vamikibart	UME
RG6321	Susvimo	wAMD, 36-week
RG7716	Vabysmo	CNV

Registration US & EU (2 NMEs + 7 AI)

RG6171	giredestrant + everolimus†	post-CDK4/6 ER+/HER2- BC
	giredestrant†	ER+ BC adj
RG7446	Tecentriq	stage III dMMR/MSI-H colon cancer
RG7828	Lunsumio + Polivy†	2L+ DLBCL
RG7159	Gazyva	systemic lupus erythematosus
	Gazyva	childhood onset idiopathic nephrotic syndrome*
	Gazyva	membranous nephropathy
RG6168	Enspryng†	TED
RG6321	Susvimo††	wAMD

†Filed in US
††Filed in EU, approved in US

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

Status as of July 23, 2026

+RGxxxx number to be confirmed; [1]Alnylam Pharmaceuticals managed; [2]In collaboration with IONIS; [3]US rights with Sarepta; T:Tecentriq, *also known as pediatric nephrotic syndrome (PNS)

Expected regulatory submissions*

New Molecular Entities: Lead and additional indications

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

*Filing timelines reflect the anticipated filing of a potential indication; projects shown are in phase II and phase III
✓ Indicates submission to health authorities has occurred
Unless stated otherwise submissions are planned to occur in US and EU
*Filed in US; +RGxxxx number to be confirmed; [1]In collaboration with IONIS ; [2]In collaboration with Pfizer; [3]Alnylam Pharmaceuticals managed; [4]Zealand Pharma managed

2026

RG6171	**giredestrant ✓*** ER+ BC adj
RG6330	**divarasib** 2L NSCLC
RG7845	**fenebrutinib** RMS &PPMS
RG6179	**vamikibart** UME

2027

RG6171	**giredestrant + Phesgo** 1L ER+/HER2+ BC
RG6171	**giredestrant + CDK4/6i** 1L ET resistant ER+/HER2- BC
RG6299[1]	**sefaxersen (ASO factor B)** IgA nephropathy
RG6631	**afimkibart (anti-TL1A)** ulcerative colitis
RG6321	**Susvimo** wAMD, 36-week refill
RG6321	**Susvimo** DME (EU)

2028

RG6512	**zemocimig (NXT007)** Hemophilia A
RG6102	**trontinemab** early Alzheimer's
RG6640	**enicepatide (CT-388)** obesity +/- T2D
RG6881	**pegozafermin** MASH with fibrosis stage F2 or F3
RG6881	**pegozafermin** MASH with fibrosis stage F4

2029 and beyond

RG6160	**cevostamab** 2-4L r/r MM
RG6180	**autogene cevumeran** solid tumors
RG6330	**divarasib + pembrolizumab** 1L NSCLC
RG6596	**HER2 TKI (ZN-1041)** 2L+ HER2+ BC
RG6797	**dirloctocogene samoparvovec** Hemophilia A
RGxxxx+	**bexobrutideg** r/r CLL
RG6631	**afimkibart (anti-TL1A)** Crohn's disease
RG6631	**afimkibart (anti-TL1A)** atopic dermatitis
RG6631	**afimkibart (anti-TL1A)** rheumatoid arthritis
CHU	**anti-HLA-DQ2.5 x gluten peptides antibody** celiac disease
RG6730[2]	**p40xTL1A bispecific antibody** UC
RG6182	**MAGL inhibitor** multiple sclerosis
RG6356	**Elevidys** DMD, <4 years
RG6356	**Elevidys** DMD, 8 yrs and older
RG6289	**nivegacetor** (γ-secretase modulator) Alzheimer's
RG7935	**prasinezumab** Parkinson's
RG6501	**OpRegen** geographic atrophy
RG6237	**emugrobart (GYM 329)** obesity
RG6418	**selnoflast (NLRP3i)** coronary artery disease
RG6615[3]	**zilebesiran** hypertension
RG6652	**GLP-1 RA (CT-996)** obesity/ T2D glycemic control
RG6849[4]	**petrelintide** obesity +/- T2D

Status as of July 23, 2026

Expected regulatory submissions*

Marketed products: Additional indications

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

*Filing timelines reflect the anticipated filing of a potential indication; projects shown are in phase II and phase III
✓ Indicates submission to health authorities has occurred
Unless stated otherwise submissions are planned to occur in US and EU
**also known as pediatric nephrotic syndrome (PNS)

2026				2027		2028	2029 and beyond	
RG6107	**PiaSky** aHUS	RG7159	**Gazyva✓** membranous nephropathy	RG6013	**Hemlibra** Type 3 VWD		RG7828	**Lunsumio** systemic lupus erythematosus
RG7446	**Tecentiq✓** Stage III dMMR-deficient or MSI-H colon cancer (US)	RG7159	**Gazyva✓** systemic lupus erythematosus	RG6114	**Itovebi + Phesgo** 1L HER2+ PIK3CA-mut. mBC		RG6114	**Itovebi+ribociclib+fulvestrant** 1L HR+ Chr8P-loss PIK3CA-wt mBC
RG7828	**Lunsumio + lenalidomide** 2L FL+	RG7159	**Gazyva✓** childhood onset idiopathic nephrotic syndrome**	RG6114	**Itovebi + fulvestrant** post CDKi HR+ PIK3CA-mut. BC		RG6114	**Itovebi + CDK4/6i + letrozole** 1L ES PIK3CA-mut. HR+ HER2- advanced BC
RG7828	**Lunsumio + Polivy✓** 2L+ DLBCL (US)	RG6168	**Enspryng** MOGAD	RG3502	**Kadcyla + Tecentriq** HER-2+ eBC high-risk		RG6114	**Itovebi + enzalutamide** CRPC
		RG6168	**Enspryng✓** TED (US)	RG6026	**Columvi + Polivy + R-CHP** 1L DLBCL			
		RG7716	**Vabysmo** CNV	RG6026	**Columvi** r/r MCL			
				RG6168	**Enspryng** autoimmune encephalitis			

Status as of July 23, 2026

Major pending approvals 2026

US

RG6171	**giredestrant + everolimus** post-CDK4/6 ER+/HER2- BC Filed December 2025
RG6171	**giredestrant** ER+ BC adj Filed March 2026
RG7446	**Tecentriq** stage III dMMR/MSI-H colon cancer Filed April 2026
RG7828	**Lunsumio + Polivy** 2L+ DLBCL Filed April 2026
RG7159	**Gazyva** childhood onset idiopathic nephrotic syndrome* Filed March 2026
RG7159	**Gazyva** systemic lupus erythematosus Filed February 2026
RG7159	**Gazyva** membranous nephropathy Filed June 2026
RG6168	**Enspryng** TED Filed April 2026

EU

RG7446	**Tecentriq** stage III dMMR/MSI-H colon cancer Filed July 2026
RG6321	**Susvimo** wAMD Filed July 2025
RG7159	**Gazyva** systemic lupus erythematosus Filed Feb 2026
RG7159	**Gazyva** childhood onset idiopathic nephrotic syndrome* Filed May 2026
RG7159	**Gazyva** membranous nephropathy Filed June 2026

China

RG6171	**giredestrant** ER+ BC adj Filed April 2026
RG7828	**Lunsumio + Polivy** r/r large B-cell lymphoma Filed July 2026
RG7159	**Gazyva** lupus nephritis Filed March 2026
RG7159	**Gazyva** systemic lupus erythematosus Filed March 2026
RG7159	**Gazyva** childhood onset idiopathic nephrotic syndrome* Filed April 2026
RG7159	**Gazyva** membranous nephropathy Filed May 2026
RG6168	**Enspryng** TED Filed May 2026

Japan-Chugai

RG7446	**Tecentriq** ctDNA+ high-risk MIBC Filed January 2026
RG6171	**giredestrant** post-CDK4/6 ER+/HER2- BC Filed May 2026
RG6171	**giredestrant** ER+ BC adj Filed May 2026
RG7159	**Gazyva** childhood onset idiopathic nephrotic syndrome* Filed May 2026
RG6321	**Susvimo** wAMD, DME Filed June 2026
RG7446	**Tecentriq** LA esophageal cancer maintenance Filed June 2026

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

*also known as pediatric nephrotic syndrome (PNS)

Status as of July 23, 2026

Major granted approvals 2026

US		EU		China		Japan-Chugai	
RG7446	**Tecentriq** ctDNA+ high-risk MIBC May 2026			RG7596	**Polivy + chemo** r/r DLBCL March 2026	RG7828	**Lunsumio + Polivy** r/r large B-cell lymphoma March 2026
						RG7853	**Alecensa** ALK+ solid tumors May 2026
						RG435	**Avastin** neurofibromatosis Type 2 June 2026

New Molecular Entity (NME)
Additional Indication (AI)
Oncology / Hematology
Immunology
Cardiovascular, Renal & Metabolism
Neurology
Ophthalmology
Other

Status as of July 23, 2026

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

Pharma sales appendix

Diagnostics sales appendix

Foreign exchange rates information

Marketed products development programmes

Oncology / Hematology	
RG7853	Alecensa
RG6114	Itovebi
RG3502	Kadcyla
RG7446	Tecentriq
RG6026	Columvi
RG7828	Lunsumio
RG7596	Polivy
RG6013	Hemlibra
RG6107	PiaSky

Neurology	
RG6356	Elevidys
RG6168	Enspryng
RG7916	Evrysdi

Immunology	
RG7159	Gazyva
RG7828	Lunsumio

Ophthalmology	
RG6168	Enspryng
RG6321	Susvimo
RG7716	Vabysmo

CVRM

Alecensa (alectinib, RG7853)

CNS-active inhibitor of anaplastic lymphoma kinase (ALK)

Indication	Locally advanced, unresectable, stage III NSCLC
Phase/study	**Phase III** **HORIZON-01**
# of patients	N=70
Design	▪ **ARM A:** Alecensa 600mg BID ▪ **ARM B:** durvalumab 1500 Q4W
Primary endpoint	▪ Progression-free survival
Status	▪ FPI Q4 2022 ▪ Recruitment completed Q2 2025
CT Identifier	NCT05170204

In collaboration with Chugai
CNS: Central nervous system; NSCLC: Non-small cell lung cancer

Itovebi (inavolisib, RG6114, GDC-0077)

A potent, orally available, and selective PI3Kα inhibitor

Indication	PIK3CA-mutant HR-positive metastatic breast cancer	post CDKi HR-positive PIK3CA-mutant breast cancer
Phase/study	**Phase III** **INAVO120**	**Phase III** **INAVO121**
# of patients	N=320	N=400
Design	▪ **ARM A:** Itovebi + palbociclib + fulvestrant ▪ **ARM B:** Placebo + palbociclib + fulvestrant	▪ **ARM A:** Itovebi + fulvestrant ▪ **ARM B:** alpelisib + fulvestrant
Primary endpoint	▪ Progression-free survival	▪ Progression-free survival
Status	▪ FPI Q1 2020 ▪ Recruitment completed Q3 2023 ▪ Study met its primary endpoint of PFS Q4 2023 ▪ Data presented at SABCS 2023 ▪ BTD granted by FDA Q2 2024 ▪ Filed in US (priority review) and EU Q2 2024 ▪ Data presented at ASCO 2024 and ASCO 2025 ▪ Approved in US Q3 2024 ▪ Published in NEJM 2024;391:1584-1596 and NEJM 2025;393:151-161 ▪ Approved in EU July 2025	▪ FPI Q2 2023 ▪ Recruitment completed Q4 2024
CT Identifier	NCT04191499	NCT05646862

ASCO: American Society of Clinical Oncology; HR: Hormone receptor; HER2: Human Epidermal growth factor Receptor 2; NEJM: New England Journal of Medicine; PIK3CA-mut: phosphatidylinositol 3-kinase, catalytic, alpha polypeptide mutated; SABCS: San Antonio Breast Cancer Symposium

Itovebi (inavolisib, RG6114, GDC-0077)

A potent, orally available, and selective PI3Kα inhibitor

Indication	1L HER2-positive PIK3CA-mutant metastatic breast cancer	1L endocrine-sensitive PIK3CA-mutant HR+, HER2-, adv. breast cancer
Phase/study	**Phase III** **INAVO122**	**Phase III** **INAVO123**
# of patients	N=230	N=450
Design	▪ **ARM A:** Itovebi + Phesgo after induction therapy with Phesgo + taxane ▪ **ARM B:** Placebo + Phesgo after induction therapy with Phesgo + taxane	▪ **ARM A:** Itovebi + CDK4/6i + letrozole ▪ **ARM B:** Placebo + CDK4/6i + letrozole
Primary endpoint	▪ Progression-free survival	▪ Progression free survival
Status	▪ FPI Q3 2023	▪ FPI Q2 2025
CT Identifier	NCT05894239	NCT06790693

HR: Hormone receptor; HER2: Human Epidermal growth factor Receptor 2; PIK3CA-mut: Phosphatidylinositol 3-kinase, catalytic, alpha polypeptide mutated; Phesgo: FDC of Perjeta and Herceptin for SC administration with Halozyme's rHuPH20/ Halozyme's human hyaluronidase

Itovebi (inavolisib, RG6114, GDC-0077)

A potent, orally available, and selective PI3Kα inhibitor

Indication	1L HR-positive 1L HR+ Chr8P-loss PIK3CA-wildtype metastatic breast cancer
Phase/study	**Phase II**
# of patients	N=80
Design	▪ **ARM A:** Itovebi + ribociclib + fulvestrant ▪ **ARM B:** Placebo + ribociclib + fulvestrant
Primary endpoint	▪ Confirmed Objective Response (cORR)
Status	▪ FPI Q2 2026
CT Identifier	NCT07405801

HER2: Human Epidermal growth factor Receptor 2; HR: Hormone receptor; PIK3CA: Phosphatidylinositol 3-kinase, catalytic, alpha polypeptide

Itovebi (inavolisib, RG6114, GDC-0077)

A potent, orally available, and selective PI3Kα inhibitor

Indication	post-ARPI metastatic prostate cancer (mCRPC)
Phase/study	**Phase II** **InavoPC**
# of patients	N=100
Design	▪ **ARM A:** Itovebi + enzalutamide ▪ **ARM B:** ARPi switch (enzalutamide or abiraterone) or docetaxel
Primary endpoint	▪ Radiographic Progression-free Survival (rPFS)
Status	▪ FPI Q1 2026
CT Identifier	NCT07287150

Kadcyla (trastuzumab emtansine, RG3502)

First ADC for HER2-positive breast cancer

Indication	HER2-positive early breast cancer (BC) high-risk patients
Phase/study	**Phase III** **ASTEFANIA**
# of patients	N=1150
Design	▪ **ARM A:** Kadcyla + Tecentriq ▪ **ARM B:** Kadcyla + placebo
Primary endpoint	▪ Invasive disease-free survival
Status	▪ FPI Q2 2021 ▪ Recruitment completed Q4 2024
CT Identifier	NCT04873362

In collaboration with Abbvie

ADC: antibody drug conjugate; HER2: Human Epidermal growth factor Receptor 2

Tecentriq (atezolizumab, RG7446)

Anti-PD-L1 cancer immunotherapy – lung cancer

Indication	1L maintenance extensive-stage SCLC
Phase/study	**Phase III** **IMforte**[1]
# of patients	N=450
Design	▪ **ARM A:** Platinum-etoposide + Tecentriq followed by maintenance Tecentriq + lurbinectedin ▪ **ARM B:** Platinum-etoposide + Tecentriq followed by maintenance Tecentriq
Primary endpoint	▪ Progression-free survival and overall survival
Status	▪ FPI Q4 2021 ▪ Recruitment completed Jan 2024 ▪ Study met primary endpoints Q3 2024 ▪ Filed in US (priority review) Q2 2025 ▪ Data presented at ASCO 2025 ▪ Approved in US Q3 2025 ▪ Approved in EU Q2 2026
CT Identifier	NCT05091567

[1]In collaboration with Jazz Pharma

PD-L1: Programmed cell death-ligand 1; SCLC: Small cell lung cancer; ASCO: American Society of Clinical Oncology

Tecentriq (atezolizumab, RG7446)

Anti-PD-L1 cancer immunotherapy – urothelial carcinoma

Indication	ctDNA+, high-risk muscle-invasive bladder cancer (MIBC)
Phase/study	**Phase III** **IMvigor011**
# of patients	N=240
Design	▪ **ARM A:** Tecentriq monotherapy ▪ **ARM B:** Placebo
Primary endpoint	▪ Disease-free survival (DFS)
Status	▪ FPI Q2 2021 ▪ Recruitment completed Q2 2025 ▪ Study met primary point of DFS Q3 2025 ▪ Data presented at ESMO 2025 ▪ Filed in US Q4 2025 ▪ Approved in US Q2 2026
CT Identifier	NCT04660344

ESMO: European Society for Medical Oncology; PD-L1: Programmed cell death-ligand 1

Tecentriq (atezolizumab, RG7446)

Anti-PD-L1 cancer immunotherapy – colon cancer

Indication	Stage III dMMR-deficient or MSI-H colon cancer
Phase/study	**Phase III** **ATOMIC**
# of patients	N=700
Design	▪ **ARM A:** Tecentriq + mFOLFOX6 (24 weeks) followed by Tecentriq up to 1yr from start of induction ▪ **ARM B:** mFOLFOX6 (24 weeks)
Primary endpoint	▪ Disease-free survival (DFS)
Status	▪ FPI Q1 2018 ▪ Recruitment completed Q4 2022 ▪ Study met primary point Q2 2025 ▪ Filed in US and EU Q2 2026
CT Identifier	NCT04660344

dMMR: Deficient DNA mismatch repair; MSI-H: Microsatellite instability-high; PD-L1: Programmed cell death-ligand 1

Columvi (glofitamab, RG6026)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	Relapsed or refractory Non-Hodgkin's lymphoma (NHL)	
Phase/study	**Phase I**	**Phase I**
# of patients	N=700	N=18-36
Design	**Cohort 1:** Single-agent dose escalation study ▪ Initial dose escalation ▪ Expansion cohort in r/r DLBCL ▪ Expansion cohort in r/r FL ▪ Expansion cohort in r/r MCL ▪ All patients will receive pretreatment with a single dose of Gazyva (1000mg) **Cohort 2:** Columvi + Gazyva (i.e. continuous treatment with Gazyva)	Columvi SC ▪ Part 1 dose escalation
Primary endpoint	▪ Efficacy, safety, tolerability and PK	▪ Safety
Status	▪ Data presented at ASH 2018, 2020, 2021, 2022, 2023, ICML 2019, 2021, EHA 2020, 2021, 2022 and ASCO 2021, 2022 and 2023 ▪ Data published in *J Clin Oncology* 2021; 39:18:1959-1970 and *NEJM* 2022; 387:2220-2231 ▪ Filed in EU Q2 2022 and US Q4 2022 ▪ Approved in Canada Q1, US Q2 and EU Q3 2023	▪ FPI Q3 2021 ▪ Recruitment completed Q1 2024
CT Identifier	NCT03075696	ISRCTN17975931

DLBCL: Diffuse large B cell lymphoma; FL: Follicular lymphoma; r/r: Relapsed or refractory; PK: Pharmacokinetics; ASCO: American Society of Clinical Oncology; ASH: American Society of Hematology; EHA: European Hematology Association; ICML: International Conference on Malignant Lymphoma; NEJM: New England Journal of Medicine

Columvi (glofitamab, RG6026)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	2L+ SCT-ineligible DLBCL
Phase/study	**Phase Ib**
# of patients	N=112
Design	▪ **ARM A:** Columvi IV + CELMoD (CC-220 and CC-99282) ▪ **ARM B:** Lunsumio SC + CELMoD (CC-220 and CC-99282)
Primary endpoint	▪ Safety, DLT, RPTD
Status	▪ FPI Q3 2019
CT Identifier	NCT05169515

DLBCL: Diffuse large B cell lymphoma; DLT: Dose-limiting toxicity, RPTD: Recommended Phase II Dose

Columvi (glofitamab, RG6026)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	2L+ SCT-ineligible DLBCL	1L DLBCL fit (IPI 2-5)
Phase/study	**Phase III** **STARGLO**	**Phase III** **SKYGLO**
# of patients	N=270	N=1130
Design	▪ **ARM A:** Columvi + gemcitabine and oxaliplatin, followed by up to 4 cycles of Columvi monotherapy ▪ **ARM B:** Rituximab in combination with gemcitabine and oxaliplatin A single dose of Gazyva will be administered 7 days prior to the first dose of Columvi	▪ **ARM A:** Columvi + Polivy + R-CHP ▪ **ARM B:** Polivy + R-CHP
Primary endpoint	▪ Overall survival	▪ Progression-free survival
Status	▪ FPI Q1 2021 ▪ Recruitment completed Q1 2023 ▪ Study met primary endpoint April 2024 ▪ Data presented at EHA 2024 ▪ Filed in EU and US Q3 2024 ▪ Approved in EU Q2 2025 ▪ 2yr follow-up data presented at ASCO 2025 ▪ 3yr follow-up data presented at ASH 2025 ▪ Data published in Blood 2026 Jun 10; doi: 10.1182/bloodadvances.2026019764	▪ FPI Q4 2023 ▪ Recruitment completed Q3 2025
CT Identifier	NCT04408638	NCT06047080

ASCO: American Society of Clinical Oncology; ASH: American Society of Hematology; DLBCL: Diffuse large B cell lymphoma; EHA: European Hematology Association; SCT: Stem cell transplant; R-CHP: Rituxan, cyclophosphamide, hydroxydoxorubicin, prednisone

Columvi (glofitamab, RG6026)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	Relapsed or refractory mantle cell lymphoma (MCL)
Phase/study	**Phase III** **GLOBRYTE**
# of patients	N=182
Design	▪ **ARM A:** Columvi monotherapy ▪ **ARM B**: Bendamustine + rituximab or rituximab + lenalidomide
Primary endpoint	▪ Progression-free survival by IRC
Status	▪ FPI Q4 2023 ▪ BTD granted by FDA Q2 2024
CT Identifier	NCT06084936

BTD: Breakthrough Therapy Designation; IRC: Independent review committee

Lunsumio (mosunetuzumab, RG7828)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	2L+ FL
Phase/study	**Phase III** **CELESTIMO**
# of patients	N=474
Design	▪ **ARM A**: Lunsumio + lenalidomide ▪ **ARM B:** Rituximab + lenalidomide
Primary endpoint	▪ Progression-free survival
Status	▪ FPI Q4 2021 ▪ Recruitment completed Q2 2025
CT Identifier	NCT04712097

FL: Follicular lymphoma

Lunsumio (mosunetuzumab, RG7828)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	2L+ SCT ineligible DLBCL	Relapsed or refractory LBCL & MCL
Phase/study	**Phase III SUNMO**	**Phase Ib/II**
# of patients	N=222	N=235
Design	▪ **ARM A:** Lunsumio + Polivy ▪ **ARM B:** R + GemOx	Dose escalation of Lunsumio + Polivy ▪ **ARM A:** Lunsumio SC + Polivy ▪ **ARM B:** Rituximab + Polivy
Primary endpoint	▪ Progression-free survival	▪ Safety/tolerability and response
Status	▪ FPI Q2 2022 ▪ Recruitment completed Q4 2024 ▪ Study met dual primary endpoints (ORR, PFS) Q2 2025 ▪ Data presented at EHA, ICML 2025 ▪ Filed in US Q2 2026	▪ FPI Q3 2018 ▪ Initial data presented at ASCO 2021 and ASH 2021, 2022 ▪ Data presented at ASH 2023 ▪ Data published in *Nature Medicine* 2023; 30, 229–239 ▪ Recruitment completed Q1 2024
CT Identifier	NCT05171647	NCT03671018

ASCO: American Society of Clinical Oncology; ASH: American Society of Hematology; DLBCL: Diffuse large B cell lymphoma; EHA: European Hematology Association; ICML: International Conference on Malignant Lymphoma; MCL: Mantle cell lymphoma; LBCL: Large B cell lymphoma; R: Rituxan/MabThera; GemOx: Gemcitabin und Oxaliplatin

Polivy (polatuzumab vedotin, RG7596)

ADC targeting CD79b to treat B cell malignancies

Indication	1L DLBCL
Phase/study	**Phase III** **POLARIX**
# of patients	N=879
Design	▪ **ARM A:** Polivy + R-CHP ▪ **ARM B:** R-CHOP
Primary endpoint	▪ Progression-free survival
Status	▪ Data presented at ASH 2021 and 2022 and 2024 ▪ Filed in EU, Japan and China Q4 2021 and in the US Q3 2022 ▪ Published in *NEJM* 2022 27;386(4):351-363 ▪ Approved in EU Q2 2022, Japan Q3 2022, China Q1 2023 and US April 2023
CT Identifier	NCT03274492

In collaboration with Pfizer

ASH: American Society of Hematology, DLBCL: diffuse large B cell lymphoma; NEJM: New England Journal of Medicine; R-CHP: Rituxan, cyclophosphamide, hydroxydoxorubicin, prednisone; R-CHOP: Rituxan, cyclophosphamide, doxorubicin, vincristine, and prednisone

Hemlibra (emicizumab, RG6013)

Factor VIII mimetic for treatment of hemophilia A

Indication	Type 3 Von Willebrand disease (VWD)
Phase/study	**Phase III** **WILL-EMI**
# of patients	N=66
Design	Open-label in participants aged 2 years and above diagnosed with Type 3 von Willebrand disease (VWD) on prior SOC on demand therapy ▪ **ARM A:** Hemlibra prophylaxis ▪ **ARM B:** Continuation of SOC on-demand therapy ▪ **ARM C:** Participants on prior SOC prophylactic therapy (Hemlibra prophylaxis) - to be assessed via intra-participant analysis with data obtained from the preceding non-interventional study (NIS)
Primary endpoint	▪ Efficacy (Annualized Bleed Rate at 24w), safety, pharmacokinetics, and pharmacodynamics
Status	▪ FPI Q2 2025
CT Identifier	NCT06998524

In collaboration with Chugai; Note: Hemlibra autoinjector in development for existing and future potential Hemlibra indications

PiaSky (crovalimab, RG6107, SKY59)

A humanized monoclonal antibody against complement C5

Indication	Atypical hemolytic uremic syndrome (aHUS) - adults	Atypical hemolytic uremic syndrome (aHUS) - paediatric
Phase/study	**Phase III COMMUTE-a**	**Phase III COMMUTE-p**
# of patients	N=90	N=35
Design	Single-arm study of aHUS patients ▪ **Cohort 1**: not previously treated with C5i ▪ **Cohort 2:** switching from C5i ▪ **Cohort 3:** known C5 polymorphism	Single-arm study of aHUS patients ▪ **Cohort 1:** not previously treated with C5i ▪ **Cohort 2:** switching from C5i ≤18y/o ▪ **Cohort 3:** previously treated with C5i (includes participants with known C5 polymorphism)
Primary endpoint	▪ Cohort 1+3: proportion of patients with complete TMA response anytime between baseline and week 25 ▪ Cohort 2: proportion of patients with maintained TMA control from baseline through week 25	▪ Cohort 1: proportion of patients with complete TMA response anytime between baseline and week 25 ▪ Cohort 2: proportion of patients with maintained TMA control from baseline through week 25
Status	▪ FPI Q4 2021 ▪ Recruitment completed Q2 2025 ▪ Positive results Q4 2025 ▪ Data presented at ERA 2026	▪ FPI Q4 2021 ▪ Recruitment completed Q1 2025 ▪ Positive results Q4 2025 ▪ Data presented at ERA 2026
CT Identifier	NCT04861259	NCT04958265

In collaboration with Chugai
aHUS: Atypical Hemolytic Uremic Syndrome; C5i: C5 inhibitor; TMA: thrombotic microangiopathy

Elevidys (delandistrogene moxeparvovec, RG6356, SRP-9001)

rAAVrh74.MHCK7.Micro-dystrophin gene therapy

Indication	Duchenne muscular dystrophy (DMD)
Phase/study	**Phase II** **ENVOL**
# of patients	N=13
Design	Open label single arm study in 2 to <4 year old DMD boys who will receive a single intravenous (IV) infusion of Elevidys on Day 1, separated into 4 cohorts: ▪ **Cohort A**: 10 participants who are 3 years of age ▪ **Cohort B:** 3 participants who are 2 years of age
Primary endpoint	▪ Safety
Status	▪ FPI Q4 2023 ▪ Cohort A: Recruitment completed Q3 2024 ▪ Cohort B: Recruitment stopped after 3rd enrolled patient Q1 2025
CT Identifier	NCT06128564

In collaboration with Sarepta
DMD: Duchenne muscular dystrophy

Enspryng (satralizumab, RG6168, SA237)

Anti-IL-6 receptor humanized monoclonal antibody

Indication	Myelin oligodendrocyte glycoprotein antibody disease (MOGAD)	Autoimmune encephalitis (AIE)
Phase/study	**Phase III METEOROID**	**Phase III CIELO**
# of patients	N=152	N=152
Design	▪ **ARM A**: Enspryng at weeks 0, 2, 4 (loading doses) and maintenance doses Q4W ▪ **ARM B:** Placebo	▪ **ARM A**: Enspryng at weeks 0, 2, 4 (loading doses) and maintenance doses Q4W ▪ **ARM B**: Placebo
Primary endpoint	▪ Time from randomization to the first occurrence of a MOGAD relapse	▪ Efficacy (proportion of participants with mRS score improvement ≥ 1 from baseline and no use of rescue therapy at week 24 (NMDAR) and week 52 (LGI1) and safety
Status	▪ FPI Q3 2022 ▪ ODD granted by FDA in Q4 2021 ▪ Study met primary endpoint Jan 2026 ▪ Data presented at AAN 2026	▪ FPI Q3 2022 ▪ ODD granted for NMDAR AIE in US Q3 22 and for LGI1 AIE Q4 2024 ▪ Recruitment completed Q2 2026
CT Identifier	NCT05271409	NCT05503264

In collaboration with Chugai; MG-AD: Myasthenia Gravis Activities of Daily Living; MOGAD: Myelin Oligodendrocyte Glycoprotein Antibody Disease; AIE: Autoimmune encephalitis; NMDAR AIE: Anti-N-Methyl-D-Aspartic Acid Receptor Autoimmune Encephalitis; ODD: Orphan drug designation

Evrysdi (risdiplam, RG7916)

Oral SMN2 splicing modifier

Indication	Spinal muscular atrophy (SMA)	
Phase/study	**Phase II RAINBOWFISH**	**Phase II JEWELFISH**
# of patients	N=25	N=174
Design	▪ Infants aged from birth to 6 weeks who have been genetically diagnosed with SMA but are not yet presenting with symptoms	▪ Adult and pediatric patients with previously treated SMA type 1, 2 and 3
Primary endpoint	▪ Proportion of participants with two copies of the SMN2 gene and baseline CMAP>=1.5 millivolt who are sitting without support	▪ Safety, tolerability, PK/PD
Status	▪ FPI Q3 2019 ▪ Recruitment completed Q1 2022 ▪ Initial data presented at CureSMA , WMS 2021, MDA and WMS 2022 ▪ Primary data presented at WMS 2023 ▪ Filed in US and EU Q4 2021 ▪ Approved in US Q2 2022 and EU Q3 2023 ▪ 2-year data presented at WMS 2024 ▪ 2yr data published in NEJM 2025; 393:671-682 ▪ 3-year data presented at WMS 2025	▪ Data presented at WMS 2017, AAN 2018, WMS 2018, CureSMA 2019, WMS 2019, CureSMA 2020 and 2021 ▪ 2-year data presented at WMS 2022 ▪ Data published J Neurol. 2024 Aug;271(8):4871-4884 ▪ 5-year data presented at WMS 2025
CT Identifier	NCT03779334	NCT03032172

In collaboration with PTC Therapeutics and SMA Foundation

SMN: survival motor neuron; CMAP: compound muscle action potential; WMS: World Muscle Society; CureSMA: Annual SMA Conference; MDA: Muscular Dystrophy Association

Gazyva (obinutuzumab, RG7159)

Humanized anti-CD20 monoclonal antibody; immunology development program

Indication	Lupus nephritis	Childhood onset idiopathic nephrotic syndrome*
Phase/study	**Phase III** **REGENCY**	**Phase III** **INShore**
# of patients	N=252	N=80
Design	▪ **ARM A:** Gazyva 1000mg IV (6 doses through Week 52) + MFF ▪ **ARM B:** Gazyva1000 mg IV (5 doses through Week 52) + MFF ▪ **ARM C:** Placebo IV + MFF	▪ **ARM A:** Gazyva + oral steroids ▪ **ARM B:** Mycophenolate mofetil (MMF) + oral steroids
Primary endpoint	▪ Percentage of participants who achieve complete renal response (CRR)	▪ Percentage of participants with sustained complete remission at 1 year
Status	▪ FPI Q3 2020 ▪ Recruitment completed Q1 2023 ▪ Primary endpoint met Q3 2024 ▪ Filed in US and EU in Q1 2025 ▪ Published in NEJM 2025 Feb 7. doi: 10.1056/NEJMoa2410965. ▪ Approved in US Q3 2025 and EU Q4 2025	▪ FPI Q1 2023 ▪ Recruitment completed Q3 2024 ▪ Primary endpoint met Q4 2025 ▪ Filed in US Q1 2026 and EU Q2 2026 ▪ BTD granted by FDA Q2 2026 ▪ Data presented at WCN 2026
CT Identifier	NCT04221477	NCT05627557

In collaboration with Biogen

*also known as pediatric nephrotic syndrome (PNS); MMF: mycophenolate mofetil; WCN: World Congress of Nephrology

Gazyva (obinutuzumab, RG7159)

Humanized anti-CD20 monoclonal antibody; immunology development program

Indication	Systemic lupus erythematosus (SLE)	Membranous nephropathy
Phase/study	**Phase III** **ALLEGORY**	**Phase III** **MAJESTY**
# of patients	N=300	N=140
Design	▪ **ARM A:** Gazyva 1000mg IV on Day 1 and Weeks 2, 24 and 26. ▪ **ARM B:** Placebo IV	▪ **ARM A:** Gazyva 1000mg IV on top of renin-angiotensin inhibitors ▪ **ARM B:** Tacrolimus treatment for 12 months
Primary endpoint	▪ Percentage of participants who achieve Systemic Lupus Erythematosus Responder Index (SRI) at week 52	▪ Percentage of patients who achieve complete remission at week 104
Status	▪ FPI Q4 2021 ▪ Recruitment completed Q3 2024 ▪ Primary endpoint met Q4 2025 ▪ Filed in US and EU Q1 2026 ▪ Data presented at SLEuro 2026	▪ FPI Q2 2021 ▪ Recruitment completed Q4 2023 ▪ Primary endpoint met Q1 2026 ▪ BTD granted by FDA Q2 2026 ▪ Data presented at ERA 2026 ▪ Published in NEJM 2026 June. 5. doi: 10.1056/NEJMoa2602678 ▪ Filed in US and EU Q2 2026
CT Identifier	NCT04963296	NCT04629248

In collaboration with Biogen

Lunsumio (mosunetuzumab, RG7828, CD20 x CD3)

Bispecific anti-CD20/CD3 antibody engaging T and B cells simultaneously

Indication	Systemic lupus erythematosus (SLE)
Phase/study	**Phase II** **SOLUNA**
# of patients	N=30
Design	▪ Single arm Lunsumio SC
Primary endpoint	▪ Proportion of participants who have achieved remission by Week 76
Status	▪ FPI Q2 2026
CT Identifier	NCT07598396

In collaboration with Biogen
EULAR: European Alliance of Associations for Rheumatology

Enspryng (satralizumab, RG6168, SA237)

Anti-IL-6 receptor humanized monoclonal antibody

Indication	Thyroid eye disease	
Phase/study	**Phase III SatraGO-1**	**Phase III SatraGO-2**
# of patients	N=120	N=120
Design	▪ **ARM A:** Enspryng at weeks 0, 2, 4 (loading doses) and maintenance doses Q4W ▪ **ARM B:** Placebo	▪ **ARM A:** Enspryng at weeks 0, 2, 4 (loading doses) and maintenance doses Q4W ▪ **ARM B:** Placebo
Primary endpoint	▪ Proportion of participants with active disease achieving ≥ 2 mm reduction in proptosis from baseline (Day 1) at week 24 in the study eye, provided there is no deterioration of proptosis (≥ 2mm increase) in the fellow eye	▪ Proportion of participants with active disease achieving ≥ 2 mm reduction in proptosis from baseline (Day 1) at week 24 in the study eye, provided there is no deterioration of proptosis (≥ 2mm increase) in the fellow eye
Status	▪ FPI Q4 2023 ▪ Recruitment completed Q1 2025 ▪ SatraGO-1 did not meet its primary endpoint, but showed a positive numerical trend towards proptosis improvement ▪ Data presented at ASOPRS 2025 ▪ Filed in US Q2 2026	▪ FPI Q4 2023 ▪ Recruitment completed Q1 2025 ▪ SatraGO-2 met its primary endpoint ▪ Data presented at ASOPRS 2025 ▪ Filed in US Q2 2026
CT Identifier	NCT05987423	NCT06106828

In collaboration with Chugai
ASOPRS: American Society of Ophthalmic Plastic and Reconstructive Surgery

Susvimo (PDS, RG6321)

First eye implant to achieve sustained delivery of a biologic medicine

Indication	Wet age-related macular degeneration (wAMD)		
Phase/study	**Phase III Archway**	**Phase II+III extension Portal**	**Phase IIIb Velodrome**
# of patients	N=418	N=1,000	N=442
Design	▪ **ARM A:** PDS 100mg/mL Q24W ▪ **ARM B:** Intravitreal ranibizumab Q4W	▪ Ex-LADDER/ex-Archway: PDS 100mg/mL Q24W ▪ Ex-Velodrome, not meeting Q36W criteria @ week 24: PDS 100mg/mL Q24W ▪ Ex-Velodrome (completed or withdrawn): PDS Q24W or Q36W (as per Velodrome randomization)	▪ **ARM A:** PDS 100mg/mL Q36W ▪ **ARM B:** PDS 100mg/mL Q24W
Primary endpoint	▪ Change in BCVA from baseline at the average of week 36 and week 40	▪ Long term safety efficacy	▪ Change in BCVA from baseline averaged over weeks 68 and 72
Status	▪ Study met primary endpoint Q2 2020 ▪ Data presented at ASRS 2020, 44/48 week data at Angiogenesis 2021 and 2-year data at Angiogenesis 2022 ▪ Approved in US Q4 2021 ▪ 5 year data presented at ASRS 2025 ▪ 7 year data presented to EURETINA 2025 ▪ CONTIVUE® with Susvimo filed in EU Q3 2025	▪ FPI Q3 2018	▪ FPI Q3 2021 ▪ Recruitment completed Q3 2025
CT Identifier	NCT03677934	NCT03683251	NCT04657289

ASRS: American Society of Retinal Specialists; BCVA: Best corrected visual acuity; wAMD: Wet age-related macular degeneration; PDS: Port Delivery System with ranibizumab

Susvimo (PDS, RG6321)

First eye implant to achieve sustained delivery of a biologic medicine

Indication	Diabetic macular edema (DME)	Diabetic retinopathy (DR) without center-involved diabetic macular edema (DME)
Phase/study	**Phase III Pagoda**	**Phase III Pavilion**
# of patients	N=634	N=174
Design	▪ **ARM A:** Intravitreal ranibizumab (X4) followed by PDS 100mg/mL Q24W ▪ **ARM B:** Intravitreal ranibizumab Q4W until PDS 100mg/mL is received	▪ **ARM A:** Intravitreal ranibizumab (X2) followed by PDS100mg/mL (refill Q36W) ▪ **ARM B:** Q4W comprehensive clinical monitoring (with IVT ranibizumab as needed) until participants receive PDS100mg/mL (refill Q36W)
Primary endpoint	▪ Change in BCVA from baseline at the average of week 60 and week 64	▪ Percentage of participants with a ≥2-step improvement from baseline on the ETDRS-DRSS at Week 52
Status	▪ FPI Q3 2019 ▪ Recruitment completed Q2 2021 ▪ Study met its primary endpoint Q4 2022 ▪ Data presented at Angiogenesis 2023 ▪ Filed in US Q2 2024 ▪ 2-year data presented at ASRS 2024 ▪ Approved in US Q1 2025	▪ FPI Q3 2020 ▪ Recruitment completed Q3 2021 ▪ Study met its primary endpoint Q4 2022 ▪ Data presented at Angiogenesis 2023 ▪ Filed in US Q2 2024 ▪ 2-year data presented at ASRS 2024 ▪ Approved in US Q2 2025
CT Identifier	NCT04108156	NCT04503551

ASRS: American Society of Retinal Specialists; BCVA: Best corrected visual acuity; ETDRS: Early Treatment Diabetic Retinopathy Study; DRSS: Diabetic Retinopathy Severity Scale; PDS: Port Delivery System with ranibizumab

Vabysmo (faricimab, RG7716)

Bispecific antibody for the eye which targets and inhibits two signaling pathways (Ang-2 and VEGF-A)

Indication	Myopic choroidal neovascularization (CNV)
Phase/study	**Phase III** **POYANG**
# of patients	n=280
Design	▪ **ARM A:** Vabysmo 6.0 mg Q4W PRN ▪ **ARM B:** Ranibizumab 0.5 mg Q4W PRN
Primary endpoint	▪ Change from Baseline in Best-Corrected Visual Acuity (BCVA) Averaged Over Weeks 4, 8, and 12
Status	▪ FPI Q1 2024 ▪ Recruitment completed Q2 2025 ▪ Study met primary endpoint July 2026
CT Identifier	NCT06176352

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

Pharma sales appendix

Diagnostics sales appendix

Foreign exchange rates information

Roche Pharma global development programmes

Oncology / Hematology	
RG6330	Divarasib
RG6171	Giredestrant
RG6596	HER2 TKI
RG6919	Cevostamab
RG6512	Zemocimig (NXT007)
RG6797	Dirloctocogene Samoparvovec

Neurology	
RG7845	Fenebrutinib
RG7935	Prasinezumab
RG6042	Tominersen
RG6102	Trontinemab

Immunology	
RG6299	Sefaxersen
RG6631	Afimkibart

Ophthalmology	
RG6179	Vamikibart

CVRM	
RG6640	Enicepatide (CT-388)
RG6652	CT-996
RG6237	Emugrobart
RG6881	Pegozafermin

Divarasib (KRAS G12C inhibitor, RG6330, GDC-6036)

A potent, orally available, and selective inhibitor of the KRAS G12C mutant protein

Indication	1L NSCLC	2L NSCLC	1L NSCLC
Phase/study	**Phase Ib/II KRASCENDO 170**	**Phase III KRASCENDO 1**	**Phase III KRASCENDO 2**
# of patients	N=320	N=320	N=600
Design	▪ **Cohort A**: Combination of divarasib + pembrolizumab ▪ **Cohort B:** Combination of divarasib + pembrolizumab + carboplatin/cisplatin + pemetrexed	▪ H2H vs KRAS G12Ci ▪ **ARM A**: divarasib ▪ **ARM B**: locally available G12Ci (sotorasib or adagrasib)	▪ **ARM A:** divarasib + pembrolizumab ▪ **ARM B:** pembrolizumab + carboplatin/cisplatin + pemetrexed
Primary endpoint	▪ Safety, tolerability	▪ PFS	▪ PFS ▪ OS
Status	▪ **Cohort A**: FPI Q2 2023 ▪ **Cohort B**: FPI Q1 2024 ▪ Recruitment completed Q4 2025 ▪ Data presented at ASCO 2026	▪ FPI Q3 2024 ▪ Recruitment completed Q4 2025 ▪ Study met primary endpoint Q2 2026	▪ FPI Q4 2025
CT Identifier	NCT05789082	NCT06497556	NCT06793215

NSCLC: Non-small cell lung cancer

Giredestrant (SERD (3), RG6171, GDC-9545)

A selective estrogen receptor degrader or downregulator

Indication	Adjuvant ER-positive breast cancer (BC)	1L ER-positive metastatic breast cancer (mBC)	ET resistant ER+/HER2-negative breast cancer (BC)
Phase/study	**Phase III lidERA Breast Cancer**	**Phase III persevERA Breast Cancer**	**Phase III pionERA**
# of patients	N=4,100	N=978	N=1050
Design	▪ **ARM A:** Giredestrant monotherapy ▪ **ARM B:** Tamoxifen or aromatase inhibitor	▪ **ARM A:** Giredestrant + palbociclib ▪ **ARM B:** Letrozole + palbociclib	▪ **ARM A**: Giredestrant + CDK4/6i ▪ **ARM B**: Fulvestrant + CDK4/6i
Primary endpoint	▪ Invasive disease-free survival	▪ Progression-free survival	▪ Progression-free survival in ESR1m and ITT
Status	▪ FPI Q3 2021 ▪ Recruitment completed Q3 2023 ▪ Study met its primary endpoint at interim analysis Q4 2025 ▪ Presented at SABCS 2025 ▪ Filed in US Q1 2026	▪ FPI Q4 2020 ▪ Recruitment completed Q1 2023 ▪ Study did not meet primary endpoint Q1 2026	▪ FPI Q4 2023
CT Identifier	NCT04961996	NCT04546009	NCT06065748

ER: Estrogen receptor; HER2: Human Epidermal growth factor Receptor; SABCS: San Antonio Breast Cancer Symposium

Giredestrant (SERD (3), RG6171, GDC-9545)

A selective estrogen receptor degrader or downregulator

Indication	post-CDK4/6 ER-positive/HER2-negative breast cancer (BC)	1L ER-positive/HER2-positive breast cancer (BC)
Phase/study	**Phase III evERA**	**Phase III heredERA**
# of patients	N=373	N=812
Design	▪ **ARM A**: giredestrant + everolimus ▪ **ARM B**: exemestane + everolimus	Induction Phesgo + taxane followed by maintenance with either: ▪ **ARM A:** Giredestrant + Phesgo ▪ **ARM B:** Phesgo
Primary endpoint	▪ Progression-free survival in ESR1m and ITT	▪ Progression-free survival
Status	▪ FPI Q3 2022 ▪ Recruitment completed Q3 2024 ▪ Study met its primary endpoint of PFS Q3 2025 ▪ Presented at ESMO 2025 ▪ Filed in US Q4 2025	▪ FPI Q2 2022 ▪ Recruitment completed Q3 2025
CT Identifier	NCT05306340	NCT05296798

ER: Estrogen receptor; ESMO: European Society for Medical Oncology; HER2: Human Epidermal growth factor Receptor; Phesgo: FDC of Perjeta and Herceptin for SC administration with Halozyme's rHuPH20/ Halozyme's human hyaluronidase; ITT: Intention to treat

HER 2 TKI (ZN-1041, RG6596)

Oral blood-brain barrier penetrant HER2 TKI

Indication	2L+ HER2+ mBC
Phase/study	**Phase II/III** **BREnnA**
# of patients	N=650
Design	▪ **ARM A**: ZN-1041 dose 1 + trastuzumab + capecitabine ▪ **ARM B**: ZN-1041 dose 2 + trastuzumab + capecitabine ▪ **ARM C**: tucatinib + trastuzumab + capecitabine
Primary endpoint	▪ Progression free survival
Status	▪ FPI Q2 2026
CT Identifier	NCT07413939

HER2: Human Epidermal growth factor receptor 2; TKI: Tyrosine kinase inhibitor

Cevostamab (RG6919)

Bispecific anti-FcRH5/CD3 antibody

Indication	**2-4L relapsed/refractory multiple myeloma**
Phase/study	**Phase III** **CEVOLUTION**
# of patients	N=380
Design	▪ **ARM A**: Cevostamab + Pd ▪ **ARM B**: SoC (DPd, PVd, EPd)
Primary endpoint	▪ MRD-neg CR at 9 mo ▪ PFS IRC-assessed
Status	▪ FPI Q2 2026
CT Identifier	NCT07555938

CR: Complete response; DPd: Daratumumab, pomalidomide, dexamethasone; EPd: Elotuzumab, pomalidomide, dexamethasone; IRC: Independent review committee; MM MRD: Minimal residual disease; Pd: Pomalidomide, dexamethasone; PFS: Progression free survival; PVd: Pomalidomide, bortezomib, dexamethasone; RR: Relapsed/refractory; SoC: Standard of care

Zemocimig (NXT007, RG6512)

Bispecific antibody which targets Factor IXa and Factor X

Indication	Hemophilia A		
Phase/study	**Phase I/II (multiple-ascending dose)**	**Phase III (ZEBRHA 1)**	**Phase III (ZEBRHA 2)**
# of patients	N=40	N=126	N=360
Design	▪ Two loading doses of NXT007 SC (Q2W) followed by Q4W maintenance dosing	▪ H2H vs Factor VIII **ARM A:** Two loading doses, followed by Q4W maintenance of zemocimig **ARM B:** FVIII standard of care prophylaxis	▪ H2H vs Hemlibra **ARM A:** Two loading doses, followed by Q4W maintenance of zemocimig **ARM B:** Hemlibra prophylaxis
Primary endpoint	▪ Safety	▪ Annualized Bleed Rate (ABR) for Treated Bleeds Over the Main Study Treatment Period	▪ Annualized Bleed Rate (ABR) for Treated Bleeds Over the Main Study Treatment Period
Status	▪ FPI Q4 2023 ▪ Recruitment completed Q4 2024 ▪ Data presented at ASH 2025	▪ FPI Q2 2026	▪ FPI Q2 2026
CT Identifier	NCT05987449	NCT07416526	NCT07416604

In collaboration with Chugai

ASH: American Society of Hematology; FIXa: Factor 9a; FX: Factor 10

Dirloctocogene Samoparvovec (SPK-8011QQ; RG6797)

Gene therapy for Hemophilia A

Indication	**Moderately severe to severe Hemophilia A (>18yrs)**
Phase/study	**Phase IIb**
# of patients	N=5
Design	▪ Participants will receive an IV infusion of dirloctocogene samoparvovec on day 1
Primary endpoint	▪ Safety
Status	▪ FPI July 2026
CT Identifier	NCT07226206

Fenebrutinib (RG7845, GCD-0853)

Highly selective and reversible (noncovalent) bruton tyrosine kinase

Indication	Relapsing multiple sclerosis (RMS)		
Phase/study	**Phase III FENhance 1**	**Phase III FENhance 2**	**Phase II (Biomarker study) FENopta**
# of patients	N=746	N=751	N=109
Design	▪ **ARM A:** Fenebrutinib twice daily oral ▪ **ARM B:** Teriflunomide once daily oral	▪ **ARM A:** Fenebrutinib twice daily oral ▪ **ARM B:** Teriflunomide once daily oral	▪ **ARM A:** Fenebrutinib ▪ **ARM B:** Placebo
Primary endpoint	▪ Annualized relapse rate	▪ Annualized relapse rate	▪ Total number of new gadolinium-enhancing T1 lesions observed on MRI scans of the brain at 12 weeks
Status	▪ FPI Q1 2021 ▪ Recruitment completed Q1 2024 ▪ Primary endpoint met Q1 2026 ▪ Presented at AAN 2026	▪ FPI Q1 2021 ▪ Recruitment completed Q4 2023 ▪ Primary endpoint met Q4 2025 ▪ Presented at AAN 2026	▪ Data presented at EAN and ECTRIMS 2023 ▪ 48-week OLE data presented at ECTRIMS 2024 ▪ 96 week OLE data presented at CMSC 2025 and ECTRIMS 2025
CT Identifier	NCT04586010	NCT04586023	NCT05119569

AAN: American Academy of Neurology; CMSC: Consortium of Multiple Sclerosis Centers; EAN: European Academy of Neurology; ECTRIMS: European Committee for Treatment and Research in Multiple Sclerosis

Fenebrutinib (RG7845, GCD-0853)

Highly selective and reversible (noncovalent) bruton tyrosine kinase

Indication	Primary progressive multiple sclerosis (PPMS)
Phase/study	**Phase III** **FENtrepid**
# of patients	N=985
Design	▪ **ARM A:** Fenebrutinib twice daily oral ▪ **ARM B:** Ocrevus 2x300mg IV Q24W
Primary endpoint	▪ Time to onset of cCDP12
Status	▪ FPI Q4 2020 ▪ Recruitment completed Q2 2023 ▪ Primary endpoint met Q4 2025 ▪ Presented at ACTRIMS 2026
CT Identifier	NCT04544449

ACTRIMS: Americas Committee for Treatment and Research in Multiple Sclerosis; cCDP12: Time to Onset of Composite 12-week Confirmed Disability Progression

Prasinezumab (anti-αSynuclein; RG7935)

Anti-alpha-synuclein antibody under investigation for Parkinson's disease

Indication	Early-stage Parkinson's disease		
Phase/study	**PASADENA** **Phase II**	**PADOVA** **Phase IIb**	**PARAISO** **Phase III**
# of patients	316	586	N=900
Design	▪ PART 1: ▪ **ARM A:** Prasinezumab IV Q4W (low dose) ▪ **ARM B:** Prasinezumab IV Q4W (high dose) ▪ **ARM C:** Placebo Q4W ▪ Part 2: ▪ **COHORT A:** Prasinezumab IV Q4W (low dose) vs Placebo Q4W ▪ **COHORT B:** Prasinezumab IV Q4W (high dose) vs Placebo Q4W ▪ Part 3:All low dose and high dose participants to receive low dose prasinezumab IV Q4W for an additional 5 years	▪ **ARM A:** Prasinezumab IV Q4W ▪ **ARM B:** Placebo Q4W ▪ OLE: Participant to enter the OLE once double-blind tx period has been completed	▪ **ARM A:** Prasinezumab IV Q4W ▪ **ARM B:** Placebo Q4W ▪ OLE: Participant to enter the OLE once double-blind tx period has been completed
Primary endpoint	▪ Change from baseline in movement disorder society-unified Parkinson's disease rating scale (MDS-UPDRS) total score (sum of Parts I, II, and III) at week 52	▪ Time to Confirmed Motor Progression Event from BL to 28 days after last dose	▪ Time to Confirmed Motor Progression Event (MDS-UPDRS Part III Score)
Status	▪ Recruitment completed Q4 2018 ▪ Data presented at MDS & ADPD 2020-22 ▪ OLE data presented at MDS 2023 and ADPD 2024	▪ Recruitment completed Q1 2023 ▪ Primary endpoint missed, but numerical delay in motor progression and positive trends on multiple secondary and exploratory endpoints shown. ▪ Data presented at ADPD 2025 and ADPD 2026	▪ FPI Q4 2025
CT Identifier	NCT03100149	NCT04777331	NCT07174310

In collaboration with Prothena; ADPD: International Conference on Alzheimer's and Parkinson's Diseases; MDS: International Congress of Parkinson's Disease and Movement Disorders

Trontinemab (BS-anti-Aβ mAb, RG6102)

A novel Brainshuttle™ bispecific 2+1 monoclonal antibody targeting Aβ

Indication	Prodromal or mild to moderate Alzheimer's Disease	Early Alzheimer's Disease	
Phase/study	**Phase I/II**	**Phase III TRONTIER 1**	**Phase III TRONTIER 2**
# of patients	N=210	N=800	N=800
Design	▪ **PART 1** (dose escalation): Q4W trontinemab or placebo for 28w (5 dosing cohorts) ▪ **PART 2** (expansion): Q4W trontinemab vs placebo for 28w (1.8mg/kg and 3.6mg/kg cohorts) ▪ **PART 3** (PK/PD): Q4W trontinemab vs placebo (1.8mg/kg); Q12W trontinemab vs placebo (3.6mg/kg) ▪ **PART 4** (open label extension): For all parts	▪ **Randomised 1:1 placebo-controlled study** **Induction:** Q4W trontinemab (3.6 mg/kg) (7 doses) ▪ **Maintenance:** Q12W trontinemab (3.6 mg/kg) for an additional 12 months ▪ **OLE:** All switched to trontinemab from 18 months onward	▪ **Randomised 1:1 placebo-controlled study** **Induction:** Q4W trontinemab (3.6 mg/kg) (7 doses) ▪ **Maintenance:** Q12W trontinemab (3.6 mg/kg) for an additional 12 months ▪ **OLE:** All switched to trontinemab from 18 months onward
Primary endpoint	▪ Part 1-4: Percentage of Participants with AEs ▪ Part 3: Change From baseline in brain amyloid load (via PET)	▪ Change in cognition and function based on the Clinical Dementia Rating – Sum of Boxes scale after 18 months of treatment	▪ Change in cognition and function based on the Clinical Dementia Rating – Sum of Boxes scale after 18 months of treatment
Status	▪ FPI Q1 2021 ▪ Data showing rapid, robust amyloid depletion presented at ADPD, CTAD 2024 and ADPD 2025	▪ FPI Q3 2025	▪ FPI Q3 2025
CT Identifier	NCT04639050	NCT07169578	NCT07170150

ADPD: International Conference on Alzheimer's and Parkinson's Diseases; CTAD: Clinical Trials on Alzheimer's Disease

Sefaxersen (ASO factor B, RG6299)

Antisense oligonucleotide that targets factor B

Indication	IgA nephropathy (IgAN)
Phase/study	**Phase III** **IMAGINATION**
# of patients	N=428
Design	▪ **ARM A:** Sefaxersen SC at week 1, 3, 5 following Q4W dosing for 104 weeks ▪ **ARM B:** Placebo
Primary endpoint	▪ Change in UPCR at week 37 from baseline
Status	▪ FPI Q3 2023
CT Identifier	NCT05797610

In collaboration with IONIS
UPCR: Urine protein-to-creatinine ratio; ASO: Antisense oligonucleotide

Afimkibart (anti-TL1A, RG6631)

A monoclonal antibody targeting TL1A, blocking TH1 and TH17 pathways

Indication	Moderate to severe ulcerative colitis	Moderate to severe ulcerative colitis
Phase/study	**Phase III** **AMETRINE-1**	**Phase III** **AMETRINE-2**
# of patients	N=400	N=350
Design	▪ **ARM A:** Afimkibart IV induction followed by afimkibart SC maintenance ▪ **ARM B:** Placebo IV followed by placebo SC maintenance	▪ **ARM A:** Afimkibart IV induction ▪ **ARM B:** Placebo IV
Primary endpoint	▪ Modified Mayo Score 0–2 (Stool Frequency Subscore = 0 or 1, Rectal Bleeding Subscore = 0, Endoscopic subscore = 0 or 1) at week 12 or week 52	▪ Modified Mayo Score 0–2 (Stool Frequency Subscore = 0 or 1, Rectal Bleeding Subscore = 0, Endoscopic subscore = 0 or 1) at week 12
Status	▪ FPI Q3 2024 ▪ Recruitment completed Q1 2026	▪ FPI Q4 2024
CT Identifier	NCT06589986	NCT06588855

TL1A: Tumor necrosis factor-like cytokine 1A; TH: T helper cell

Afimkibart (anti-TL1A, RG6631)

A monoclonal antibody targeting TL1A, blocking TH1 and TH17 pathways

Indication	Moderate to severe Crohn's Disease	
Phase/study	**Phase III SIBERITE-1**	**Phase III SIBERITE-2**
# of patients	N=600	N=425
Design	▪ Treat-through design with no re-randomization after induction ▪ **ARM A:** Afimkibart IV induction followed by SC maintenance (high dose) ▪ **ARM B:** Afimkibart IV induction followed by SC maintenance (low dose) ▪ **ARM C:** Placebo IV followed by placebo SC maintenance	▪ Induction only ▪ **ARM A:** Afimkibart IV induction ▪ **ARM B:** Placebo IV
Primary endpoint	▪ Co-primary endpoints: ▪ Clinical remission (CDAI <150) at w52 ▪ Decrease in SES-CD from BL ≥50% at w52	▪ Co-primary endpoints: ▪ Clinical remission (CDAI <150) at w12 ▪ Decrease in SES-CD from BL ≥50% at w12
Status	▪ FPI Q1 2025	▪ FPI Q2 2025
CT Identifier	NCT06819878	NCT06819891

TL1A: Tumor necrosis factor-like cytokine 1A; CDAI: Crohn's Disease Activity Index; SES-CD: Simple Endoscopic Score for Crohn's Disease

Afimkibart (anti-TL1A, RG6631)

A monoclonal antibody targeting TL1A, blocking TH1 and TH17 pathways

Indication	Atopic dermatitis	Metabolic Dysfunction-associated Steatohepatitis (MASH)	Rheumatoid Arthritis (RA)
Phase/study	**Phase II VELARITE**	**Phase Ib FIBROLITE**	**Phase IIb dRAvite**
# of patients	N=160	N=50	N=160
Design	▪ **ARM A:** High dose afimkibart SC ▪ **ARM B:** Med dose afimkibart SC ▪ **ARM C:** Low dose afimkibart SC ▪ **ARM D:** Placebo	▪ Afimkibart IV at w0, w2, w6, w10 + afimkibart SC from w14-50	▪ **ARM A:** High dose afimkibart SC ▪ **ARM B:** Low dose afimkibart SC ▪ **ARM C:** Placebo
Primary endpoint	▪ Percentage achieving EASI-75 Response (≥75% Improvement from baseline) at week 16	▪ Percentage of participants with AEs from baseline to w52	▪ Change from Baseline in Disease Activity Score-28 for Rheumatoid Arthritis with C-Reactive Protein (DAS28-CRP) at week 14
Status	▪ FPI Q2 2025 ▪ Recruitment completed Q1 2026	▪ FPI Q2 2025 ▪ Recruitment completed Q3 2025	▪ FPI Q4 2025
CT Identifier	NCT06863961	NCT06903065	NCT07137598

TL1A: Tumor necrosis factor-like cytokine 1A; EASI-75: Eczema Area and Severity Index-75

Vamikibart (anti-IL-6, RG6179)

A monoclonal antibody that potently binds interleukin-6 (IL-6) cytokine

Indication	Uveitic macular edema (UME)	
Phase/study	**Phase III MEERKAT**	**Phase III SANDCAT**
# of patients	N=225	N=225
Design	▪ **ARM A:** Vamikibart low-dose Q4W to week 12, followed by PRN ▪ **ARM B:** Vamikibart high-dose Q4W to week 12, followed by PRN ▪ **ARM C:** Sham control Q4W to week 12, followed by PRN	▪ **ARM A:** Vamikibart low-dose Q4W to week 12, followed by PRN ▪ **ARM B:** Vamikibart high-dose Q4W to week 12, followed by PRN ▪ **ARM C:** Sham control Q4W to week 12, followed by PRN
Primary endpoint	▪ Proportion of participants with ≥ 15 letter improvement from baseline in BCVA at week 16	▪ Proportion of participants with ≥ 15 letter improvement from baseline in BCVA at week 16
Status	▪ FPI Q1 2023 ▪ Recruitment completed Q2 2024 ▪ Study met primary endpoint Q1 2025 ▪ Data presented at AAO 2025	▪ FPI Q1 2023 ▪ Recruitment completed Q4 2024 ▪ Study did not meet primary endpoint Q3 2025, but nominally significant improvements seen across primary and key secondary endpoints ▪ Data presented at AAO 2025
CT Identifier	NCT05642312	NCT05642325

AAO: American Academy of Ophthalmology; BCVA: Best corrected visual acuity; PRN: Pro re nata

Enicepatide (CT-388, RG6640)

Once-weekly subcutaneous injectable dual GLP-1/GIP receptor agonist

Indication	Overweight/obesity without type 2 diabetes	
Phase/study	**Phase II**	**Phase III Enith 1**
# of patients	N=450	N=2000
Design	▪ enicepatide (low/med/high dose) vs. placebo	▪ Arm 1: Placebo Arm 2: CT-388 dosing regimen 1 Arm 3: CT-388 dosing regimen 2 Arm 4: CT-388 dosing regimen 3
Primary endpoint	▪ Efficacy: Percent change in body weight from baseline	▪ Percent change in body weight from baseline at week 72
Status	▪ FPI Q3 2024 ▪ Recruitment completed Q4 2024 ▪ Study met primary endpoint January 2026 ▪ Presented at ADA 2026	▪ FPI Q1 2026
CT Identifier	NCT06525935	NCT07351045

GLP-1: Glucagon-like peptide-1; GIP: Glucose-dependent insulinotropic polypeptide

Enicepatide (CT-388, RG6640)

Once-weekly subcutaneous injectable dual GLP-1/GIP receptor agonist

Indication	Overweight/obesity with type 2 diabetes	
Phase/study	**Phase II**	**Phase III Enith 2**
# of patients	N=360	N=1600
Design	▪ enicepatide (low/med/high dose) vs. placebo	▪ Arm 1: Placebo Arm 2: Enicepatide dosing regimen 1 Arm 3: Enicepatide dosing regimen 2 Arm 4: Enicepatide dosing regimen 3
Primary endpoint	▪ Percent Change in Body Weight from baseline ▪ Change in Glycated Hemoglobin (HbA1c) from baseline	▪ Percent change in body weight from baseline at week 72
Status	▪ FPI Q4 2024 ▪ Recruitment completed Q4 2025	▪ FPI Q1 2026
CT Identifier	NCT06628362	NCT07351058

GLP-1: Glucagon-like peptide-1; GIP: Glucose-dependent insulinotropic polypeptide

CT-996 (GLP-1 RA, RG6652)

Once-daily oral small molecule GLP-1 receptor agonist

Indication	Overweight/obesity with or without type 2 diabetes	Obesity without type 2 diabetes	Glycaemic control trial with T2D participants
Phase/study	**Phase I**	**Phase II**	**Phase II**
# of patients	N=118	N=340	N=240
Design	▪ Single ascending dose, multiple ascending dose, multiple part study, with low to high doses of CT-996 vs placebo up to 4 weeks	▪ **ARM 1:** Placebo ▪ **ARM 2-8:** CT-996 with various uptitration schedules and step sizes towards 5 different maximum dosages	▪ **ARM 1:** Placebo ▪ **ARM 2:** Commercially available incretin to be uptitrated in line with label ▪ **ARM 3-9:** CT-996 with various uptitration schedules and step sizes towards 5 different maximum dosages
Primary endpoint	▪ Safety and tolerability	▪ Percent change in body weight at week 30	▪ Percent change in HbA1c at week 30
Status	▪ FPI Q2 2023 ▪ Positive topline results at 4 weeks in people with obesity without type 2 diabetes July 2024, data presented at EASD 2024	▪ FPI Q3 2025 ▪ Recruitment completed Q3 2025	▪ FPI Q3 2025 ▪ Recruitment completed Q2 2026
CT Identifier	NCT05814107	NCT07081958	NCT07112872

EASD: European Association for the Study of Diabetes; GLP-1: Glucagon-like peptide-1; RA: Receptor agonist

Emugrobart (RG6237, GYM 329)

Recycling and antigen-sweeping monoclonal anti-latent myostatin antibody

Indication	Obesity	
Phase/study	**Phase Ib**	**Phase II GYMINDA**
# of patients	N=30-36	N=234
Design	▪ **Cohort A** (n=15-18): Single dose emugrobart 50mg SC ▪ **Cohort B** (n=15-18): Multiple dosing 180mg SC Q4W week + loading dose for first 3 doses	▪ 48w DB Tx period: **Arm A:** Tirzepatide + Placebo **Arm B:** Tirzepatide + GYM 329 (low SC Q4W) **Arm C:** Tirzepatide + GYM 329 (med SC Q4W) **Arm D:** Tirzepatide + GYM329 (high SC Q4W) Extension: Arm A-C switched to placebo Arm D re-randomized to GYM329 Q4W or placebo
Primary endpoint	▪ PK/PD, tolerability, safety	▪ Percent (%) change in body weight from baseline at Week 48
Status	▪ FPI Q2 2024	▪ FPI Q2 2025 ▪ Recruitment completed Q3 2025
CT Identifier		NCT06965413

emugrobart (GYM 329) in collaboration with Chugai

Pegozafermin (RG6881)

Fibroblast growth factor 21 (FGF21) analog

Indication	MASH F2-F3	MASH with compensated cirrhosis (F4)
Phase/study	**Phase III ENLIGHTEN-Fibrosis**	**Phase III ENLIGHTEN-Cirrhosis**
# of patients	N=1050	N=762
Design	▪ ARM A: Placebo ▪ ARM B: Pegozafermin 30mg QW ▪ ARM C: Pegozafermin 44mg Q2W	▪ ARM A: Placebo ▪ ARM B: Pegozafermin 30mg QW
Primary endpoint	▪ Co-primary Endpoints: ▪ Proportion of participants with one-point improvement in fibrosis with no worsening of MASH at 52 weeks ▪ Proportion of participants with MASH resolution with no worsening of fibrosis at 52 weeks	▪ Proportion of participants achieving fibrosis regression at 24 months
Status	▪ FPI 2024	▪ FPI 2024
CT Identifier	NCT06318169	NCT06419374

MASH: Metabolic Dysfunction-Associated Steatohepatitis

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

Pharma sales appendix

Diagnostics sales appendix

Foreign exchange rates information

pRED oncology, neurology development programs

Molecule	Indication	Phase	# of patients	Status	CT Identifier
Oncology					
CEA212Pb PRIT (RG6375)[1]	metastatic Colorectal Cancer (mCRC)	I	160	FPI expected Q3 2026	NCT07416552
PanRAS inhibitor (RG6505)	solid tumors	I	345	FPI Q2 2025	NCT06884618
Neurology					
Brainshuttle™-CD20 (RG6035)	multiple sclerosis	I	119	FPI Q3 2021	ISRCTN16295177 NCT05704361
nivegacetor (gamma-secretase modulator, RG6289)	Alzheimer's disease	II	255	FPI Q2 2024 Recruitment completed Q2 2025	NCT06402838
MAGL inhibitor (RG6182)	multiple sclerosis	I	Up to 36	FPI Q3 2023	
		II	360	FPI Q1 2026	NCT07282574
NLRP3 inhibitor (RG6434)	neurodegenerative disorders	I	~93	FPI Q4 2024	
HTT miRNA GT (RG6662)	Huntington's disease	I	part A: 8 part B:45	FPI Q2 2025	NCT06826612
APOE ASO (RG6627)[2]	Alzheimer's disease	I	38	FPI Q1 2026	NCT07234942

Partner: [1]In collaboration with Orano Med, [2] Ionis

pRED immunology, ophthalmology and CVRM development programs

Molecule	Indication	Phase	# of patients	Status	CT Identifier
Immunology					
NME (RG6498)	IBD	I		FPI Q1 2026	
anti-TSLP:IL33 (RG6981)[1]	COPD/Asthma	I	120	FPI Q1 2026	ISRCTN22936350
Ophthalmology					
VEGF x IL6 DutaFab (RG6209)	DME	I	176	FPI Q4 2022 LPI Q4 2025	NCT06847854 (PREGONDA)
NME (RG6327)	geographic atrophy	I	27	FPI Q3 2025	NCT06961370
NME (RG6441)	DR	I	30	FPI Q2 2026	NCT07588100
CVRM					
NME (RG6622)	coronary artery disease	I		FPI Q1 2026	
Other					
zosurabalpin (Abx MCP, RG6006)	A. baumannii infections	I	204	FPI Q4 2020	NCT04605718

Partner: [1]Qyuns Therapeutics
Abx MCP: Antibiotic macrocyclic peptide

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

Pharma sales appendix

Diagnostics sales appendix

Foreign exchange rates information

gRED oncology development programs

Molecule	Indication	Phase	# of patients	Status	CT Identifier
Oncology					
autogene cevumeran (Individualized Neoantigen-Specific Therapy (iNeST); RG6180)[1]	adjuvant PDAC	II	260	FPI Q4 2023	NCT05968326 (IMcode003)
CDK4/2i (RG6794, GDC-4198)	HR+ HER2- Breast cancer	Ib/II	~285	FPI Q4 2025	NCT07100106
CDK4i (RG6795, GDC-0587)	HR+ HER2- Breast cancer	I	120-140	FPI Q1 2026	NCT07214662
enzelkitug (anti-CCR8; RG6411)	solid tumors	I	163	FPI Q4 2022	NCT05581004
AR degrader (RG6537, GDC-2992)[2]	mCRPC	I	~160	FPI Q2 2023	NCT05800665
KRAS G12D inhibitor (RG6620)	solid tumors with KRAS G12D mutations	I	410	FPI Q4 2024	NCT06619587
P-CD19 x CD20-ALLO1 CAR-T (RG6540)	NHL	I	120	FPI Q2 2024	NCT06014762
NME (RG6735)	heme tumors	I	18	FPI Q1 2026	
NME (RG6852, GDC-1261)	mCRPC	I	~260	FPI Q1 2026	NCT07567846
NME (RG6733)	MM	I	80-120	FPI Q2 2026	NCT07558915

Partner: [1] BioNTech, [2] Jeyou

gRED neurology, immunology, ophthalmology development programs

Molecule	Indication	Phase	# of patients	Status	CT Identifier
Immunology					
TMEM16A potentiator (RG6421, GDC-6988)	muco-obstructive respiratory disease	Ic	128	FPI Q4 2024	NCT06603246
p40 x TL1A bispecific (RG6730)[1]	ulcerative colitis	IIb	224	FPI Q3 2025	NCT06979336 (SUNCREST)
P-CD19xCD20-ALLO1 CAR-T (RG6540)	systemic lupus erythematosus	I	162	FPI expected H2 2026	NCT06984341
Neurology					
P-CD19xCD20-ALLO1 CAR-T (RG6540)	Multiple sclerosis	I	~60	FPI Q4 2025	NCT07008378
Ophthalmology					
OpRegen (RG6501)[2]	geographic atrophy	IIa	60	FPI Q1 2023	NCT05626114
NME (RG6697)	DME	I	66	FPI Q2 2026	NCT07425522
CVRM					
selnoflast (NLRP3i, RG6418)	coronary artery disease with high inflammatory risk	IIa	162	FPI Q2 2026	NCT07448038

Partner: [1.] Pfizer; [2.] Lineage Cell Therapeutics

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

Pharma sales appendix

Diagnostics sales appendix

Foreign exchange rates information

HY 2026: Pharma Division sales

Top 20 products

	Global		US		Europe		Japan		International	
	CHFm	% CER	CHFm	% CER	CHFm	% CER	CHFm	% CER	CHFm	% CER
Ocrevus	3,470	7	2,300	2	758	10	-	-	412	30
Hemlibra	2,492	11	1,313	9	502	4	171	9	506	26
Vabysmo	2,059	8	1,333	1	386	5	75	26	265	65
Tecentriq	1,704	6	815	9	410	-3	146	-2	333	17
Xolair	1,673	27	1,673	27	-	-	-	-	-	-
Perjeta	1,389	-7	551	-11	238	-14	24	-23	576	0
Phesgo	1,318	18	318	0	425	9	88	15	487	47
Actemra / RoActemra	1,074	-9	515	-9	238	-21	140	8	181	-3
Evrysdi	968	17	323	15	335	17	39	-1	271	25
Kadcyla	916	-6	310	-14	248	-4	39	2	319	3
Polivy	808	20	381	28	128	-18	98	16	201	47
Alecensa	795	6	261	4	126	-4	93	8	315	13
MabThera	609	5	402	14	62	-9	6	-11	139	-10
TNKase / Activase	555	11	531	11	-	-	-	-	24	12
Gazyva	489	7	240	4	117	-1	17	14	115	21
Herceptin	462	-12	94	-14	136	-7	2	-36	230	-14
Avastin	414	-15	125	-12	35	37	36	-44	218	-13
Pulmozyme	204	-8	145	-5	29	-11	-	26	30	-18
Columvi	196	70	91	28	53	116	-	-	52	173
Enspryng	195	22	50	20	23	20	79	18	43	37
Pharma Division	**23,629**	**6**	**12,229**	**6**	**4,520**	**1**	**1,360**	**11**	**5,520**	**10**

Ranking based on reported CHF values; CER: Constant exchange rates (avg. full year 2025)

HY 2026: Pharma Division sales

Products launched since 2015

	Global		US		Europe		Japan		International	
	CHFm	% CER	CHFm	% CER	CHFm	% CER	CHFm	% CER	CHFm	% CER
Cotellic	23	2	11	7	5	-9	-	-	7	2
Alecensa	795	6	261	4	126	-4	93	8	315	13
Tecentriq	1,704	6	815	9	410	-3	146	-2	333	17
Ocrevus	3,470	7	2,300	2	758	10	-	-	412	30
Hemlibra	2,492	11	1,313	9	502	4	171	9	506	26
Luxturna	22	-32	22	-32	-	-	-	-	-	-
Xofluza	74	-58	22	37	2	24	-	-	50	-69
Polivy	808	20	381	28	128	-18	98	16	201	47
Rozlytrek	66	-10	16	-33	12	10	2	-18	36	1
Enspryng	195	22	50	20	23	20	79	18	43	37
Phesgo	1,318	18	318	0	425	9	88	15	487	47
Evrysdi	968	17	323	15	335	17	39	-1	271	25
Susvimo	9	109	9	109	-	-	-	-	-	-
Vabysmo	2,059	8	1,333	1	386	5	75	26	265	65
Lunsumio	100	130	65	136	9	-17	21	348	5	500
Columvi	196	70	91	28	53	116	-	-	52	173
Elevidys	180	62	-	-	2	-	22	-	156	39
PiaSky	30	62	-	-	7	140	22	47	1	-7
Itovebi	104	212	81	164	7	*	-	-	16	*
Total	**14,613**	**11**	**7,411**	**7**	**3,190**	**6**	**856**	**16**	**3,156**	**26**

* Over 500%; Ranking based on reported CHF values; CER: Constant exchange rates (avg. full year 2025)

HY 2026: Pharma Division sales

Product sales Pharmaceuticals Division

	Global		US		Europe		Japan		International	
	CHFm	% CER	CHFm	% CER	CHFm	% CER	CHFm	% CER	CHFm	% CER
Ocrevus	3,470	7	2,300	2	758	10	-	-	412	30
Hemlibra	2,492	11	1,313	9	502	4	171	9	506	26
Vabysmo	2,059	8	1,333	1	386	5	75	26	265	65
Tecentriq	1,704	6	815	9	410	-3	146	-2	333	17
Xolair	1,673	27	1,673	27	-	-	-	-	-	-
Perjeta	1,389	-7	551	-11	238	-14	24	-23	576	0
Phesgo	1,318	18	318	0	425	9	88	15	487	47
Actemra / RoActemra	1,074	-9	515	-9	238	-21	140	8	181	-3
Evrysdi	968	17	323	15	335	17	39	-1	271	25
Kadcyla	916	-6	310	-14	248	-4	39	2	319	3
Polivy	808	20	381	28	128	-18	98	16	201	47
Alecensa	795	6	261	4	126	-4	93	8	315	13
MabThera	609	5	402	14	62	-9	6	-11	139	-10
TNKase / Activase	555	11	531	11	-	-	-	-	24	12
Gazyva	489	7	240	4	117	-1	17	14	115	21
Herceptin	462	-12	94	-14	136	-7	2	-36	230	-14
Avastin	414	-15	125	-12	35	37	36	-44	218	-13
Pulmozyme	204	-8	145	-5	29	-11	-	26	30	-18
Columvi	196	70	91	28	53	116	-	-	52	173
Enspryng	195	22	50	20	23	20	79	18	43	37
Elevidys	180	62	-	-	2	-	22	-	156	39
Itovebi	104	212	81	164	7	*	-	-	16	*
Lunsumio	100	130	65	136	9	-17	21	348	5	500
Xofluza	74	-58	22	37	2	24	-	-	50	-69
Rozlytrek	66	-10	16	-33	12	10	2	-18	36	1
Piasky	30	62	-	-	7	140	22	47	1	-7
Cotellic	23	2	11	7	5	-9	-	-	7	2
Luxturna	22	-32	22	-32	-	-	-	-	-	-
Susvimo	9	109	9	109	-	-	-	-	-	-
Other Products	1,231	-8	232	-22	227	0	240	26	532	-15
Pharma Division	**23,629**	**6**	**12,229**	**6**	**4,520**	**1**	**1,360**	**11**	**5,520**	**10**

* Over 500%; CER: Constant exchange rates (avg. full year 2025)

Pharma Division CER sales growth in %

Global top 20 products

	Q1/25	Q2/25	Q3/25	Q4/25	Q1/26	Q2/26
Ocrevus	6	10	6	16	6	7
Hemlibra	11	22	4	8	13	9
Vabysmo	18	19	4	7	13	3
Tecentriq	0	-1	4	8	4	9
Xolair	26	41	34	26	26	28
Perjeta	-10	-14	-14	-13	-11	-3
Phesgo	52	57	51	34	27	9
Actemra / RoActemra	-1	8	-2	-10	-4	-14
Evrysdi	18	-1	10	26	19	16
Kadcyla	5	12	7	6	-4	-7
Polivy	42	51	30	31	26	14
Alecensa	11	6	8	1	12	1
MabThera	-16	1	3	-3	10	0
TNKase / Activase	-2	-7	1	-2	-2	24
Gazyva	15	12	13	15	10	4
Herceptin	-20	-23	-15	-29	-12	-12
Avastin	-15	-19	-11	-21	-9	-20
Pulmozyme	10	13	26	1	-18	3
Columvi	138	58	71	64	77	64
Enspryng	27	27	25	17	31	15

Q1-Q4/25 vs. Q1-Q4/24 at CER avg. full year 2024 and Q1-Q2/26 vs. Q1-Q2/25 at CER avg. full year 2025; CER: Constant exchange rates

Pharma Division CER sales growth in %

Top 20 products by region

	US				Europe				Japan				International			
	Q3	Q4	Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4	Q1	Q2
Ocrevus	1	17	3	2	12	14	9	11	-	-	-	-	35	11	24	37
Hemlibra	-1	6	15	4	14	12	5	3	11	5	18	3	9	18	14	39
Vabysmo	-4	-1	4	-2	9	13	0	10	17	15	39	16	111	118	126	24
Tecentriq	2	4	6	12	0	7	-6	-1	-1	-6	0	-4	17	25	14	20
Xolair	34	26	26	28	-	-	-	-	-	-	-	-	-	-	-	-
Perjeta	-6	4	-12	-10	-14	-7	-15	-12	-27	-31	-19	-26	-21	-30	-9	10
Phesgo	29	22	2	-2	9	9	7	10	41	11	23	8	213	122	78	19
Actemra / RoActemra	-6	-19	-3	-13	-1	-7	-16	-27	4	6	9	7	7	6	5	-10
Evrysdi	18	0	15	15	7	21	13	21	7	-9	4	-5	4	116	40	14
Kadcyla	2	10	-11	-17	-5	0	-6	-3	-2	-7	6	-1	28	9	5	1
Polivy	18	32	26	29	21	26	-32	1	14	5	24	10	105	67	120	-7
Alecensa	15	4	-7	13	-10	-3	-5	-2	13	7	4	12	8	-3	39	-13
MabThera	5	5	16	13	13	-13	-8	-9	-3	-11	-6	-16	-5	-22	6	-24
TNKase / Activase	1	-2	-2	24	-	-	-	-	-	-	-	-	18	-4	-4	29
Gazyva	14	20	1	8	1	5	-2	-1	22	37	28	0	24	8	45	-1
Herceptin	-6	-14	-16	-13	2	-9	-7	-6	-30	-33	-24	-47	-25	-42	-14	-14
Avastin	-16	-14	-15	-8	47	-22	3	76	-19	-21	-29	-56	-13	-26	-1	-26
Pulmozyme	30	8	-21	11	-3	-9	-14	-7	13	-21	43	14	34	-19	-12	-24
Columvi	90	70	35	23	51	65	84	142	-	-	-	-	48	45	204	135
Enspryng	9	1	20	20	37	36	20	21	22	20	13	22	60	32	93	-11

Q3-Q4/25 vs. Q3-Q4/25 at CER avg. full year 2024 and Q1-Q2/26 vs. Q1-Q2/25 at CER avg. full year 2025; CER: Constant exchange rates

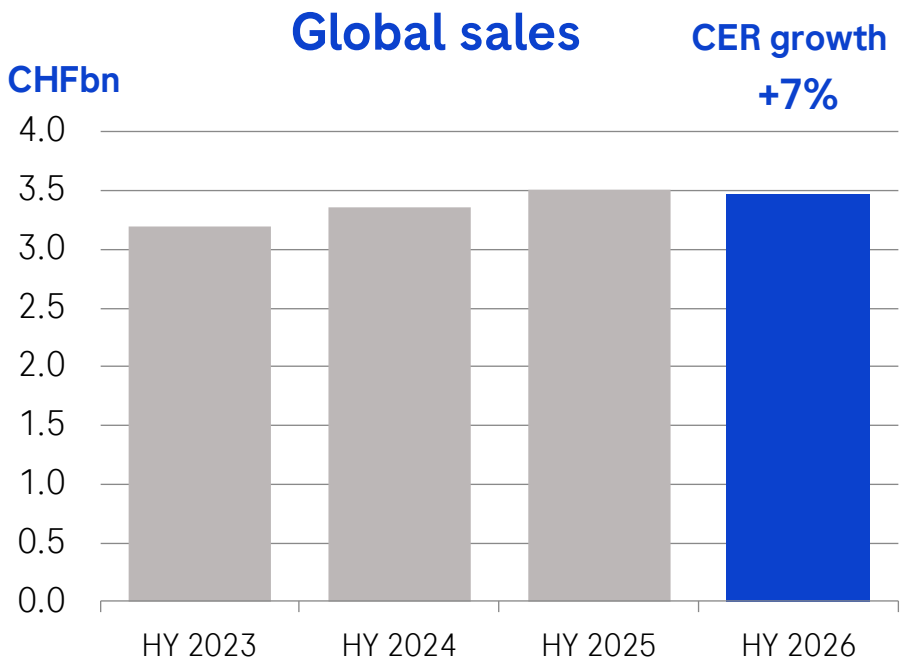
HY 2026: CER sales growth (%)

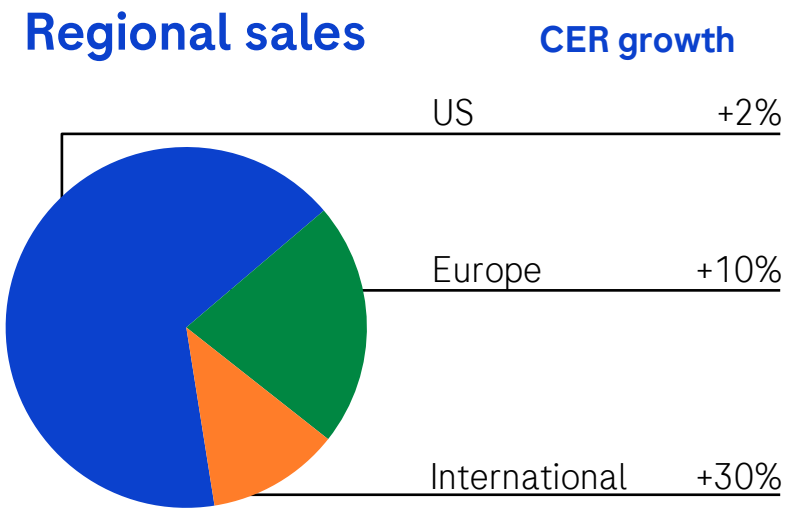
Quarterly development

	2025 vs. 2024				2026 vs. 2025	
	Q1	Q2	Q3	Q4	Q1	Q2
Pharma Division	**8**	**11**	**7**	**9**	**7**	**6**
United States	6	13	5	9	5	7
Europe	5	5	5	7	-1	3
Japan	3	7	6	4	14	9
International	18	11	11	16	16	4
Diagnostics Division	**0**	**0**	**2**	**6**	**3**	**4**
Roche Group	**6**	**8**	**6**	**8**	**6**	**5**

CER: Constant exchange rates (2025 vs. 2024 at avg. full year 2024; 2026 vs. 2025 at avg. full year 2025)

Ocrevus





HY 2026 sales of CHF 3,470m

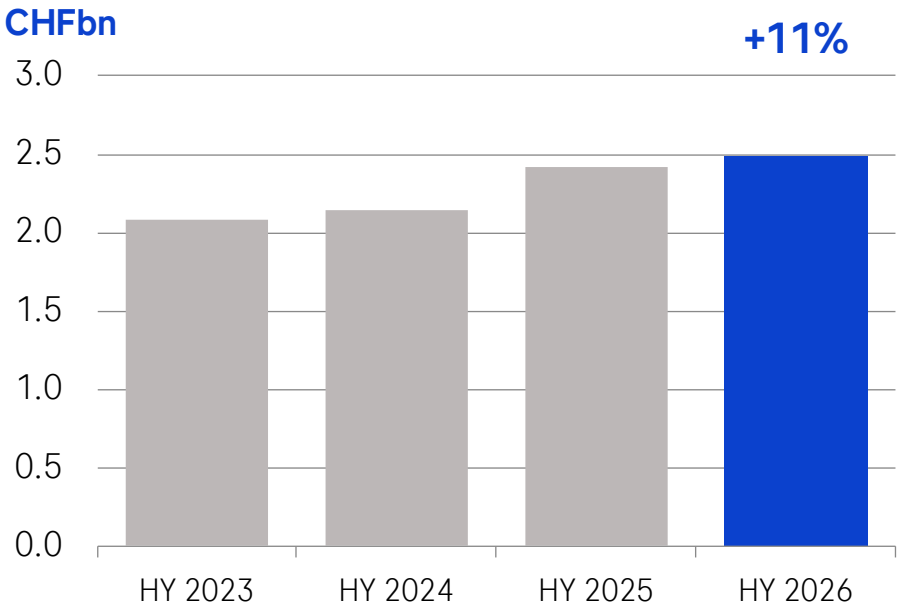
- US: Moving into earlier lines displacing orals; SC launch ongoing; #1 in US for both dynamic and total patient share
- EU: Moving into earlier lines displacing orals; SC launch ongoing

CER: Constant exchange rates

Hemlibra

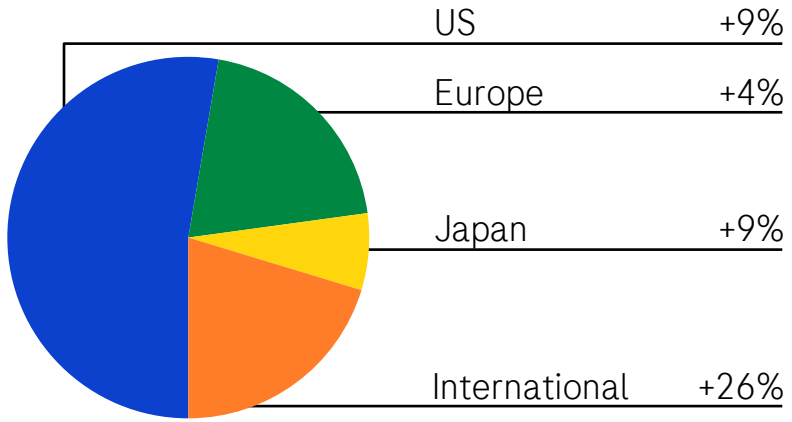
Global sales

CER growth +11%



Regional sales

CER growth

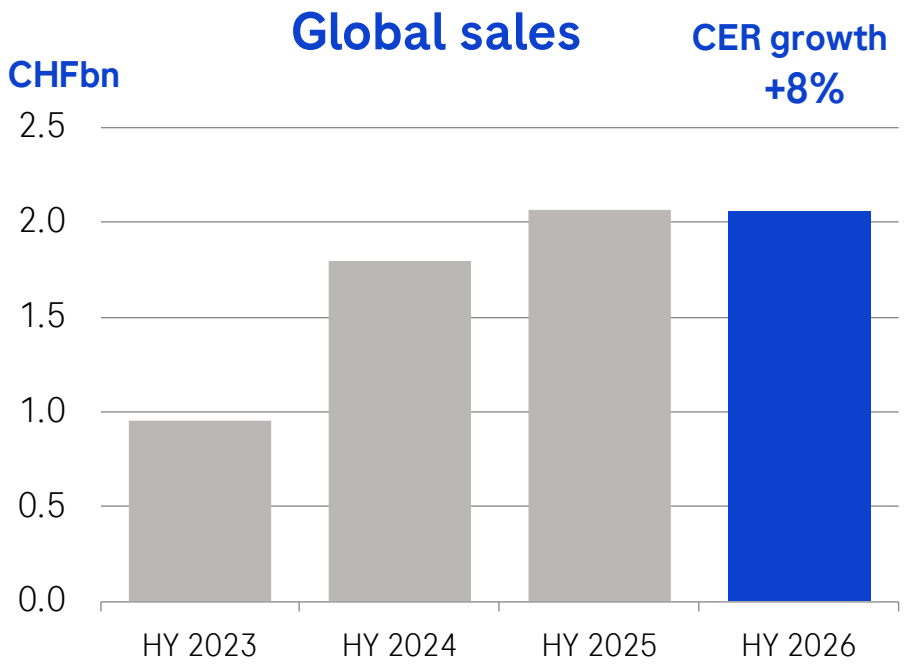


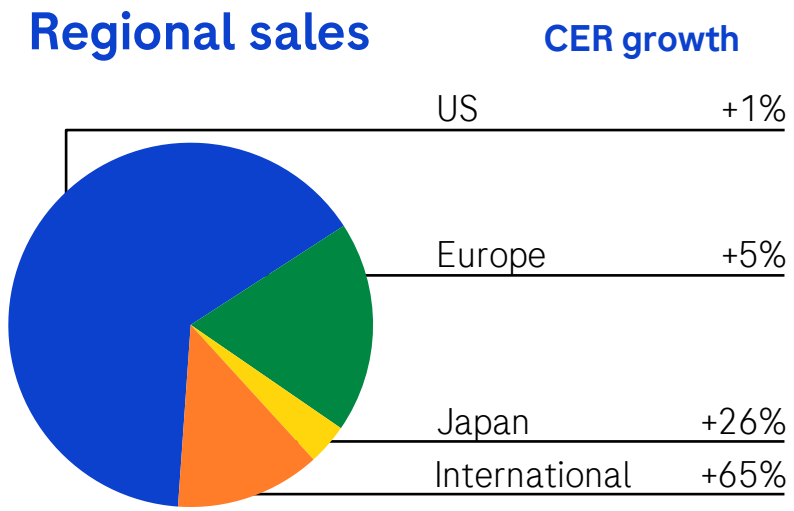
HY 2026 sales of CHF 2,492m

- US: Continued share gains in non-inhibitor patients
- EU: Continued share gains in non-inhibitor patients
- Japan: Continued uptake in non-inhibitor patients
- International: Continued uptake in non-inhibitor patients

CER: Constant exchange rates

Vabysmo





HY 2026 sales of CHF 2,059m

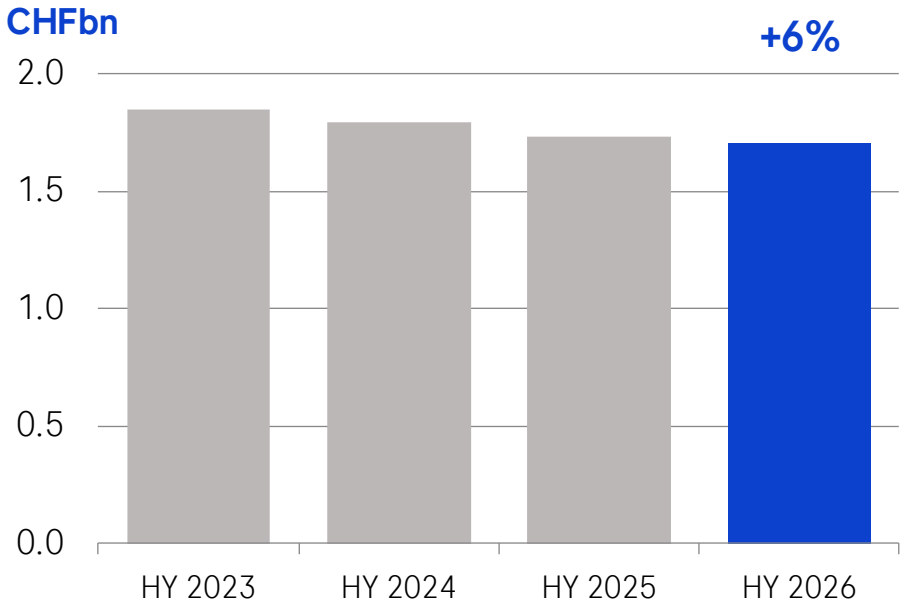
- US: Sales impacted by branded market contraction, continued market share expansion in branded IVT market
- EU: Strong volume growth impacted by price effects
- Japan: Double-digit market share and continued share growth
- International: Strong uptake in early launch countries, especially in China post NRDL

CER: Constant exchange rates

Tecentriq

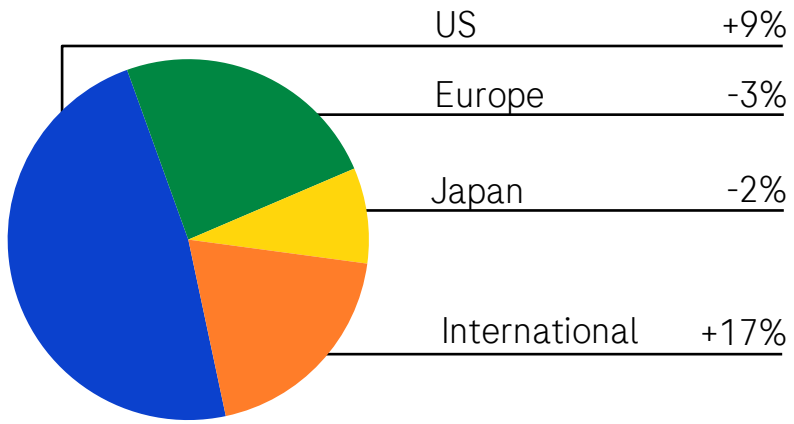
Global sales

CER growth +6%



Regional sales

CER growth

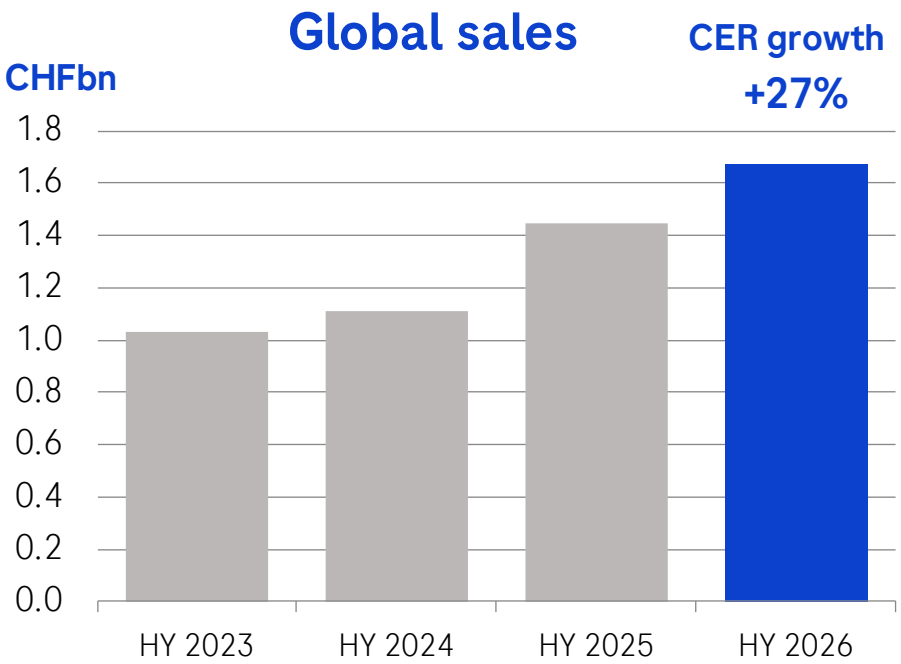


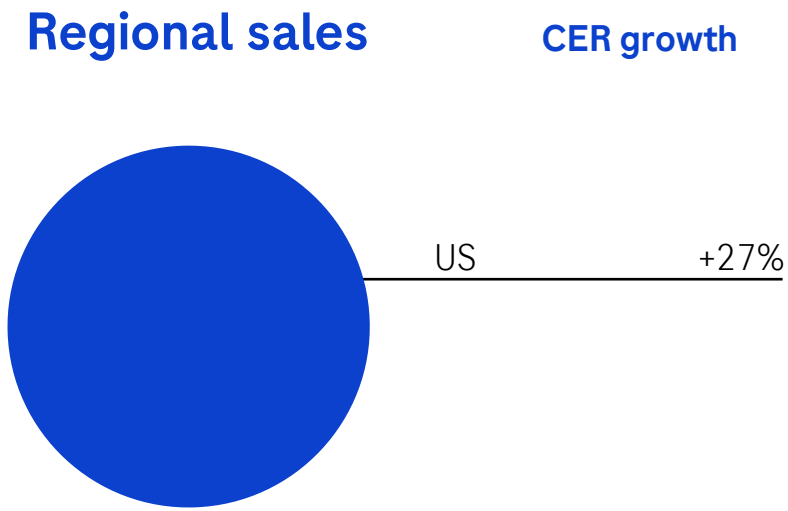
HY 2026 sales of CHF 1,704m

- US: Growth driven by new indications
- EU: Growth impacted by buying patterns in Q1
- International: Continuous growth in all regions

CER: Constant exchange rates

Xolair





HY 2026 sales of CHF 1,673m

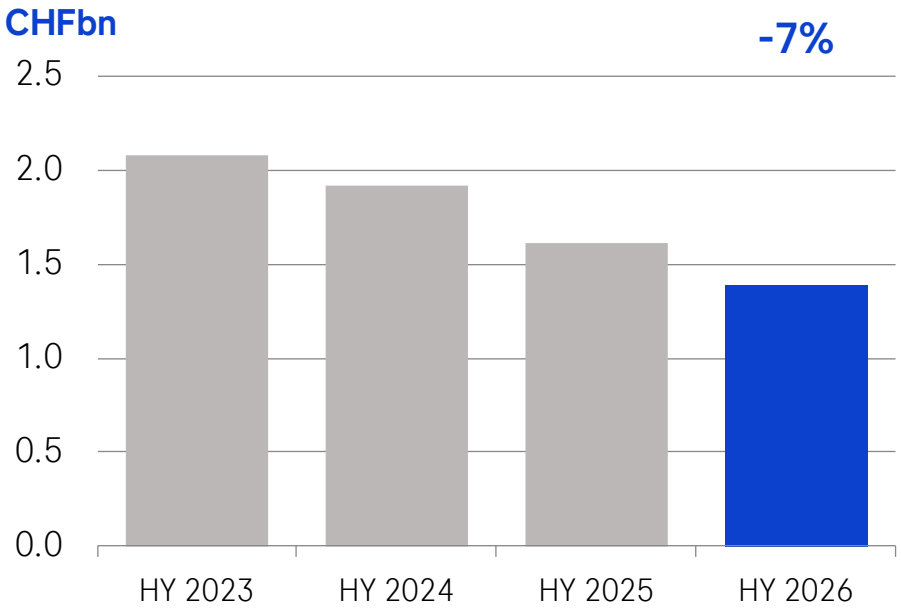
- US: Strong food allergy launch as key growth driver; Strong CSU performance driving additional growth

CER: Constant exchange rates

Perjeta

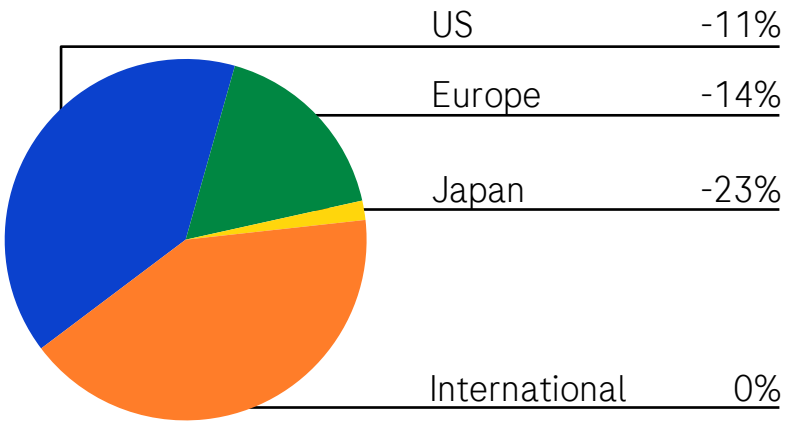
Global sales

CER growth -7%



Regional sales

CER growth



HY 2026 sales of CHF 1,389m

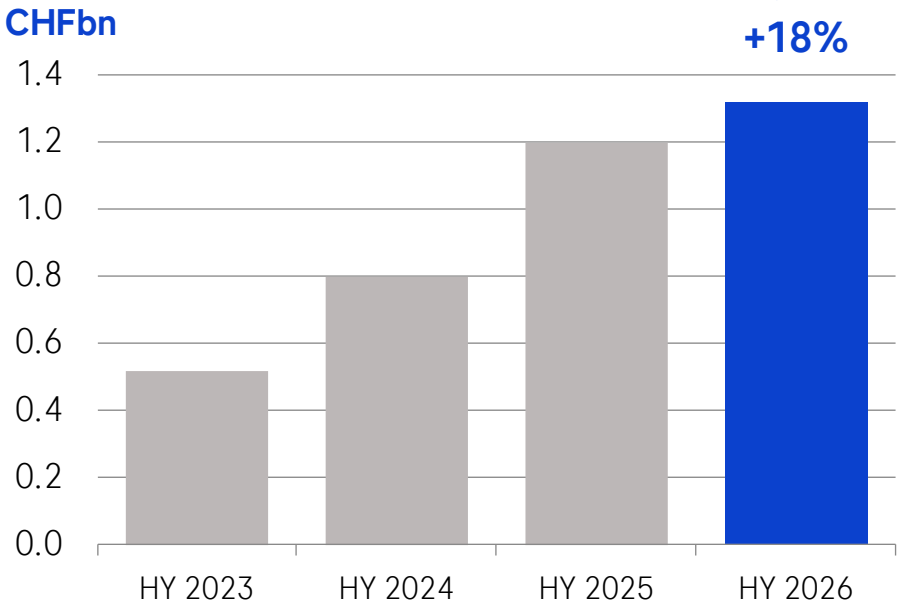
- US: Increasing conversion to Phesgo
- EU: Conversion to Phesgo
- Japan: Conversion to Phesgo
- International: Conversion to Phesgo

CER: Constant exchange rates

Phesgo

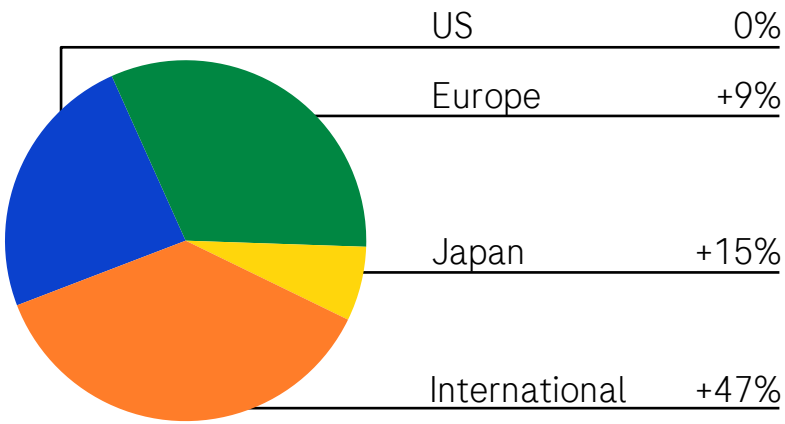
Global sales

CER growth +18%



Regional sales

CER growth



HY 2026 sales of CHF 1,318m

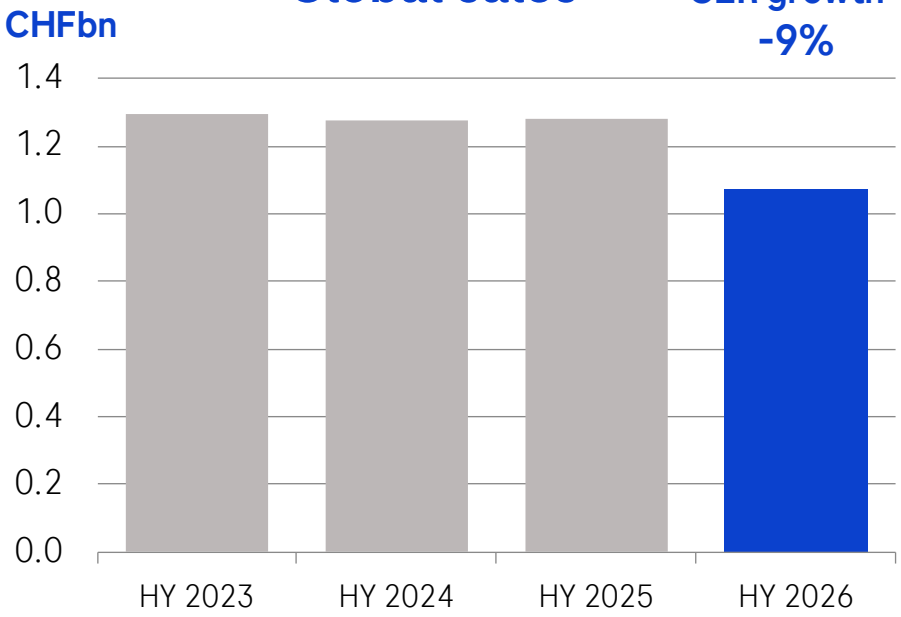
- US: Increasing conversion rates from Perjeta+Herceptin to Phesgo
- EU: Strong growth in all regions; increasing conversion rates across key markets
- International: Strong uptake in all regions

CER: Constant exchange rates

Actemra/RoActemra

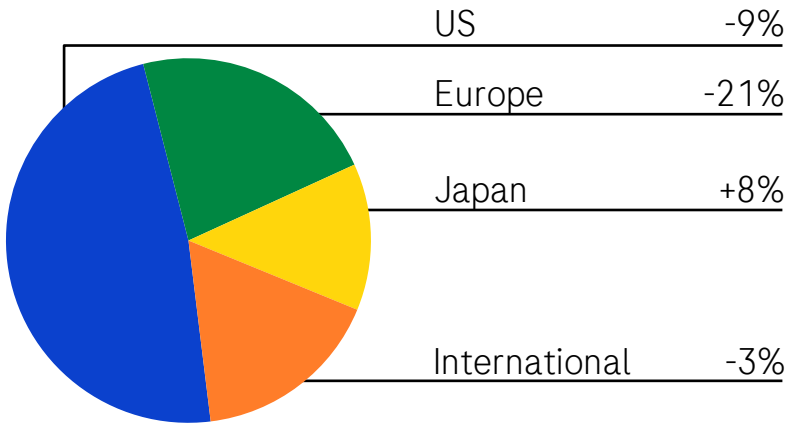
Global sales

CER growth -9%



Regional sales

CER growth



HY 2026 sales of CHF 1,074m

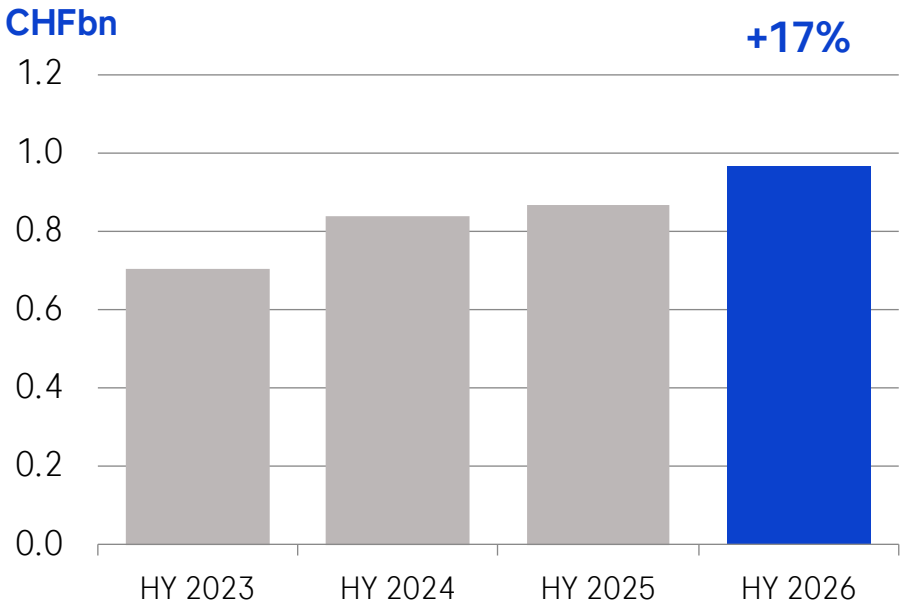
- US: Decline due to biosimilars
- EU: Decline due to biosimilars
- Japan: Maintaining top position in both patient share and market share

CER: Constant exchange rates

Evrysdi

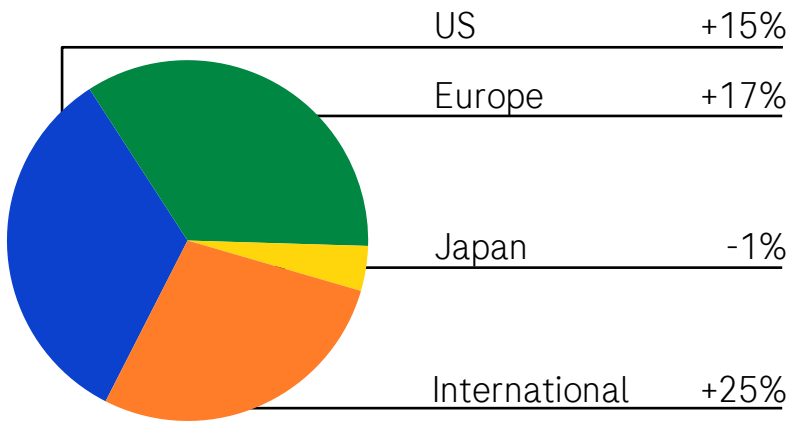
Global sales

CER growth +17%



Regional sales

CER growth

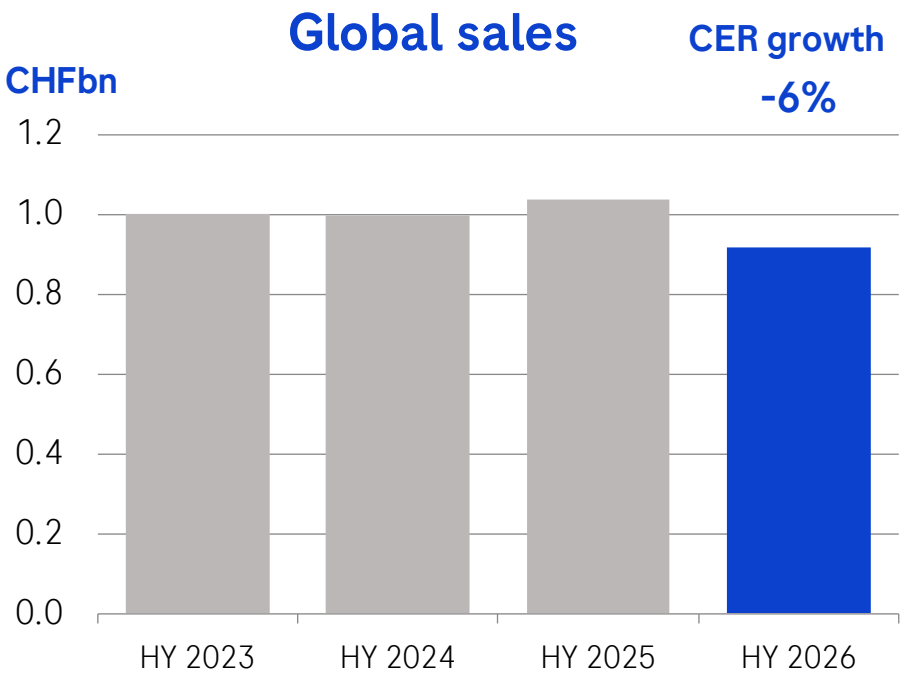


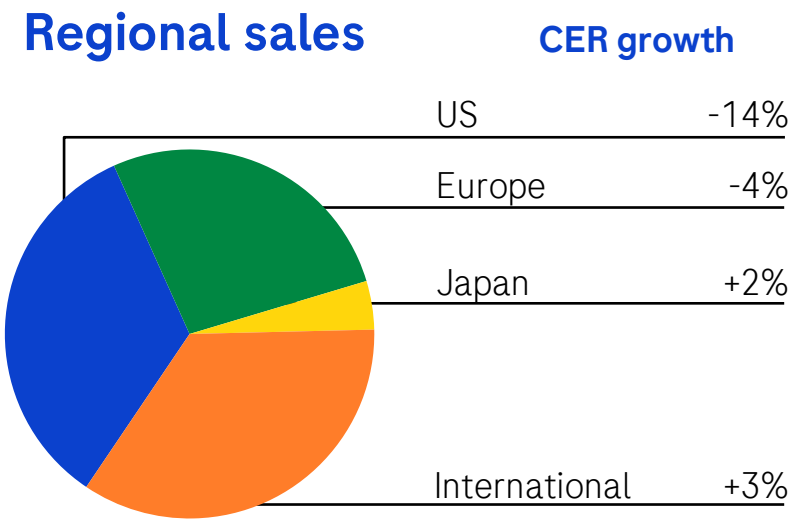
HY 2026 sales of CHF 968m

- US: Strong uptake across all patient segments; including treatment-naïve patients
- EU: Continued growth and share gains
- Japan: Market leading position
- International: Growth in HY driven by tenders

CER: Constant exchange rates

Kadcyla





HY 2026 sales of CHF 916m

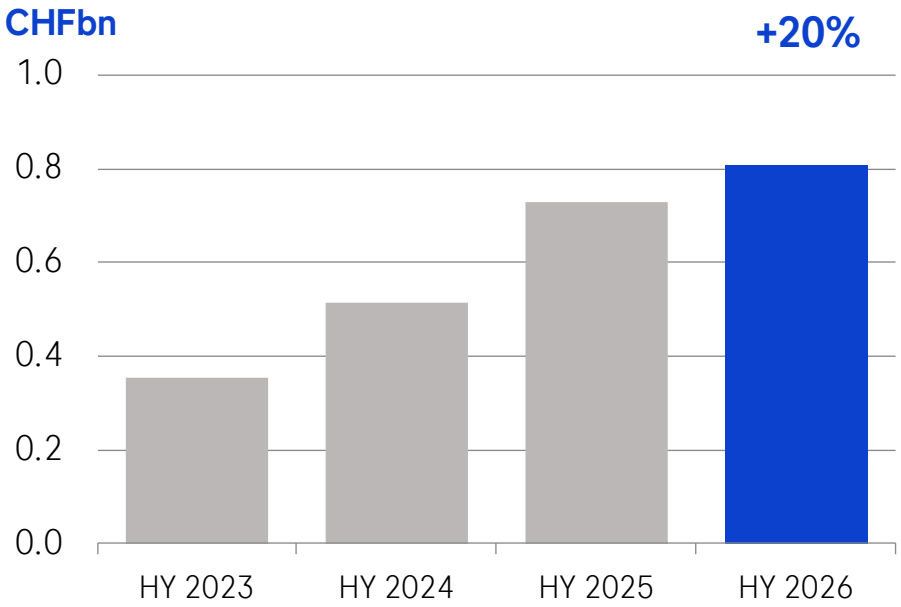
- US: Impacted by competition in metastatic BC
- EU: Share decline in metastatic BC due to competition
- Japan: Growth in adjuvant BC offsetting for share decline in metastatic BC due to competition
- International: Growth driven by uptake in adjuvant BC

CER: Constant exchange rates

Polivy

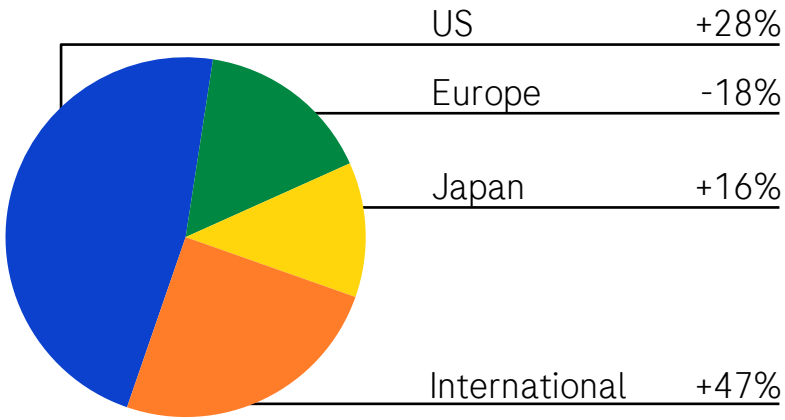
Global sales

CER growth +20%



Regional sales

CER growth



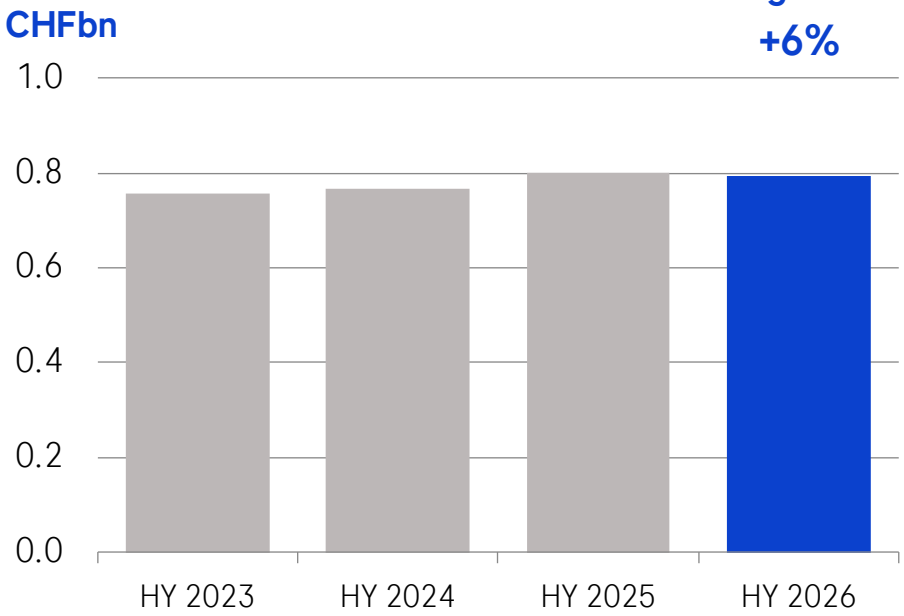
HY 2026 sales of CHF 808m

- US: Strong growth following approval in 1L DLBCL
- EU: Strong growth following approval in 1L DLBCL; Q1 impacted by one-off Q1 2025 base effect
- Japan: Strong growth following approval in 1L DLBCL; Sales impacted by price decrease
- International: Strong growth following approval in 1L DLBCL

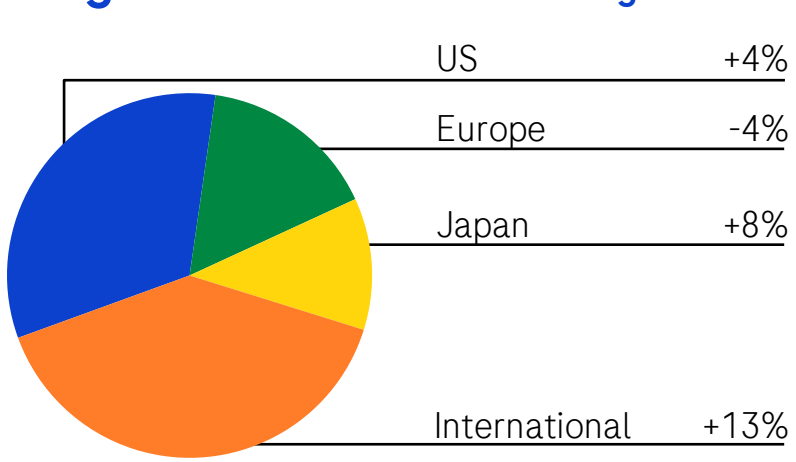
CER: Constant exchange rates

Alecensa

Global sales



Regional sales

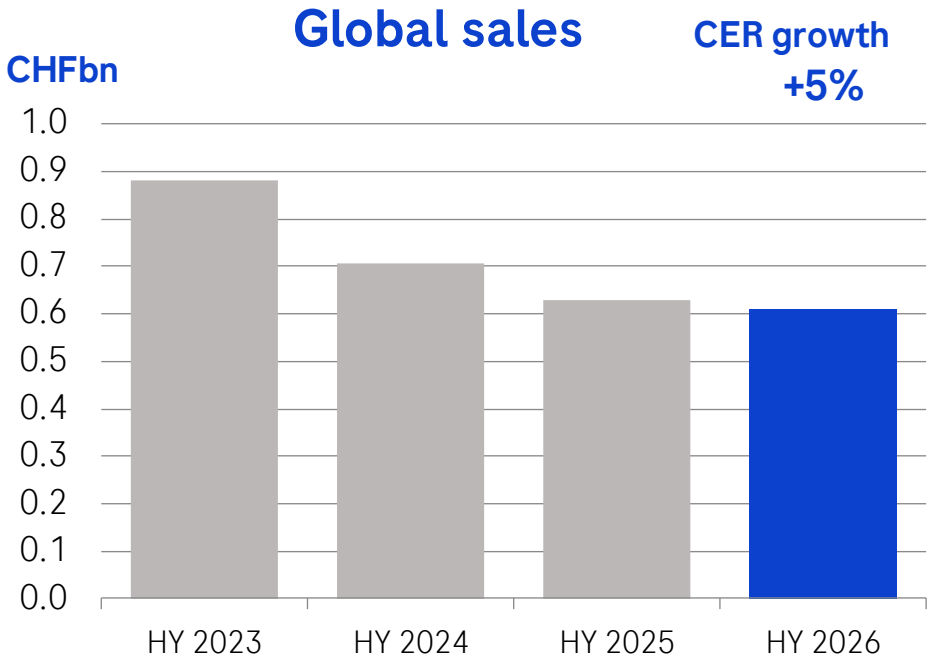


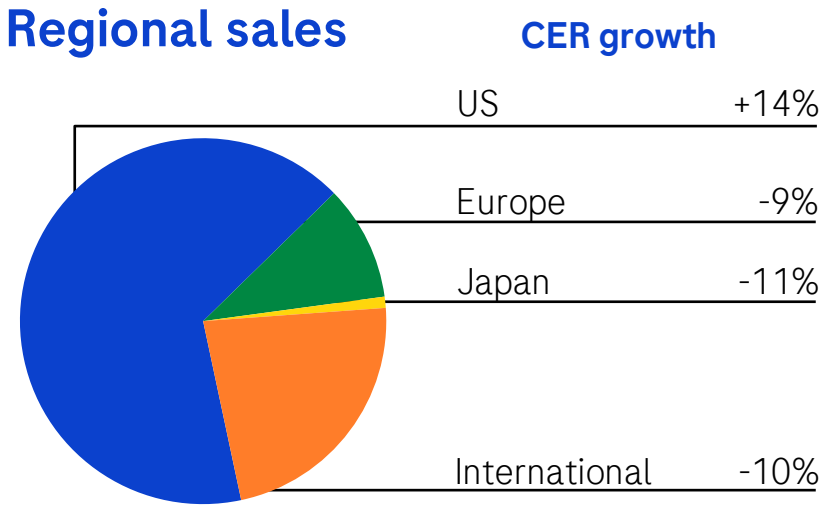
HY 2026 sales of CHF 795m

- US: Strong position in 1L ALK+ NSCLC is maintained; competitive pressure in 1L
- EU: Strong position in 1L ALK+ NSCLC is maintained; competitive pressure in 1L
- Japan: Strong position in 1L ALK+ NSCLC is maintained
- International: Strong position in 1L ALK+ NSCLC is maintained; Q1 boosted by buying patterns

CER: Constant exchange rates

MabThera/Rituxan





HY 2026 sales of CHF 609m

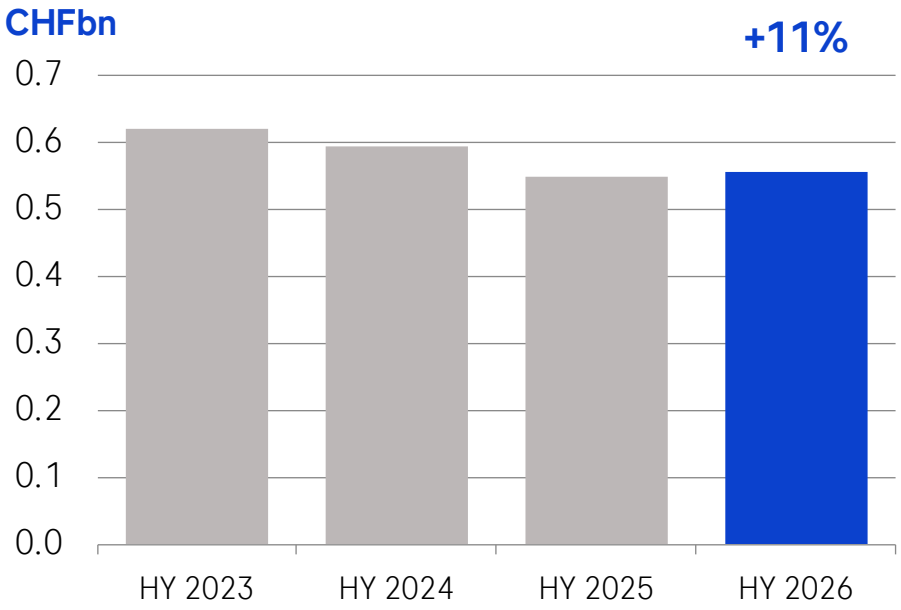
- US: Biosimilar erosion slowing; HY boosted by 2025 base effect
- EU: Biosimilar erosion bottoms out
- International: Biosimilar erosion slowing

CER: Constant exchange rates

TNKase/Activase

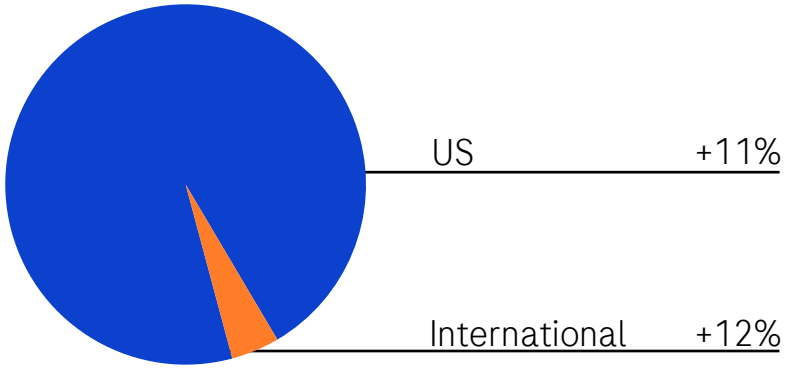
Global sales

CER growth +11%



Regional sales

CER growth

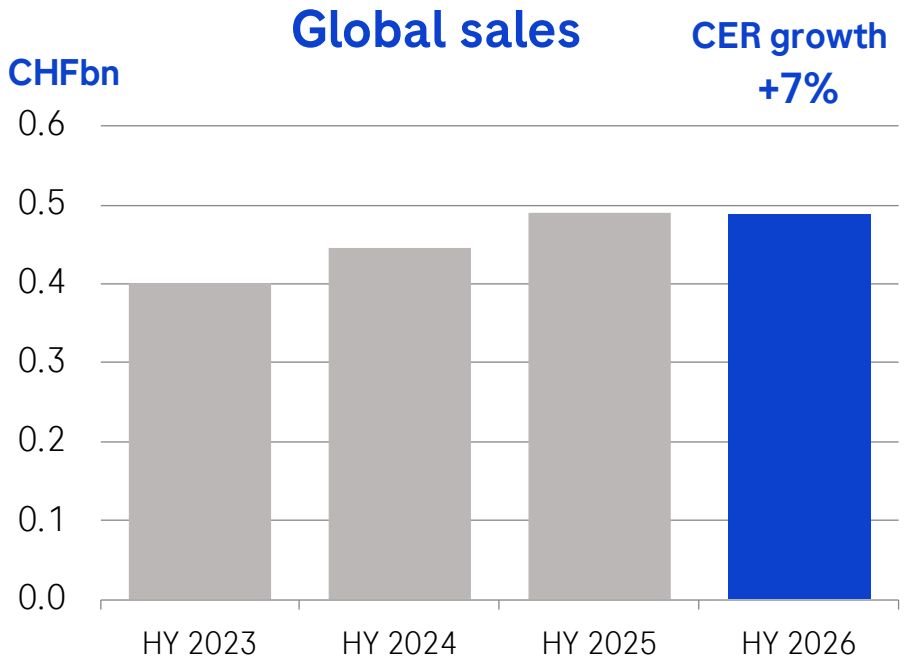


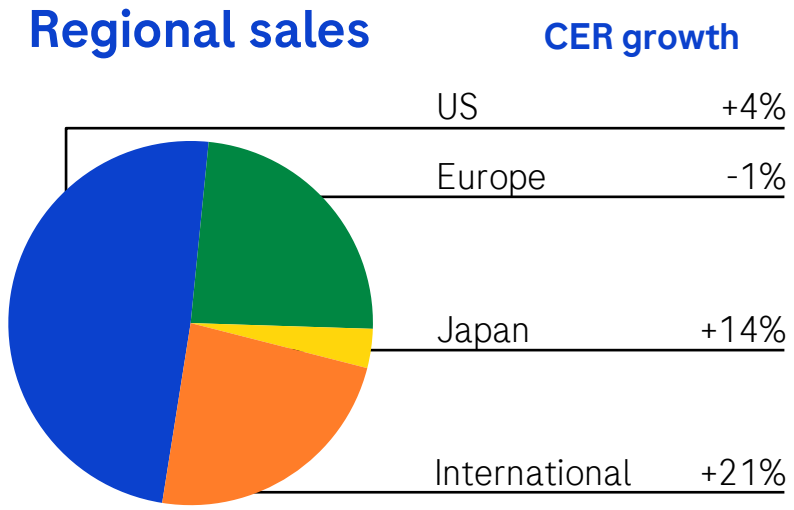
HY 2026 sales of CHF 555m

- Spontaneous TNKase use in AIS early time window; Q2 boosted by one-off buying pattern

CER: Constant exchange rates

Gazyva



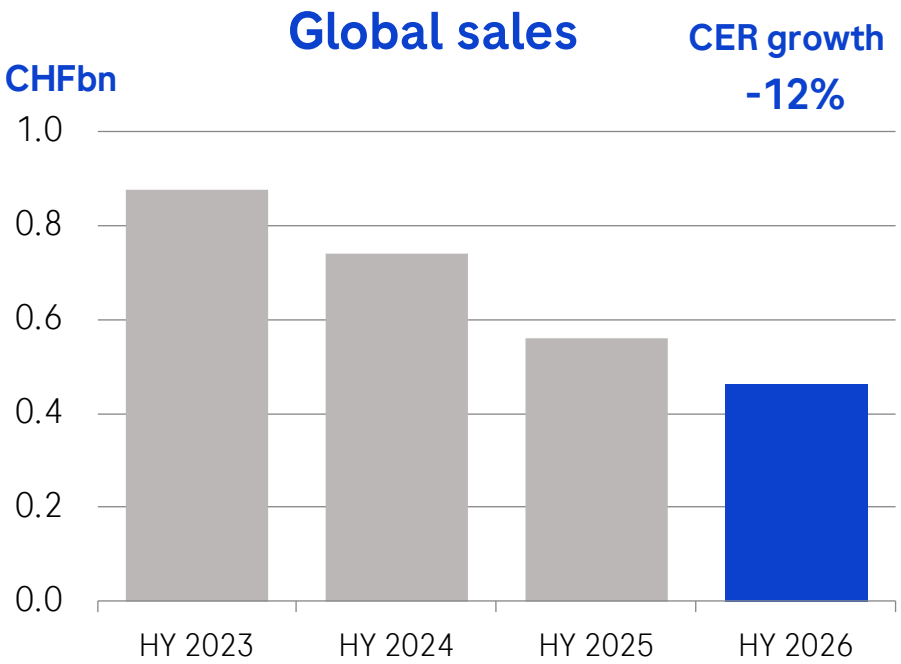


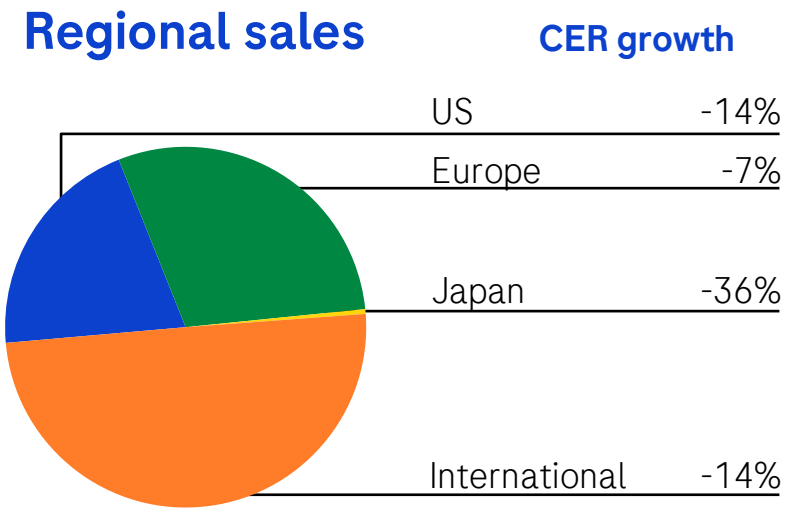
HY 2026 sales of CHF 489m

- US: Increasing competitive pressure in CLL and FL; Ongoing launch in LN
- EU: Growth in 1L CLL partially offset by price cuts in some countries
- International: Continued growth in all regions

CER: Constant exchange rates

Herceptin





HY 2026 sales of CHF 462m

- US: Biosimilar erosion slowing; Switching of patients with residual disease to Kadcyla; Conversion to Phesgo
- EU: Biosimilar erosion slowing; Switching of patients with residual disease to Kadcyla; Conversion to Phesgo
- International: Decline due to biosimilars; Conversion to Phesgo

CER: Constant exchange rates

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

Pharma sales appendix

Diagnostics sales appendix

Foreign exchange rates information

HY 2026: Diagnostics Division CER growth

By Region and Customer Area (vs. 2025)

	Global		EMEA		North America		Asia-Pacific		Latin America	
	CHFm	% CER	CHFm	% CER	CHFm	% CER	CHFm	% CER	CHFm	% CER
Core Lab	3,756	4	1,520	6	799	12	1,076	-5	361	11
Molecular Lab	1,196	3	343	5	646	4	159	-6	48	6
Near Patient Care	909	-5	376	-9	281	0	146	-11	106	6
Pathology Lab	874	11	227	10	485	14	141	-1	21	17
Diagnostics Division	**6,735**	**3**	**2,466**	**3**	**2,211**	**8**	**1,522**	**-5**	**536**	**10**

Totals may include differences due to rounding; CER: Constant exchange rates; EMEA: Europe, Middle East and Africa

HY 2026: Diagnostics Division quarterly sales and CER growth[1]

	Q2 25		Q3 25		Q4 25		Q1 26		Q2 26	
	CHFm	% CER	CHFm	% CER	CHFm	% CER	CHFm	% CER	CHFm	% CER
Core Lab	1,935	-3	1,849	0	1,926	6	1,798	4	1,958	5
Molecular Lab	616	3	611	5	666	6	571	0	625	6
Near Patient Care	482	0	459	-5	506	-2	465	-5	444	-5
Pathology Lab	435	13	429	16	442	15	419	12	455	9
Diagnostics Division	**3,468**	**0**	**3,348**	**2**	**3,540**	**6**	**3,253**	**3**	**3,482**	**4**

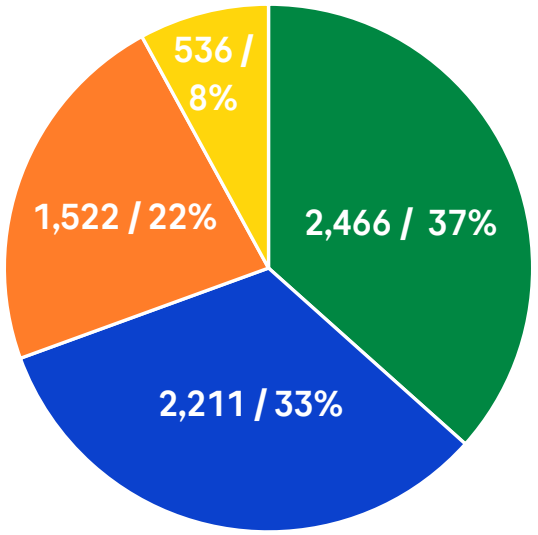
Totals may include differences due to rounding; CER: Constant exchange rates

HY 2026: Diagnostics Division regional sales

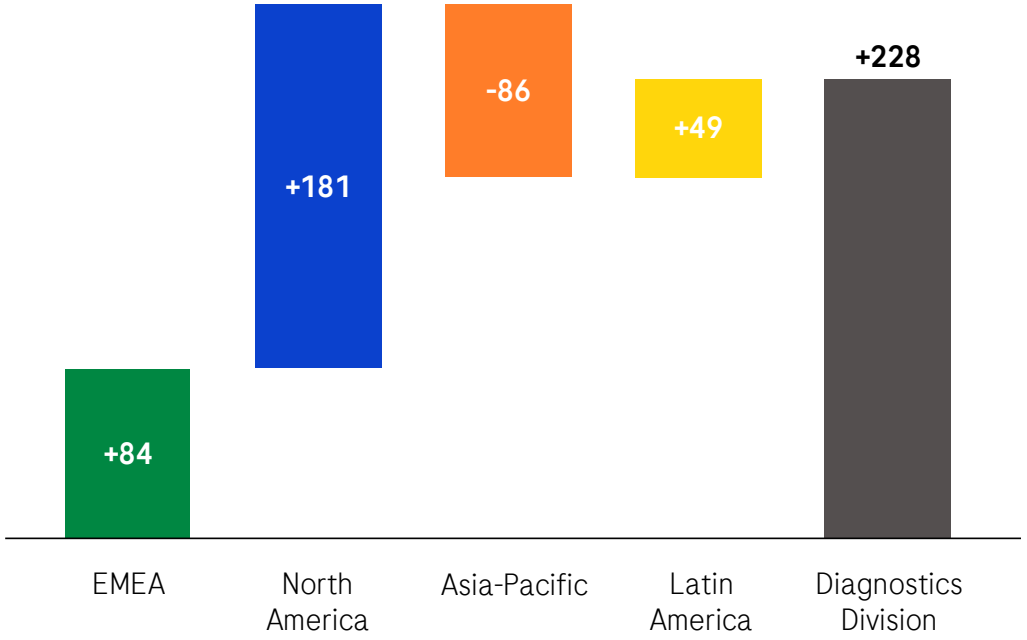
Growth across regions, offset by APAC

Sales YTD CHFm / % of total sales

Total CHFm = 6,735 (3% at CER)

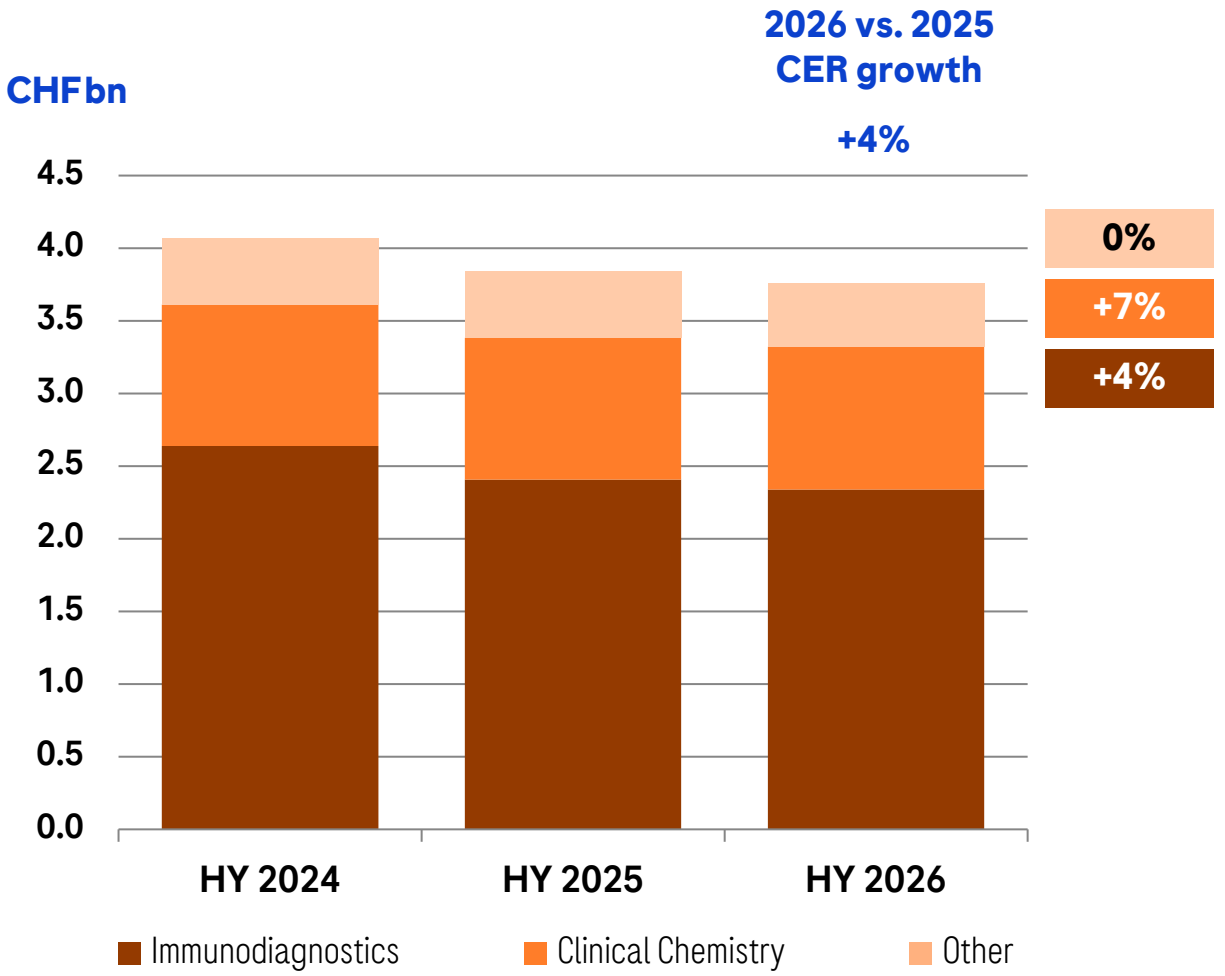


	Growth at CER %
EMEA	+3
North America	+8
of which US	*+8*
Asia-Pacific	-5
of which China [1]	*-15*
Latin America	+10



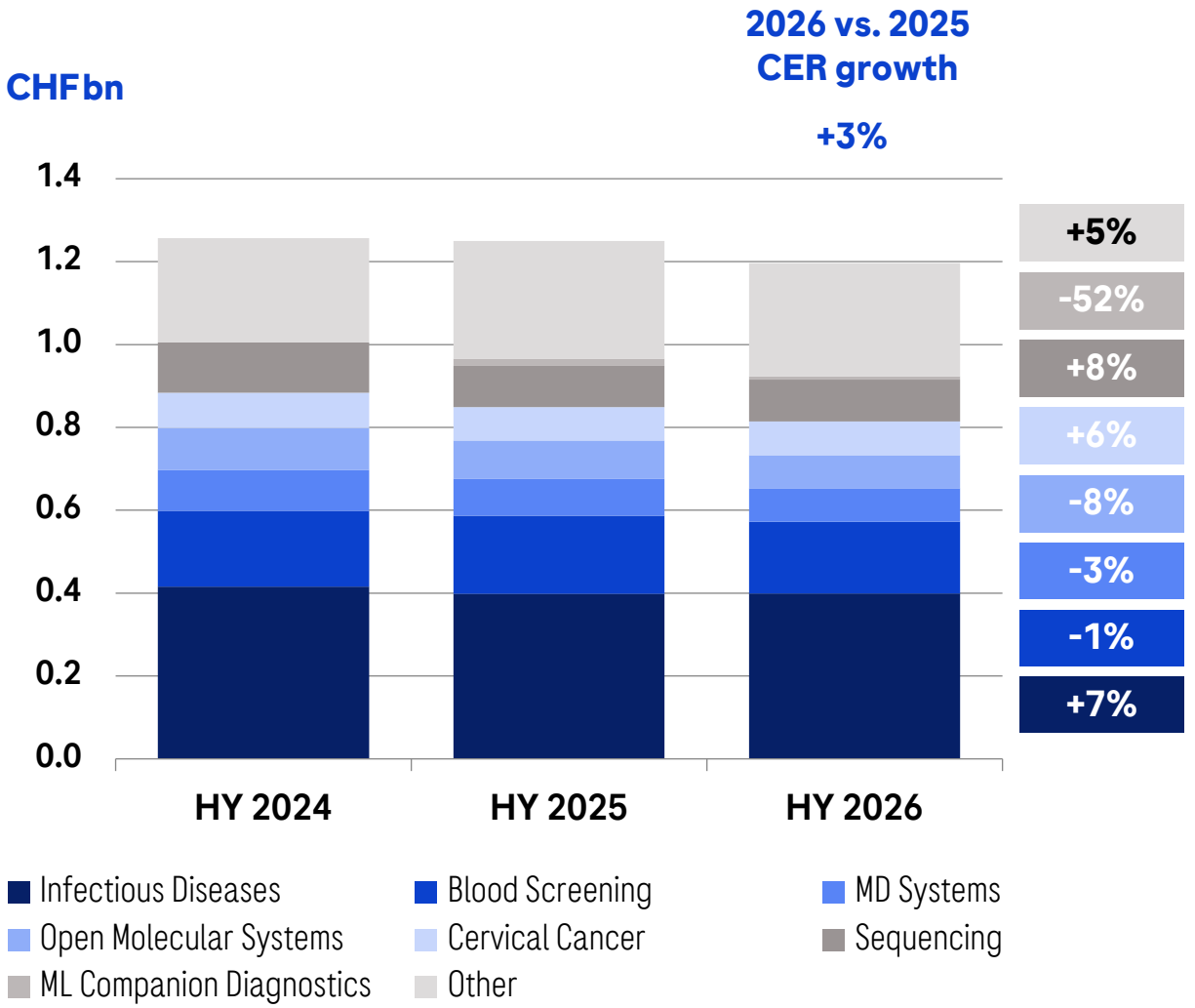
1. China incl. Hong Kong; CER: Constant exchange rates (avg. full year 2025); EMEA: Europe, Middle East and Africa

Core Lab



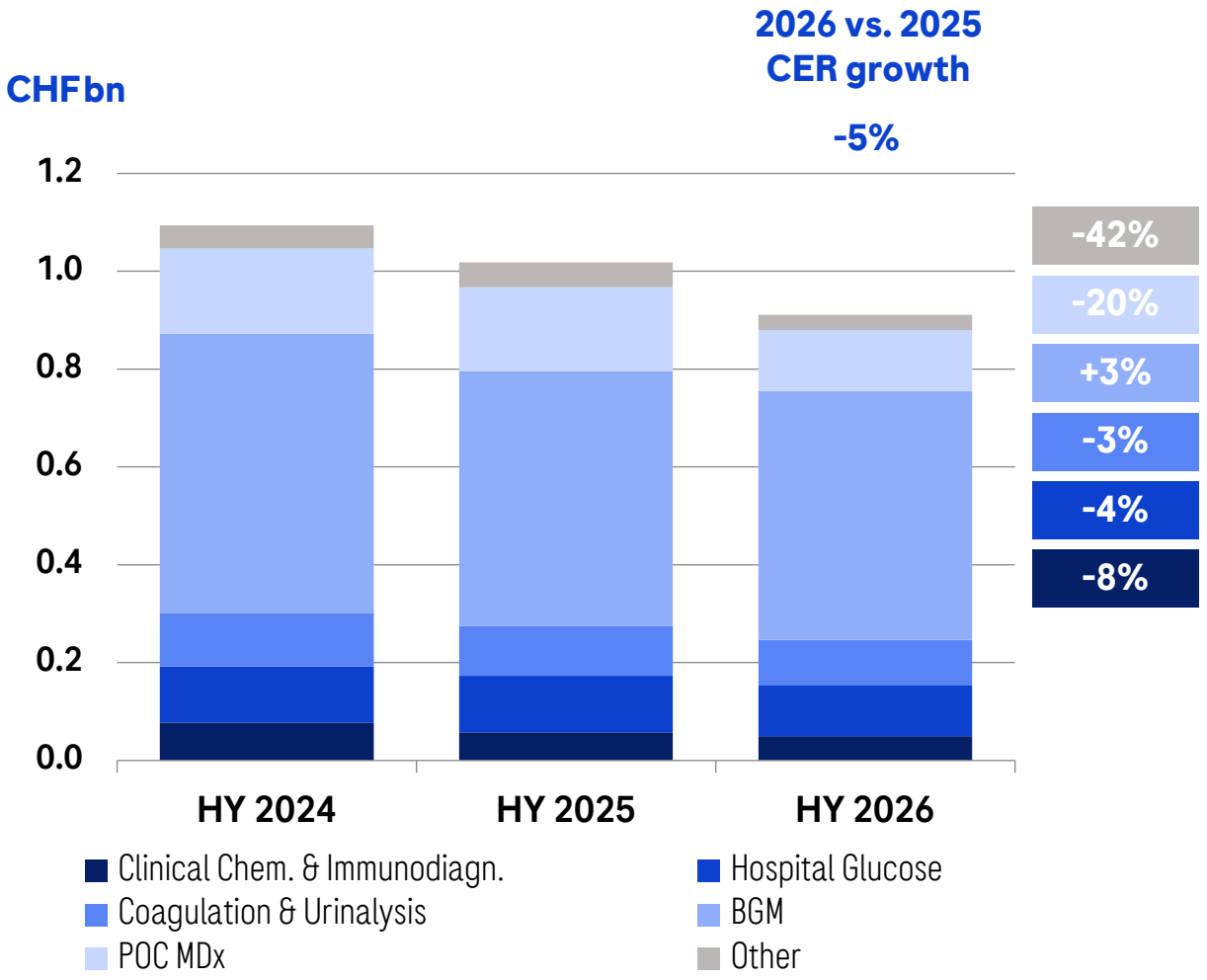
CER: Constant exchange rates (avg. full year 2025)

Molecular Lab



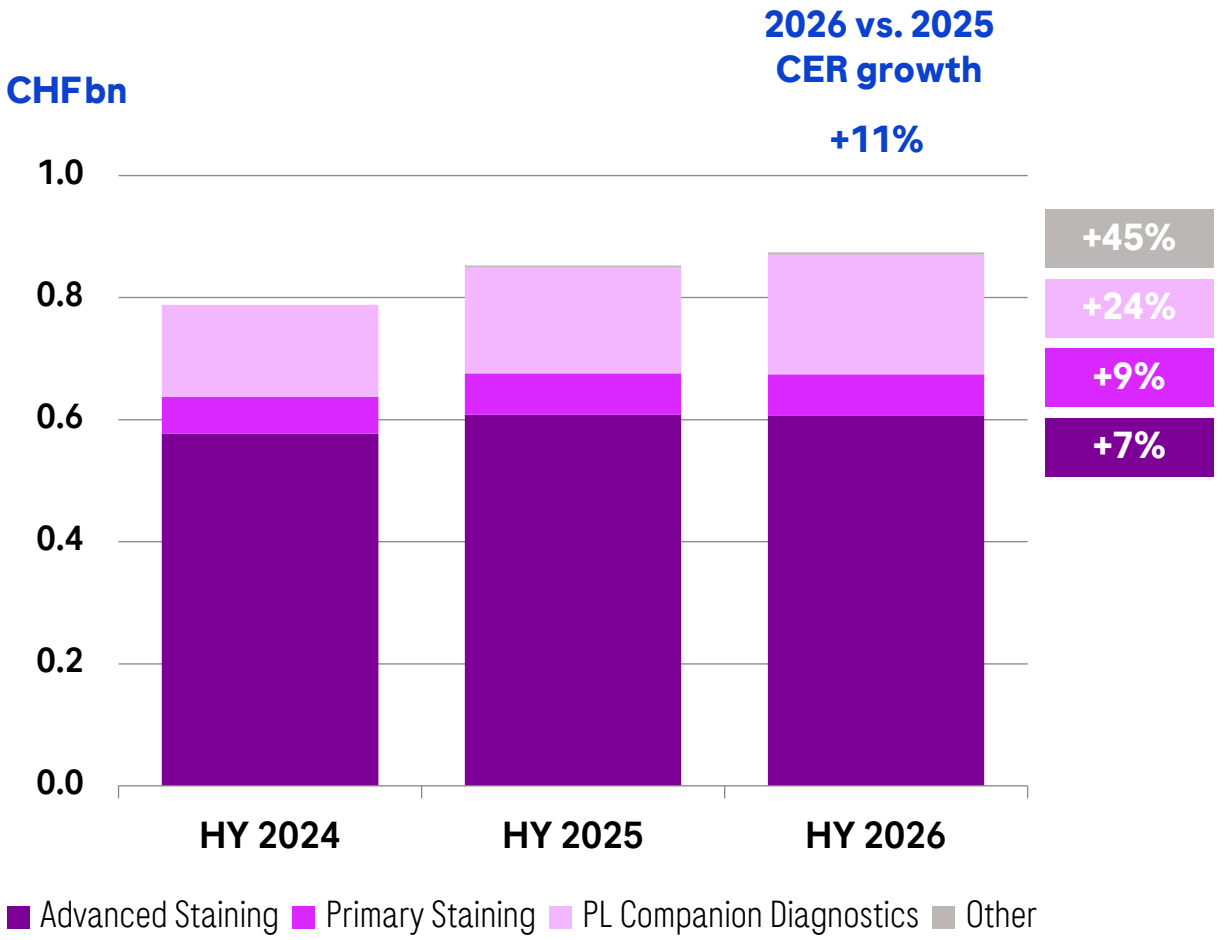
CER: Constant exchange rates (avg. full year 2025); MD: Molecular Diagnostics; ML: Molecular Lab

Near Patient Care



CER: Constant exchange rates (avg. full year 2025); BGM: Blood Glucose Monitoring; MDx: Molecular Diagnostics; POC: Point of Care

Pathology Lab



CER: Constant exchange rates (avg. full year 2025); PL: Pathology Lab

Roche Group development pipeline

Marketed products development programmes

Roche Pharma global development programmes

Roche Pharma research and early development (pRED)

Genentech research and early development (gRED)

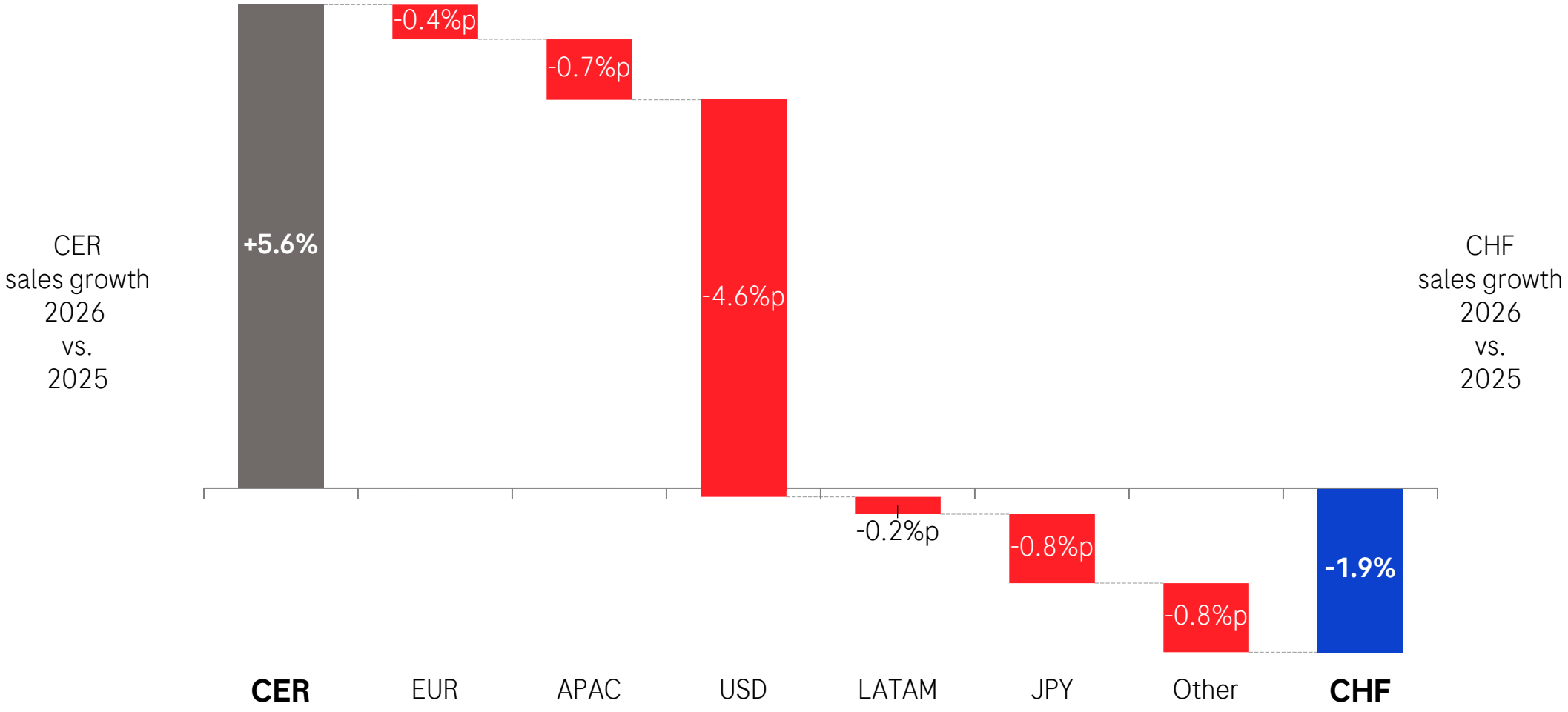
Pharma sales appendix

Diagnostics sales appendix

Foreign exchange rates information

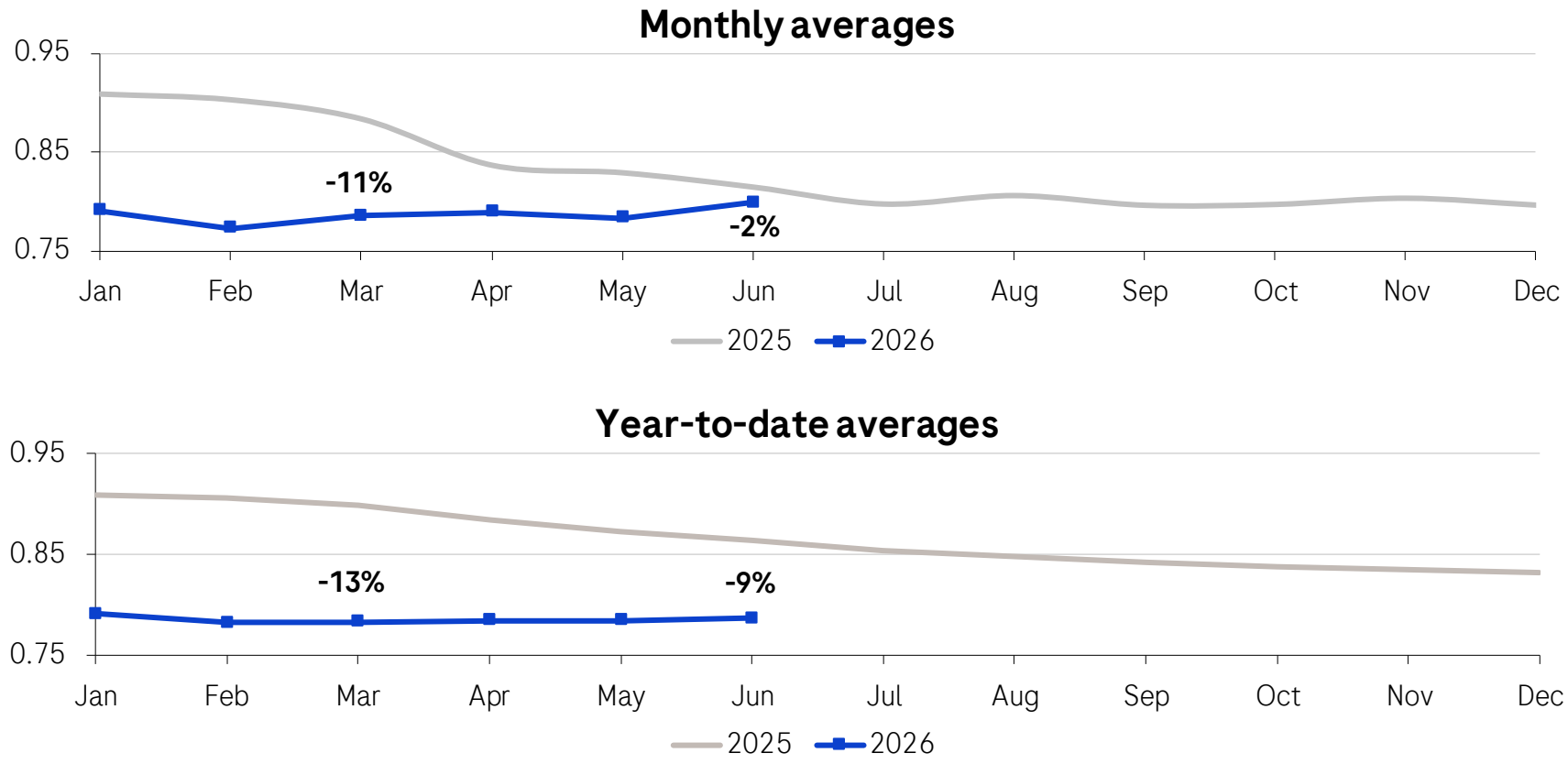
Exchange rate impact on sales growth

Negative impact mainly driven by the USD, JPY, EUR, ARS (LATAM), CNY (APAC) and TRY (Other)

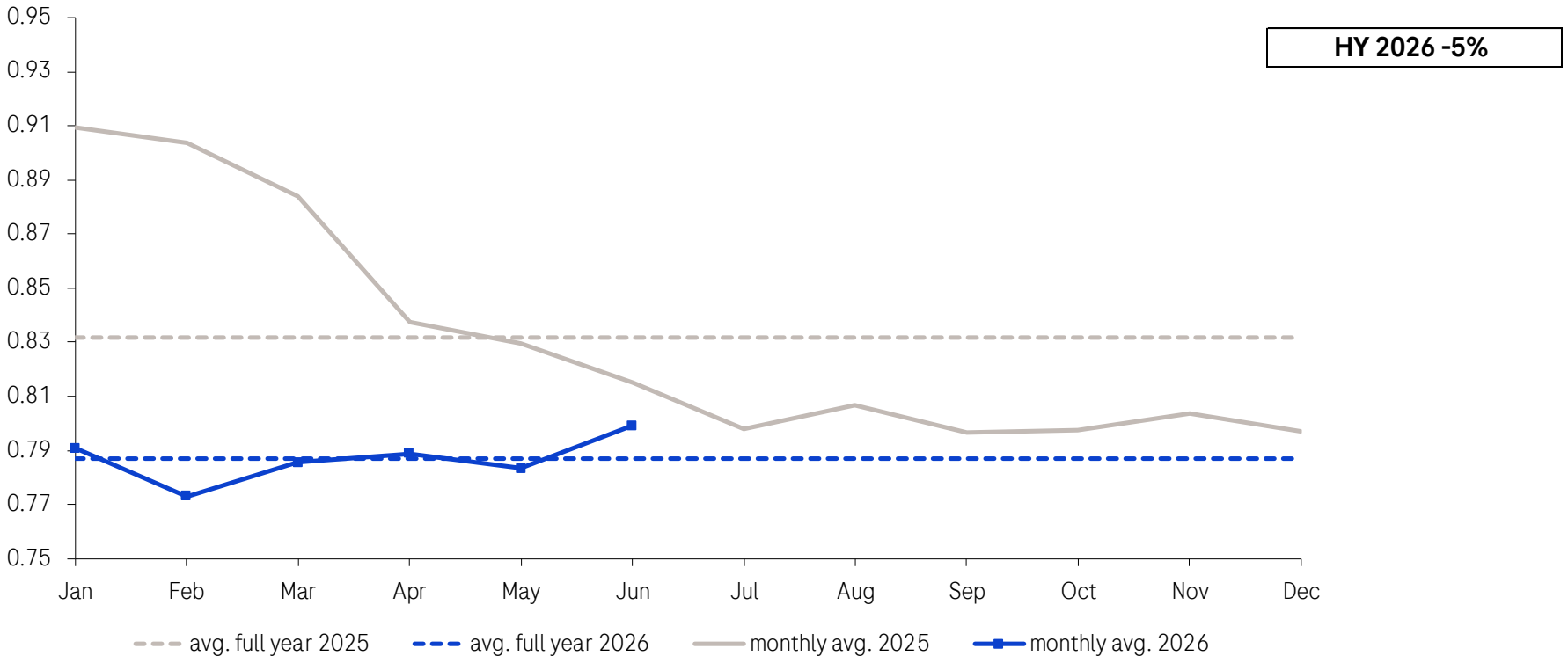


CER: Constant exchange rates (avg. full year 2025)

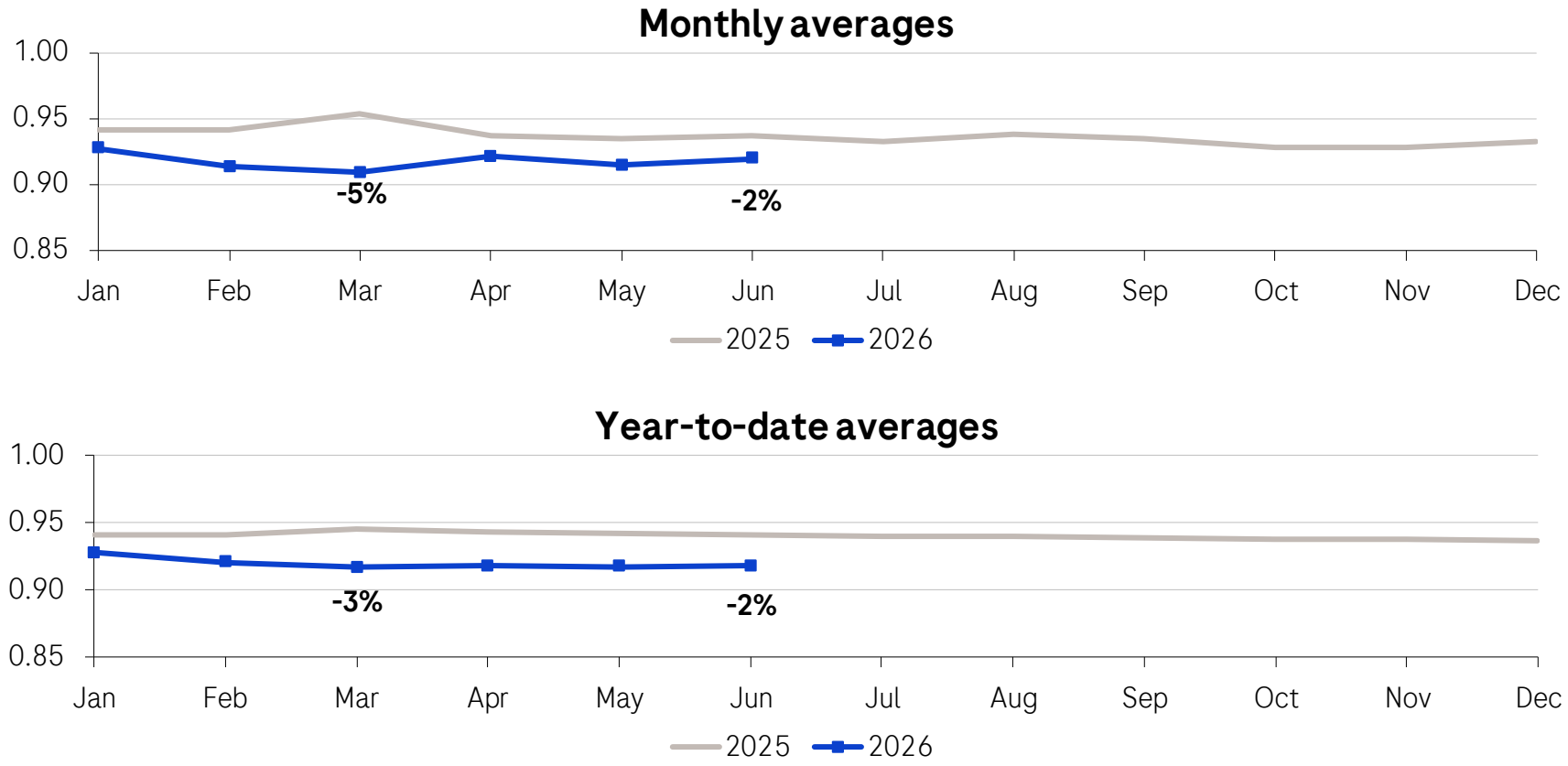
CHF/USD


Monthly averages
0.95
0.85
0.75
-11%
-2%
Jan
Feb
Mar
Apr
May
Jun
Jul
Aug
Sep
Oct
Nov
Dec
2025
2026
Year-to-date averages
0.95
0.85
0.75
-13%
-9%
Jan
Feb
Mar
Apr
May
Jun
Jul
Aug
Sep
Oct
Nov
Dec
2025
2026

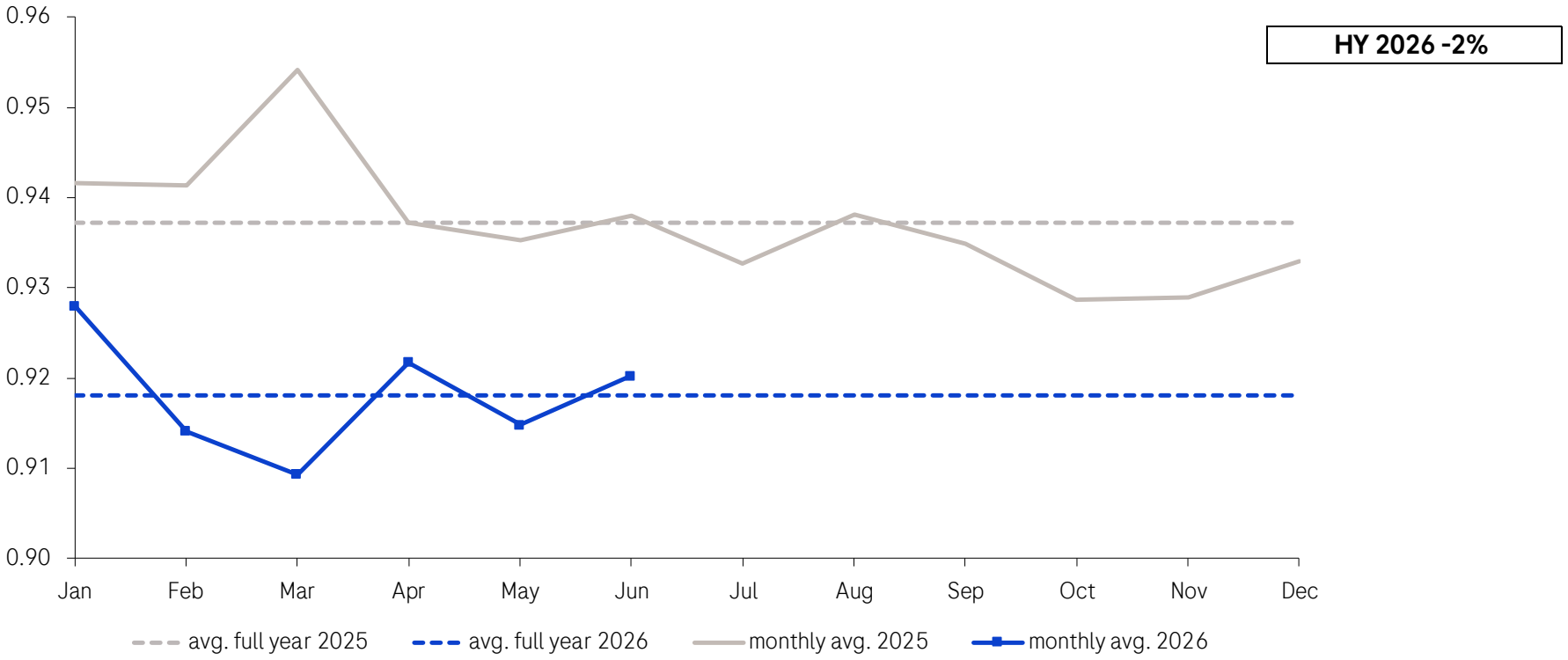
CHF/USD


HY 2026 -5%
0.95
0.93
0.91
0.89
0.87
0.85
0.83
0.81
0.79
0.77
0.75
Jan
Feb
Mar
Apr
May
Jun
Jul
Aug
Sep
Oct
Nov
Dec
avg. full year 2025
avg. full year 2026
monthly avg. 2025
monthly avg. 2026

CHF/EUR

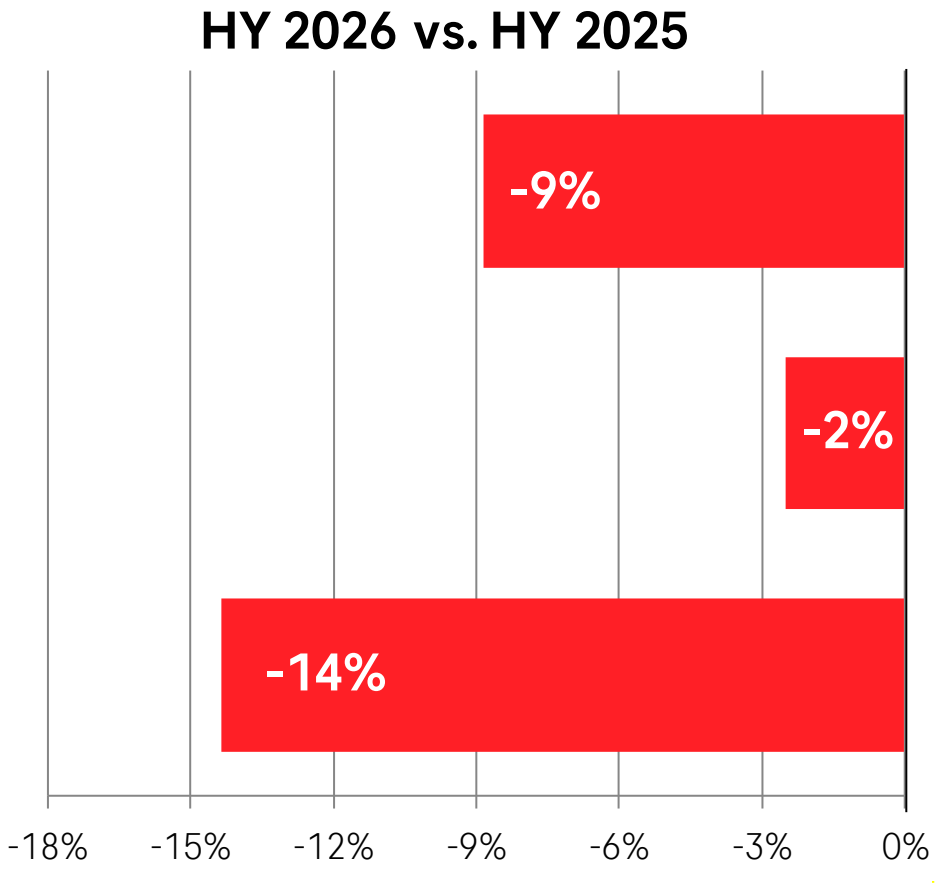

Monthly averages
1.00
0.95
0.90
0.85
-5%
-2%
Jan
Feb
Mar
Apr
May
Jun
Jul
Aug
Sep
Oct
Nov
Dec
2025
2026
Year-to-date averages
1.00
0.95
0.90
0.85
-3%
-2%
Jan
Feb
Mar
Apr
May
Jun
Jul
Aug
Sep
Oct
Nov
Dec
2025
2026

CHF/EUR


HY 2026 -2%
0.96
0.95
0.94
0.93
0.92
0.91
0.90
Jan
Feb
Mar
Apr
May
Jun
Jul
Aug
Sep
Oct
Nov
Dec
avg. full year 2025
avg. full year 2026
monthly avg. 2025
monthly avg. 2026

Average CHF exchange rates

	HY 2026	HY 2025
USD	0.79	0.86
EUR	0.92	0.94
JPY	0.50	0.58



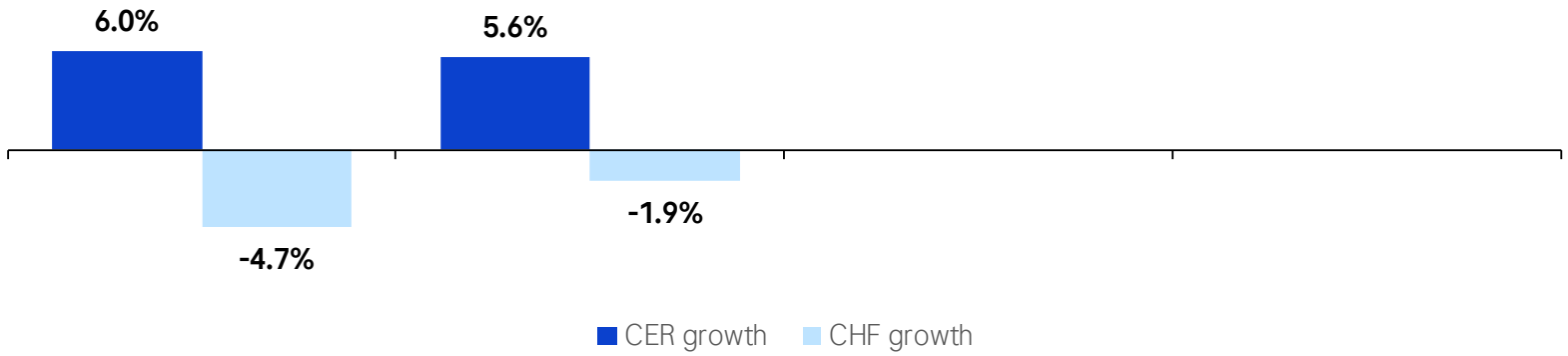
Exchange rate impact on sales growth

HY 2026: Negative impact of JPY, USD, CNY, and EUR

Development of average exchange rates versus prior year period

	Q1	HY	YTD Sep	FY
CHF / USD	-12.9%	-8.9%		
CHF / EUR	-3.0%	-2.5%		
CHF / JPY	-15.3%	-14.3%		
CHF / CNY	-8.3%	-3.6%		
Difference in CHF / CER growth	-10.6%p	-7.5%p		

Sales growth 2026 vs. 2025



CER: Constant exchange rates (avg. full year 2025)

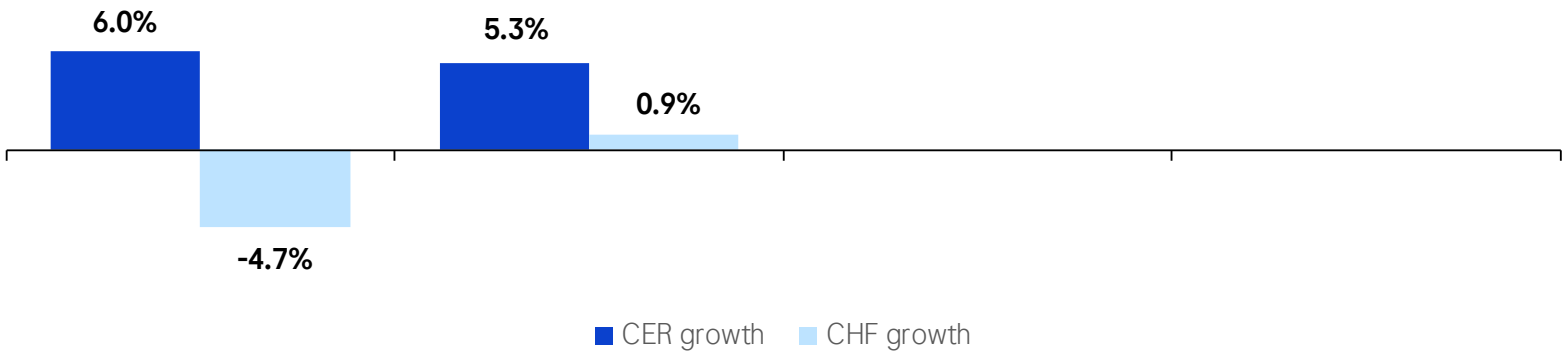
Exchange rate impact on sales growth

HY 2026: Negative impact of JPY, USD and EUR

Development of average exchange rates versus prior year period

	Q1	Q2	Q3	Q4
CHF / USD	-12.9%	-4.5%		
CHF / EUR	-3.0%	-1.9%		
CHF / JPY	-15.3%	-13.3%		
CHF / CNY	-8.3%	1.5%		
Difference in CHF / CER growth	-10.6%p	-4.4%p		

Sales growth 2026 vs. 2025



CER: Constant exchange rates (avg. full year 2025)

Doing now what patients need next